

DRAFT #3

FINAL REPORT

CDPH-UIC Health Disparities Grant Evaluation

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ACRONYM GLOSSARY

CA - Community Area
CBO - Community-Based Organization
CDPH - Chicago Department of Public Health
CHA - Community Health Assessment
CPEZ - Community Planning and Equity Zones
CIA - Cumulative Impact Assessment
CL - Community Lead
EDST - Equity Decision Support Tool
HC25 - Healthy Chicago 2025
HCEZ - Healthy Chicago Equity Zones
HEiAP - Health Equity in All Policies
HEZ - Health Equity Zone
HiAP - Health in All Policies
PC - Project Coordinator
REAP - Racial Equity Action Plan
RL - Regional Lead
TA - Technical Assistance

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Each of the main authors of this report are faculty or students in the Division of Community Health Sciences at the University of Illinois Chicago School of Public Health. We are an interprofessional, transdisciplinary group of educators, researchers, practitioners, students, and staff dedicated to promoting health equity as a social justice imperative. We focus on the role of fundamental biopsychosocial determinants of health across populations and through the life course with the goal of improving and sustaining population health and well-being. We are committed to developing, enhancing, and using evidence-informed approaches in the planning, designing, implementation, and evaluation of public health interventions and innovations. These interventions occur at multiple levels (e.g., individual, family, community, policy, systems) and across multiple populations and sectors. To effectively carry out our mission to improve community and population health, we collaborate with communities, public health agencies, research partners/networks, government, and nonprofit organizations. Brief bios and positionality statements for each author appear below:

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Additional student authors – including Aeysha Chaudhry, Candace Gary, Hodan Jibrell, Abha Mahajan, Simone Taylor, and Izziah Thabath – are former master’s or PhD students in the Division of Community Health Sciences who have graduated at the time of this writing

EXECUTIVE SUMMARY

Introduction

In accordance with their vision of eliminating long-standing health and life-expectancy inequities in Chicago, the Chicago Department of Public Health (CDPH) has established two offices: 1) the Office of Community Planning and Equity Zones (CPEZ), and 2) the Office of Health Equity in All Policies (HEiAP). The CPEZ office is responsible for implementing the Healthy Chicago Equity Zones (HCEZ) initiative which facilitates the development and implementation of hyperlocal interventions for improving community health throughout all 77 Chicago neighborhoods. The HEiAP office supports institutional and systems changes that strengthen public health-relevant infrastructure across multiple health and non-health-focused sectors in Chicago. Together, the CPEZ and HEiAP offices, and their initiatives, aim to enhance the capacities of CDPH, other City of Chicago departments, and community-based organizations throughout the city to effectively promote health and racial equity.

In 2022, CDPH began working with a team of scholars and students from the Division of Community Health Sciences at the University of Illinois Chicago School of Public Health to evaluate these initiatives. This report describes the current results of this evaluation. The report begins with an overview of the foundational elements of each initiative, drawing on literature from implementations of similar programs in other U.S. contexts (sections 1.1 and 1.2). We then describe how these initiatives came to be in Chicago (section 1.3) and provide descriptive overviews of the main activities of each initiative (sections 1.4. and 1.5). Following this, we provide both a general (section 2) and detailed (section 3) synopsis of our methodology. We then report our main findings and interpretations (section 4), acknowledge the limitations of our work (section 5), and offer some recommendations (section 6) and conclusions (section 7).

Evaluation Approach

This evaluation was *formative* in nature, investigating these initiatives in real time as they are implemented in order to provide information to CDPH that will guide their improvement going forward. Furthermore, we have striven to incorporate principles of *developmental* evaluation into our work, aiming for tight integration between evaluators and program staff, and allowing for methodological flexibility. We have also incorporated the principles of *participatory* evaluation, including the establishment of a multi-partner advisory group. The data collection, analysis, and interpretation in our participatory evaluation relied on a convergent *mixed methods* design, where both quantitative and qualitative data are collected and analyzed concurrently and come together – or converge – in the interpretation phase of the project

Key steps in our evaluation included:

- 1) **Engagement of an advisory group (AG)** to ensure that the evaluation was grounded in the priorities of the key partners involved. Our AG consisted of representatives from the multiple partner groups including CDPH staff, and external partners of the CPEZ and

HEiAP offices. The AG guided all steps of the evaluation, and members were compensated for their participation.

- 2) **Development of a program theory and logic model** that represented a comprehensive shared understanding of the HCEZ initiative as well as several HEiAP initiatives. These tools were developed iteratively by compiling and reviewing informational resources about HCEZ and various HEiAP initiatives that were provided to us by CDPH, and by conducting informal interviews with key CDPH, CPEZ, and HEiAP staff.
- 3) **Co-development of evaluation questions**, drawn from what CDPH and the AG considered to be the most important, innovative, and potentially impactful elements of the larger HCEZ/HEiAP program theory. Following a review of data availability and data collection feasibility, we arrived at a final list of seven evaluation questions.
- 4) **Data collection and compilation** to address our evaluation questions. Our main sources of data included key informant interviews with 32 individuals, key documents describing HCEZ and HEiAP operations and products, HCEZ monthly reports on performance, and an HCEZ community lead survey (n=33).
- 5) **Mixed methods analysis** that included deductive and inductive approaches to analyzing the interview data, document review and analysis of HCEZ and HEiAP reports and presentations, and quantitative analysis of HCEZ performance measure and survey data.
- 6) **Interpretation of findings** through the engagement of our AG in an interactive “Community Café” exercise, and via our team’s development and review of a joint data display that presented our key qualitative and quantitative results and enabled us to assess convergence.

Key Findings

Our evaluation produced a set of integrated findings for each evaluation question. These findings by evaluation question are highlighted in **Table 5** of the main report. Here, we present what we have concluded are this evaluation's "5 Key Findings" that cut across multiple evaluation questions.

Key Finding #1

The HCEZ and HEiAP initiatives have helped to build Chicago's "health equity muscle." As such, the city is now better prepared to protect its most vulnerable residents from both emergent and chronic health challenges.

Brief Summary: We have observed clear signs of improvement in several areas that can be considered fundamental contributors to, or determinants of, health inequities, including:

- The **buildup of skills and resources (i.e., capacity)** for identifying and confronting health priorities within communities. For example:
 - On average, HCEZ community lead survey respondents reported "somewhat" or "strong" agreement with the statement about developing "*useful new skills*" through their involvement in the HCEZ initiative (mean = 3.71 out of 4).
 - Each region of Chicago reported dozens of improvements in data collection and reporting capacity.
- The **buildup of awareness and prioritization of health equity** within CDPH and other City departments. For example:
 - Through efforts such as the creation of the new Equity Decision Support Tool (EDST) and their role guiding the Community Impact Assessment, the HEiAP office has laid the groundwork for future health equity work across the department and the City and has made progress in efforts to "*normalize health equity framing*" within the department.
- The **increase in meaningful and diverse partnerships** between and among CBOs and the City. For example:
 - HCEZ regional and community leads have identified over 600 partnerships that have been built or strengthened through their involvement in the HCEZ initiative.

Key Finding #2

Together, the HCEZ and HEiAP initiatives have led to increases in the extent to which CBOs and City departments have adopted new health equity goals into their long-term work plans.

Brief Summary: Our analysis of available data reveals several examples of increases in the organizational adoption of health equity goals around Chicago. For example:

- On average, HCEZ community lead survey respondents reported substantial agreement with the idea that they have been “*working collaboratively towards shared goals*,” with average responses closer to “strongly agree” than “somewhat agree” (mean = 3.69 out of 4).
- As stated by one HCEZ regional lead: “*We have established, I think five or six core areas around social determinants of health that is still guiding our work today around public health. ... [HCEZ] has definitely impacted our agency and as a result had had a direct impact in our communities. And we’re seeing the results of that voices now being raised where they may in the past feel like they didn’t even have a voice.*”
- HEiAP efforts have also focused on helping other City departments to become aware of how their work may impact health equity in Chicago. For example, with technical assistance support from HEiAP, all City departments and sister agencies published REAPs where they communicated their goals on improving racial equity.

Key Finding #3

Growth in trust among community organizations and between communities and the City is evident but has been slowed by structural barriers.

Brief Summary: Our findings suggest that the CDPH health equity initiatives have fostered a sense of shared leadership and growing perceptions of trust, both within and between communities, and between communities and City government. However, due to several structural challenges, growth in these important areas has progressed more slowly than growth in capacity/skills or growth in partnerships. For example:

- On average, HCEZ community lead survey respondents reported “somewhat” or “strong” agreement with the statement about greater levels of “*mutual trust*” between their organization and CDPH (mean = 3.38 out of 4). However, it is meaningful that this mean score is closer to “somewhat” than “strong” agreement. We also note the presence of substantial regional variation in responses.
- Multiple HCEZ regional leads mentioned feeling strong, positive connections to the CDPH staff with whom they worked most closely.

- At the same time, several key barriers to growth in trust were identified in our data, including the slow contracting process with the City and communication challenges.

Key Finding #4

The buildup of skills, collaborators, and trust with (and within) communities is strongly linked (i.e., correlated) with progress in action planning and sustainability.

Brief Summary: A correlation analysis of the HCEZ community lead survey provides compelling evidence that the building of skills/capacity, the strengthening of collaborations, and growth in trust are each directly associated/correlated with perceived progress with respect to the HCEZ work. Furthermore, perceived progress is associated/correlated with confidence in the sustainability of this work. We infer from these findings that continued investments in building skills, a sense of collaboration, and trust with and within communities is necessary in order to achieve continued and long-term progress in addressing health inequities in Chicago.

Key Finding #5

Now is a critical moment for the sustainability of HCEZ- and HEiAP-related work.

Brief Summary: Our findings indicate that to continue progress in addressing health equity, both through HCEZ and HEiAP, there is an urgent need for continued investment in support of these initiatives. For example:

- On average, HCEZ community lead survey respondents reported a level of agreement with the statement about their ability to “*sustain (their) health equity work*” that is closer to “somewhat agree” than “strongly agree” (mean = 3.32 out of 4).
- Our interviews revealed the concern that “*Some in (CDPH) leadership are struggling to understand the benefit of the transformative work*” that the initiatives were promoting and expressed concern that because both projects were funded through COVID-era grants and operated outside of the standard structure of the department, they would be ended as ‘COVID-era’ initiatives.

Finally, we should add that in working to understand our findings within the larger environmental contexts in which CPEZ/HCEZ and HEiAP exist, we have identified some structural factors that likely have a profound influence on the implementation and sustainability of these projects. Specifically, we note that while both the HCEZ and HEiAP initiatives have shown a deep commitment to implementing anti-racist principles as highlighted in Healthy Chicago 2025, we have observed that the current internal practices and processes of CDPH may

be out of alignment with these principles, particularly with regard to the heavy workload and low level of power among junior Black and brown CDPH staff working on the initiatives.

Recommendations

Taken together, our findings offer compelling evidence that in a large and diverse city like Chicago, the use of a hyperlocal lens to support equity-centered and people-powered efforts that champion resilient communities to make their own decisions on health priorities and utilize local assets to solve health problems is both innovative and impactful. However, these gains are at great risk of being lost. To prevent this from happening, we offer the following recommendations for consideration:

- 1) Ensure adequate support for CPEZ and HEiAP staff, as well as RL and CL partner agencies, as part of City-funded public health services.**
 - a. Ideally, this would entail finding additional revenue from the City of Chicago, rather than relying on one-off grant support.
 - b. This will require institutional commitment to maintaining adequate internal staffing and support for both CPEZ/HCEZ and HEiAP.
 - c. CDPH should also consider ways in which it can continue to provide low-cost support for the types of generative partnership development activities that have been initiated via the HCEZ and HEiAP initiatives.
 - d. Some cost efficiencies may be achieved by establishing mechanisms that ensure a more reliable, timely, and consistent matching of TA with the needs of community groups.
- 2) Invest in less hierarchical structures.**
 - a. CDPH should continue to look for ways to reduce top-down approaches when implementing power-sharing initiatives with communities, as equity-focused work requires ensuring that all voices are heard and valued, especially those with less power and autonomy.
 - b. We specifically recommend increased communication between CLs and CDPH.
 - c. We also recommend that front level staff at CDPH – those in closest contact with communities (e.g., PCs in CPEZ) – be granted greater power within CDPH to weigh in on decisions and enact change.
- 3) Continue to deepen genuine collaboration with community partners.**
 - a. Truly advancing health equity in Chicago will require the institutionalization within CDPH of the foundational trust that community partners have the ability to co-develop solutions.
 - b. We specifically recommend that CDPH include all levels of community partners in decision-making about the future shape of these initiatives. This includes being more transparent and more hands-on when creating scopes of work for communities.

- c. We also recommend attention to the city-wide (i.e., not unique to CDPH) structural barriers that inhibit timely compensation and contracting, as these barriers can negatively impact trust-building and partnership development.
- 4) **Continue CDPH's commitment to increasing community power to address SDoH while deepening civic engagement to hold policy makers accountable for policies that advance healthy equity.**
 - a. Community organizations want more connections to City offices, and so we suggest that CDPH facilitate this by building intentional linkages. Towards this end, opportunities may exist for HEiAP and CPEZ/HCEZ to work in closer concert with one another.
- 5) **Raise awareness of the transformative potential of HEiAP.**
 - a. We recommend that CDPH Executive Leadership create more transparency on the strategic direction for HEiAP to foster health equity in Chicago, as this will raise critical awareness of the transformative value of this work.

Conclusions

Clearly, these initiatives are moving Chicago in the right direction. Yet to maintain and continue this progress, CPEZ/HCEZ and HEiAP need sustained commitment from the City and CDPH. Steady funding, increased transparency, and efforts to address structural barriers to collaboration should be critical priorities for the City. With such a sustained commitment, Chicago and CDPH has a real chance to be a model for the country in how to do this type of community co-led work effectively.

1. INTRODUCTION AND BACKGROUND

With the goal of eliminating long-standing health and life-expectancy inequities in Chicago, the Chicago Department of Public Health (CDPH) is leading a broad-based effort to address the root causes of health inequities, namely the inequitable distribution of the social determinants of health (SDoH), defined as “the conditions in which people are born, grow, work, live, and age” (U.S. Department of Health and Human Services, n.d.; World Health Organization 2023), and the systems that create and sustain these conditions. CDPH’s work in this area partially entails an initiative to facilitate the development and implementation of hyperlocal interventions for improving community health throughout all 77 Chicago neighborhoods (i.e., Community Planning and Equity Zones’ (CPEZ’s) Healthy Chicago Equity Zones initiative (HCEZ)), and an office to create institutional and systems change that can bolster the public health-relevant infrastructure across multiple health and non-health-focused sectors (i.e., the Office of Health Equity in All Policies (HEiAP)).

With funding from the Centers for Disease Control and Prevention’s (CDC’s) *National Initiative to Address COVID-19 Health Disparities Among Populations at High-Risk and Underserved, Including Racial and Ethnic Minority Populations and Rural Communities* (the “Health Disparities Grant”), CDPH’s immediate focus was on reducing disparities in COVID-19. In the long-term, CDPH hopes that the initiatives included in the Health Disparities Grant will address underlying structural, social, economic, and physical conditions that persistently create and sustain racial inequities in other highly prevalent and emergent health conditions within Chicago.

As described below, similar Health Equity Zones (HEZs) and HEiAP initiatives have been successfully implemented in other U.S. contexts (Patriarca & Ausura, 2016; Rhode Island Department of Health, 2021) and have served as inspiration for the Chicago versions.

1.1. Health Equity Zones: Concept and Literature

Health Equity Zones (HEZs) recognize the importance of building community power to aid in addressing SDoH at the local level. Using that power, communities may hold policy makers accountable to implement the systems-level policy changes needed to address health inequities (Iton et al., 2021). Interventions that focus on building community power have been proven effective in management of disease (including both prevention and reducing severity), promotion of high-quality healthcare services, and environmental change (Iton et al., 2021). HEZ initiatives put communities at the center of this systems-level work by building multi sectoral partnerships with local community organizations, health departments, school districts, businesses, and key government partners.

According to the Rhode Department of Health (RIDOH) and ChangeLab Solutions (2021), HEZs are geographic areas where public investment responds to community-specific health inequities through the unique skills, opportunities, and tools that exist in the area. HEZs are structured as community-led collaboratives with the long-term goal of supporting each zone as they develop into a self-sustaining, self-funding entity that can respond to evolving community needs and priorities.

1.1.1. Key Elements of HEZs

To implement a successful and sustainable HEZ initiative, RIDOH (2021) identifies four essential components. In particular, HEZs must follow: (1) a health equity-centered approach to prevention work that is (2) place-based, (3) community-led, and (4) addresses SDoH.

Being health equity-centered means that evaluating and responding to population health inequities must be the primary organizing strategy for the respective organization's approach to prevention. Health equity should be at the forefront of its culture and departmental structures should endeavor to include equity-informed decision-making for the long-term mission and vision, as well as daily practices. Health equity principles must also extend to external collaborations with partners, communities, businesses, nonprofits, government agencies, and consultants.

Place-based suggests that an equitable prevention strategy must focus on providing tools and resources to particular geographic areas based on their unique needs instead of funding all places equally. Health outcomes are intricately linked to where individuals live – the surrounding environment that includes their homes, schools, community centers, and workplaces. Any effective prevention strategy must address the environmental factors that contribute to health inequities in specific communities.

Community-led indicates that the state or local department of health must share power with community members in a significant manner such that project selection is based on the communities' identified needs and priorities. Prevention strategies that are mainly steered by health officials and external experts rarely turn out to be sustainable because decision-making power and resources are not in the hands of those most negatively affected by the health threats (National Academies of Sciences, Engineering, and Medicine, 2017). State and local health department leaders should trust that communities best understand their own needs and priorities. In community-led public health work, interventions and strategies serve the public, not vice versa.

SDoH are the conditions in the environments where people are born, live, work, play, and age that shape a wide array of health outcomes (U.S. Department of Health and Human Services, n.d.; World Health Organization 2023). SDoH include factors like access to quality job opportunities, education, healthy food, safe housing, and political participation. Effective, equity-centered prevention strategies are necessary to address SDoH. SDoH are shaped by structural drivers of health inequities rooted in the socio-political system (Solar & Irwin, 2010).

1.1.2. HEZs in Action

To date, HEZ initiatives and similar programs integrating the four essential components identified above have been implemented at the state, county, and city levels. Arguably the most well-established HEZ program is in the state of Rhode Island: the Rhode Island Health Equity Zones (RIHEZs) program. RIDOH created the RIHEZ initiative using lessons learned from their Centers for Health Equity and Wellness (CHEW) initiative – a project supporting community

organizations in their promotion of health equity and wellness through prevention – the (Rhode Island Department of Health & ChangeLab Solutions, 2021).

The RIHEZs are geographic locations where communities are charged with addressing a health inequity most affecting their specific community, with a focus on five domains: (1) integrated healthcare, (2) community resilience, (3) physical environment, (4) socioeconomic, and (5) community trauma (Rhode Island Department of Health & ChangeLab Solutions, 2021). Since 2015, RIHEZ initiative has supported community leaders and residents to form 15 HEZ collaboratives in communities across Rhode Island (the program initially created 11 HEZs; ten of these original zones still exist and an additional five have been added). Examples of interventions emerging from RIHEZ work include multi-sector behavioral health training with law enforcement and first responders; workforce development, civic education, and leadership training; green jobs for those who were previously incarcerated; and other interventions addressing transportation access and food insecurity (Rhode Island Department of Health & ChangeLab Solutions, 2021). These hyperlocal collaboratives have also been able to respond to changing community needs. For example, during COVID-19, they served as hubs for personal protective equipment (PPE) distribution and as vaccination clinics (Rhode Island Department of Health & ChangeLab Solutions, 2021).

Similar statewide programs have been launched in Washington, Oregon, Georgia, Minnesota, Maryland, and Pennsylvania, as highlighted in **Appendix Table A.1**. Moreover, the HEZ model and similar equity-focused, community-led, place-based initiatives are spreading rapidly. In addition to the programs discussed above, there are many HEZ and related initiatives that are still under development, including in Illinois, Vermont, Massachusetts, Indiana, Michigan, Ohio, Texas, and California.

1.1.3. Challenges in Measuring Success

According to our review of the available literature, the HEZ concept can be considered a successful innovation to address health equity. This positive assessment of the program has been reflected by states, counties, and cities across the country who have chosen to adopt the HEZ model (Rhode Island Department of Health, 2021). In addition, the National Academy of Medicine (2024) cites the success of the strategy across the systems where it is being implemented, and other organizations that serve populations with highly specific health concerns have also supported the HEZ model as a way to access populations that are often excluded from national health equity strategies (National Resource Center for Refugees, Immigrants, and Migrants, 2024).

Uniform assessment of the HEZ model in order to understand specific outcomes and impact is a challenge due to the widely disparate nature of interventions and goals and the lack of standardization across zones. RIDOH has recommended developing evaluation plans in tandem with building HEZs. These plans include having each HEZ establish evaluation goals that connect to the greater context of the initiative and goals that are tailored to the unique needs of the zone. Each evaluation goal should have specific evaluation metrics tied to program outputs and outcomes with benchmarks to track progress and a system in place to collect feedback from community members and partners. RIDOH suggests that early phases of evaluation focus on

process metrics, rather than intervention impact metrics, with iterations to the evaluation plan as the program progresses (Rhode Island Department of Health & ChangeLab Solutions, 2021). Some scholars have supported and built on this suggestion, proposing that early assessment should focus on the initiative's success at fostering collaboration and building capacity for implementing new programs and advocating for policy change, rather than assessing health outcomes according to specific health metrics (Putland et al., 2013; Warner, 2022).

From this process-focused perspective, the HEZ model has produced many successes. Of the initiatives that have begun programming, HEZs have been able to create new staff positions, establish and maintain programs, and engage community partners and service communities across each zone to address their targeted disparities. For example, since the initial implementation of the project, Rhode Island has been able to maintain and develop programming across zones and create new coalitions to implement interventions for nearly a decade (Rhode Island Department of Health & Change Solutions, 2021; Rhode Island Senate Fiscal Office, 2023). Even among those initiatives that have not yet begun implementation, the initial program design and application processes have already created and formalized new coalitions and fostered relationship building between service providers, community organizations, and state health departments.

An example of this is in Washington State, where implementation has not yet started but there has already been significant success in the coalition building process. During early development of the initiative, they collected data and evaluated the process of developing, nominating, and selecting zones in order to establish an iterative nature to the process. The Washington State Evaluation Team was successful in identifying and addressing points of tension and confusion through surveys, interviews, and group reflection activities. The team also selected the year 2024 to begin evaluation of HEZ outcomes and impact. At the time of writing, these data are not yet publicly available and it is unclear if funding distribution and programming in the Urban and Rural Zones has started (Washington Department of State, 2023).

Elsewhere, many HEZ initiatives have already begun tracking outputs and outcomes, even attributing significant statewide gains in health equity to implementation of the HEZ model. **Appendix Table A.1.** provides a summary of the available evaluation data from the reviewed state initiatives. It is important to note that the data available are from different years, evaluate different timeframes, and are not standardized by population size or characteristics. Though the data displayed are the most recent outputs and outcomes available publicly, some are from progress reports and evaluations published more than five years ago.

1.2. Health Equity in All Policies: Concept and Literature

Chicago's Office of Health Equity in All Policies (HEiAP) can be thought of as an outgrowth of the more general concept of Health in All Policies (HiAP), which was first introduced in 1978 during the World Health Organization Declaration of Alma-Ata (Rudolph et al., 2013). In recent years, interest in and implementation of this approach has grown steadily across localities in the U.S. and abroad.

In *Health in All Policies: A Guide for State and Local Government*, HiAP is defined as “a collaborative approach to improving the health of all people by incorporating health considerations into decision-making across sectors and policy areas” (Rudolph et al., 2013, p. 5). In other words, HiAP is characterized by its emphasis on the public health sector’s collaboration with non-health entities in order to tackle what we have long understood to be the many complex factors influencing the health of individuals and communities. This approach is strongly linked to the concept of SDoH.

To improve health outcomes and reduce inequities, we must find a way to influence the structural drivers and numerous factors affecting communities as described by SDoH. While public health is a wide-reaching field that often draws from multiple disciplines, it is not feasible for any one sector to address all of these factors alone. Instead, there is a need to recognize and engage with other sectors and agencies already embedded in the policies and processes surrounding key issues affecting health and equity such as housing, transportation, food access, and the environment (Rudolph et al., 2013). Not only does this collaborative approach have the capability to increase impact, but effective and sustainable change is more likely when health objectives are embedded across sectors and agencies (Rudolph et al., 2013; Wernham & Teutsch, 2015).

1.2.1. Key Elements of HiAP

Perhaps a unique aspect of HiAP is the consensus that there is no “right” way to engage in this work (Rudolph et al., 2013). Unlike other frameworks aimed at creating change, there is no one rule book or guide, rather there is an assortment of potential avenues and activities that could be included depending on the needs and wants of those involved in the work. This can be both freeing and challenging for those engaging in HiAP efforts. While it can be a formidable hurdle for those starting out or looking to establish buy-in from their organizations to bring HiAP to their localities, the lack of strict guidelines allows ample space for desired creativity and necessary tailoring (NACCHO, 2017).

There are, however, five key elements that have been identified and described by Rudolph et al. (2013) as essential to HiAP work. These are described in detail below.

The first key element is promoting health and equity (Rudolph et al., 2013). To do this we must begin to incorporate health and equity into all aspects of our own work and advocate for their inclusion in all policies, programs, and processes across government sectors and agencies. In addition, these elements should be introduced into all decision-making processes as an integral consideration with equal weight to other points of deliberation.

The second element is to support collaboration across sectors. As previously discussed, collaboration is vital to HiAP work as it recognizes that health equity is affected by a variety of factors and that those factors are under the purview of many groups both within government and within communities. These groups must be invited to the table and included as partners, building trust and sharing in decision-making, in this work in order for it to be successful. Doing this will also increase skills and resources that can be utilized for these efforts.

Third is to ensure benefits – or co-benefits – for all partners. In line with the collaborative spirit at the core of this work, we should not only seek to address the priorities of the public health sector but create space and opportunity for all involved to incorporate and achieve the goals of their own agencies and organizations around equity.

Fourth is to identify and engage with partners outside of government. While HiAP is often located within government at the state or local level and places emphasis on collaboration with other government sectors, this alone is not enough to create impact. As with all public health work, community involvement is essential to the success of HiAP efforts (Rudolph et al., 2013). Community partners should include everyone from community-based organizations (CBOs) and community members to private-sector partners and funders (Rudolph et al., 2013). As noted above, these partners should have the opportunity to co-benefit from HiAP involvement and include their objectives and goals.

Finally, the fifth element to HiAP as proposed by Rudolph et al. (2013) is to create lasting and sustained change to structures, policies, and procedures in order to support health and equity work going forward. In particular, these changes should be institutionalized within government agencies and departments, creating new ways of doing things that place health and equity as central pillars in all activities and initiatives.

1.2.2. HiAP in Action

NACCHO's *Health in All Policies: Experiences from Local Health Department* describes the overall nature of HiAP implementation across the country so far saying, "While HiAP in the U.S. is still in its formative stages and there are no formal HiAP best practices to-date, key informant and listening session participants noted several promising strategies that can help to develop a strong foundation for practice" (NACCHO, 2017, p. 41).

One of the earliest and most well-known examples of HiAP implementation is the California HiAP Task Force. Created in 2010 via an Executive Order signed by then Governor Arnold Schwarzenegger, the California HiAP Task Force brought together over twenty state government agencies in the first state-level use of HiAP (Rudolph et al., 2013).

Still in existence today, the Task Force has had many recorded successes including (California Strategic Growth Council, 2022):

- Increasing access to local, healthy food in California schools by connecting distributors with local farms through the California Farm-to-Fork Office.
- Increasing safe and healthy transportation options, such as walking, biking, and public transit, in partnership with California Department of Transportation.
- Developing nutrition guidelines for meals served to incarcerated individuals with the Department of General Services and Department of Corrections and Rehabilitation.

To better understand and increase potential for replicating these achievements, in 2012, a formal process evaluation of the Task Force was conducted with support from the Public Health Institute and the American Public Health Association (Rudolph et al., 2013). Through a series of

surveys and interviews with Task Force members and key partners, evaluators were able to establish a list of the top three “elements of success” attributed to the Task Force. These were: (1) politically and financially feasible and actionable recommendations, (2) intersectoral participation, and (3) establishment of a high-level directive (Rudolph et al., 2013).

In addition, 59% of Task Force members noted that HiAP participation had led them to feel greater trust in other state agencies and 54% reported that it increased their collaboration with non-government and CBOs (Rudolph et al., 2013).

In 2016, these findings were affirmed in another survey of government agency staff where nearly two thirds of survey participants felt participation in the Task Force had led to their agency doing more to promote equity in their work (Caplan et al., 2017). Several also shared that they now worked more often with their colleagues in the health sector after their time in the Task Force.

Given the importance of cross-sector collaboration to HiAP initiatives, the findings of these evaluations over time positively show the promise and potential of this approach and should be inspiration for future efforts.

1.2.3. Challenges in Measuring Success

As with its lack of strict guidelines – what NACCHO (2017) calls the “the amorphous and undefined nature of HiAP practice” – there are currently no prescribed evaluation practices for HiAP work. In fact, it is precisely because of the flexibility afforded to each HiAP team in identifying their own objectives and creating their own strategies that it can be difficult to measure success and systematically evaluate efforts across HiAP initiatives. Further, within each HiAP team, limited capacity and resources to design and implement an evaluation serve as additional hurdles to evaluation. These challenges are similar to those faced by HEZ initiatives as previously described and the recommendations for overcoming them are the same in calling for purposeful planning for what and how they will measure success.

While collaborating across sectors is key to this work, it may also create challenges to developing centralized and standardized data collection efforts. To begin with, each sector and agency may or may not collect data, and if they do, they may use their own measures and processes. Without a universal data collection system in place, it may take longer to identify, understand, and interpret findings in ways that are timely or useful for evaluation efforts.⁹

In addition, unlike targeted health outcomes-focused initiatives, it can be difficult to identify tangible short-term outcomes to HiAP work (NACCHO, 2017). As with any wide-reaching effort to improve population health, results will not appear overnight, and in fact, may take years to manifest (Rudolph et al., 2013).

Rather than depend solely on waiting for this long-term health impact data, some HiAPs – including California’s HiAP Task Force as described previously – look to track and record changes in policies and processes as measures of success for their work (California Strategic Growth Council, 2022; NACCHO, 2017). However, it should be noted that measuring this kind

of change can be challenging in itself, and also difficult to unequivocally link to health outcomes, short- or long-term (NACCHO, 2017).

Despite the challenges, ensuring that measures of success and evaluation efforts are put in place for HiAP initiatives and findings are well articulated and disseminated widely is critical not only to HiAP, but all health and equity work (NACCHO, 2017). Without this information, it will only become more difficult to show the value and promise of this work in improving strategies that envision health and equity for all people.

1.3. Healthy Chicago 2025 and the Origins of CDPH's Health Equity Initiatives

In Chicago, Illinois, the third largest city in the U.S., the average white Chicagoan lives about 10 years longer than the average Black Chicagoan (Illinois Department of Public Health, Death Certificate Data Files, 2020). In the face of such large racial gaps in life expectancy, the City health department has considered new approaches to address systemic and structural racism as underlying causes of this inequity. In 2020, CDPH, with support from the Partnership for Healthy Chicago, made up of CDPH and community partners, released the *Healthy Chicago 2025 (HC25)* community health assessment and improvement plan report. As Chicago's current five-year community health improvement plan, the report lays out the following vision for Chicago as:

A city where all people and all communities have power, are free from oppression and are strengthened by equitable access to resources, environments and opportunities that promote optimal health and well-being ([CDPH, 2020](#)).

The guiding principles for this work are antiracism, asset-based, equity-focused, trauma-informed, capital-building, and community-led action ([CDPH, 2020](#)). *HC25* also explicitly emphasizes the need for systems-level change and reshaping power relationships to address health equity, including within the City government's own practices ([CDPH, 2020](#)).

The *HC25* report stipulates actions that support community power and transform systems that oppress people as a means of addressing the root causes of health inequities ([CDPH, 2020](#)). Preliminary work on this plan showed that community members want a role in decision-making, but many experience barriers to participation and some have lost faith that their voices can make a difference ([CDPH, 2020](#)). This, along with the knowledge that participation in issues that affect one's health is a recognized human right (United Nations, 2008), has led CDPH to recognize the need to make it easier for people affected by inequities to become involved in health decision-making and to rebuild trust with community members so that they can feel confident that their solutions are valued by CDPH ([CDPH, 2020](#)).

With the intent to remove the social and institutional barriers that limit community members' ability to make decisions about their own health, *HC25* calls for a hyperlocal systems-change approach – an approach to long-term transformative change that starts by building resources, capacity, and a sense of empowerment in local neighborhoods. To implement this approach,

CDPH created the Healthy Chicago Equity Zones (HCEZ) initiative, led by the CPEZ office, and established the HEiAP office using funding from CDC’s OT21-2103 grant as a part of the *National Initiative to Address COVID-19 Health Disparities Among Populations at High-Risk and Underserved, Including Racial and Ethnic Minority Populations and Rural Communities*. These two offices are charged with carrying out community co-led work, embedding equity into City practices and decision-making, and reducing the health disparities of populations most impacted by COVID-19.

Chicago’s HCEZ initiative was activated during the COVID-19 pandemic when it became clear that some city neighborhoods were not only disproportionately impacted by the pandemic, but also faced challenges to vaccine access and acceptance. To address these issues and support equitable response and recovery efforts, Chicago created the Racial Equity Rapid Response Teams and Protect Chicago/Protect Chicago Plus Programs ([CDPH, 2023b](#)). These programs laid the foundation for the HCEZ approach, building networks of hyperlocal partnerships to tackle community-identified challenges using community-led solutions. Then, the COVID-19 Contact Tracing Corps (Chicago Cook Workforce Partnership, 2024) adopted the HCEZ approach.

Following the success of HCEZs in increasing vaccination uptake among priority communities, CDPH continued its plans to utilize the HCEZ model to confront the social and environmental factors that contribute to ongoing health and racial inequities ([CDPH, 2024b](#)). In 2022, HCEZs conducted community health assessments (CHA) with support from CDPH and their technical assistance partners. The CHAs were intended to identify barriers to health equity that could be addressed using the now established zones ([CDPH, 2023b](#)). The assessments engaged over 500 partners and collected more than 7,000 surveys from Chicagoans across the city, a higher level of civic engagement than any CDPH CHA effort in the past. The results of the CHA identified three top priorities: behavioral health, public safety, and chronic disease ([CDPH, 2023b](#)). Having identified these areas of focus, in 2024 the HCEZs will be launching action plans that represent community-led solutions to the identified health priorities ([CDPH, 2024e](#)).

1.4. Healthy Chicago Equity Zones (HCEZ): Overview of Program and Current Status

The HCEZ initiative is based on a network of hyperlocal partnerships between the CDPH and community organizations working together to advance a key goal of *HC25*: closing the city’s racial life expectancy gap ([CDPH, 2024e](#)). As of August 2023, CDPH has invested \$29 million in HCEZ work, showing their dedication and confidence in this approach for making a positive impact in the lives of Chicagoans ([CDPH, 2024e](#)).

Inspired by the RIHEZ initiative, HCEZs support communities in identifying and finding solutions to the health challenges that most affect them. HCEZs use a hyperlocal lens, focused on defined areas, to support equity-centered and people-powered efforts that champion resilient communities where members make their own decisions on health priorities and utilize local assets to solve health problems ([CDPH, 2024e](#)).

In the city of Chicago, HCEZs are composed of six geographical regions designated as Equity Zones. Together, these regions represent all of the city's 77 distinct community areas ([CDPH, 2023b](#)). Within each Equity Zone, a local network of regional lead and community lead organizations, plus a variety of other local partners, collaborate on efforts intended to strengthen public health infrastructure and support local capacity for implementing effective change.

Each Equity Zone has one identified regional lead organization that is funded by the CDPH. Regional Leads (RL) are tasked with working directly with Community Lead (CL) organizations within their zone, providing funding, administrative and project management support, and other resources. Community Lead organizations in turn engage with local neighborhood partners – including community-based groups, healthcare providers, schools, faith-based organizations, and local businesses – to develop and implement health equity strategies that address their specific needs and challenges ([CDPH, 2024e](#)). This includes being able to create effective culturally and linguistically tailored programs and practices.

Key to this approach is building community capacity ([CDPH, 2023b](#)). By collaborating with organizations already embedded in neighborhoods across the city, HCEZs help communities to develop or strengthen internal skills and resources for collecting and analyzing data, prioritizing health challenges, and developing action plans. Additionally, communities become more equipped to advocate for policy and process changes that are the source of many health and racial inequities.

Other resources resulting from CPEZ efforts so far include an HCEZ Intervention Catalog (developed by a technical assistance consultant) providing example strategies to address health inequities in communities, and an HCEZ Action Planning Tool (also developed by a technical assistance consultant) that RLs can use to develop their HCEZ work ([CDPH, 2023b](#)). CPEZ continues to look for ways to ensure HCEZ sustainability into the future.

Finally, the CPEZ office is working collaboratively with HEiAP's office on shared goals around health and racial equity and achieving the vision of *HC25* by closing these gaps to ensure all Chicagoans have equal access to the resources they need for better health ([CDPH, 2023b](#)).

1.5. CDPH's Health Equity in All Policies (HEiAP): Overview of Program and Current Status

In 2016, Chicago adopted a HiAP model to address health equity in the city, creating a Task Force charged with working together across government sectors and agencies to find ways to improve health for all Chicagoans ([CDPH, 2017](#)). This work was further advanced in 2020 when *HC25* announced its goal to tackle the root causes of health inequity and reduce the racial life expectancy gap ([CDPH, 2024d](#)), and was cemented by the 2021 Mayoral [declaration of racism as a public health crisis](#) in Chicago ([City of Chicago, 2021](#)). To contribute to this goal and the vision of *HC25*, the Office of Health Equity in All Policies (HEiAP) was established with the help of federal grant funding ([CDPH, 2024c](#)). A full timeline of the development of HEiAP can be found on the [HEiAP website](#) and in the program theory below (Figure 2). and in the program theory below (Figure 2).

HEiAP applies the HiAP model while centering health and racial equity as the fundamental components of advancing health. HiAP has always included equity as part of its work – according to NACCHO, “the achievement of health equity is a central tenet for HiAP practice” (NACCHO, 2017, p. 6). However, HEiAP emphasizes the ways in which health and racial inequities are at the root of health disparities and focuses efforts on tackling these inequities to improve health outcomes. Leading this work, the HEiAP office was established in 2022 within the Bureau of Community Health in the CDPH ([CDPH, 2024c](#)).

CDPH defines Chicago’s HEiAP office as “a collaborative approach to improving the health of all people by putting equity at the center of decision-making across policy areas and sectors” ([CDPH, 2024c](#)). As with HiAP, the core of this work is building transformative partnerships and fostering collaboration and shared decision-making that can support community-led strategies and structural changes that address health inequities ([CDPH, 2024c](#)).

Chicago’s HEiAP approach is based around three main strategies as outlined by the CDPH (2023d):

Capacity Building: We normalize health equity framing across sectors and policy areas and build capacity to center health equity. We do this by providing training, technical assistance, and supporting health equity champion development.

Organizing and Mobilizing: We organize transformative community and government partnerships to build, share, and shift power. We do this by supporting community leadership to organize and mobilize cross-sector collaborations to address the social and structural determinants of health.

Policy Research and Technical Assistance: We operationalize health equity in decision making to advance policy and systems change. We do this by providing technical assistance, conducting policy-focused research that evaluates health impacts and inequities, and by amplifying the use of public health data, research, expertise, and framing (CDPH, 2023d, p. 12).

Examples of how these strategies have been implemented in HEiAP activities are described below.

1.5.1. Supporting Community Co-Led Tables

To support cross-sector collaboration, HEiAP has prioritized activities that support not only intergovernmental partnerships, but partnerships between government agencies and communities. These efforts are exemplified by the HEiAP office’s support of community co-led tables in which community members are engaged as partners in developing solutions and making decisions to address health in their communities (CDPH, 2023d).

Elevated Chicago is one example of a community co-led table that is supported by HEiAP. Elevated Chicago is a collaborative that promotes equitable transit-oriented development (ETOD) to ensure all Chicagoans have access to safe, mixed-use, and user-friendly development near public transit hubs ([CDPH, 2023a](#); [City of Chicago, 2022](#)). To inform recommendations for

ETOD policy and implementation, CDPH and the HEiAP office have engaged diverse partners including community members, youth leaders, and the City’s Disability Advisory Committee, as well as government departments, elected officials, and private and civic sector representatives to develop the Health and Racial Equity Impact Assessment ([City of Chicago, 2022](#)). With this support, in 2022, the City Council passed Elevated’s Connected Communities Ordinance, which called for the creation of standards for ETOD near transit ([City of Chicago, 2024a](#)).

Another example is the Chicago Food Equity Council, whose goal is to create a more equitable food system in Chicago ([CDPH, 2023a](#)). The HEiAP office has engaged with the Mayor’s Office and other City departments and agencies to support this co-led table where government and community partners work together to address food access and inequity (CDPH, 2023d; [CDPH, 2023a](#)). HEiAP also provided technical assistance and policy research which helped the Food Equity Council to continue this work via an Executive Order (CDPH, 2023d).

1.5.2. Building an Equity Decision Support Tool

HEiAP defines itself as an approach focused on putting equity at the center of all decision-making ([CDPH, 2024c](#)). But to do this effectively, there remains a need for guidance on how this can happen – both in regard to creating awareness and agreement around a broad concept of health in the city, with its social determinants, that extends beyond the public health department, and with prioritizing equity. To address equity, the HEiAP office has collaborated with partners to develop the Equity Decision Support Tool (EDST) ([CDPH, 2024a](#)). The purpose of this tool is to help local government staff and elected officials work with community-based organizations to include equity in all aspects of their work as well as their everyday decision-making to support the work of community partners ([CDPH, 2023a](#)).

While still in final stages of approval, the EDST will be crucial in HEiAP’s strategy focused on operationalizing health equity, providing capacity building and actionable steps to those looking for ways to ensure health equity becomes a consistent consideration when developing programs, policies, and systems (CDPH, 2023d).

1.5.3. Supporting Racial Equity Action Plans

Health equity cannot exist without racial equity. Just as health inequities naturally lead to a difference in health outcomes, we know that racial inequities impact health in layered and complex ways. This understanding is reflected in the vision of *HC25*, as well as within the work of HEiAP ([CDPH, 2024d](#)).

HEiAP has prioritized the need to address racial equity by partnering with the Mayor’s Office of Equity and Racial Justice (OERJ) to support City government departments in the development of their own Racial Equity Action Plans (REAP) (CDPH, 2023d; [CDPH, 2024a](#)). Through the creation of this multi-year strategic plan, departments must outline how they will face issues of racial equity and improve equitable outcomes across their teams and processes ([City of Chicago, 2024b](#)). To further support this work, HEiAP and OERJ established three Equity & Racial Justice Learning Cohorts to support departments in developing their action plans, identifying priorities, and finding areas for cross-collaboration ([CDPH, 2024c](#); CDPH, 2023d). In addition, HEiAP and

OERJ provided training and technical assistance (TA) that helped to build internal capacity within departments so that they could continue this work into the future, and HEiAP continues to support as reviewers during the City’s Budget Equity Process. ([City of Chicago, 2024b](#)).

1.5.4. Measuring Environmental and Social Cumulative Impacts

Through our understanding of SDoH, it has long been clear that the environment has a significant influence on health outcomes. To truly address health equity, environmental factors and how they relate to health and racial disparities must also be addressed, as well as how they may relate to and compound with other factors.

In order to better understand the current environmental impacts, the CDPH and the Department of Environment worked with community co-leads from the Environmental Equity Working Group (EEWG) to conduct a Cumulative Impact Assessment (CIA), with the goal of identifying neighborhoods that experience unequal exposure to multiple environmental, health, and social stressors and providing recommendations on how to address these inequities ([CDPH, 2024a](#); [City of Chicago, 2023](#)). This was the first assessment of its kind for Chicago; the HEiAP office’s role was to support the community partners in co-designing and co-leading this effort with City staff, centering equity throughout the process.

Through their support of the EEWG in working groups – whose members are community representatives, environmental leaders, and local organizations – HEiAP applied its strategic approach mobilizing cross-sector collaboratives to co-lead and co-design solutions with the communities and neighborhoods most affected by these issues ([City of Chicago, 2023](#); [CDPH, 2024b](#)). Additionally, HEiAP provided policy analysis and project management expertise to center equity throughout the CIA process (CDPH, 2023d; [CDPH, 2024b](#)).

1.5.5. Championing Institutional Change

Institutional change is a key priority across all HEiAP efforts. This type of change is powerful in its ability to spur action and create tangible results, which is why it can and should be considered integral to any health equity work. As the HEiAP office looks for ways to influence practices and proposals that affect equity within government agencies and organizations, and to embed equity into decision-making processes in those places, they consider what types of institutional change may be necessary (HEiAP, 2023).

The HEiAP office has worked to support institutional change in different priority areas. For example, the Systems Change Collaborative, which the HEiAP office supported with the help of IPHI, CDPH Behavioral Health team members, and other community and technical partners, is aimed at improving the health of people experiencing homelessness (CDPH, 2023d). Working together with a large gathering of agencies, organizations, experts, and community members experiencing homelessness, the Systems Change Collaborative defined “housing is healthcare” and developed a list of system-level changes to improve health outcomes for Chicagoans experiencing homelessness (City of Chicago, 2023; Systems Change Collaborative, 2023, p. 7). This list included a broad expanse of recommendations from increasing access to high-quality health services to developing centralized best practices and standards of care.

As stated by CDPH, “Fully implementing these recommendations will require action across partners and levels of government in partnership with CDPH, including affordable and supportive housing systems, homeless response systems, policymakers, behavioral health providers, public health systems, everyday Chicagoans and others” ([CDPH, 2024b](#)).

The work of the Systems Change Collaborative shows the true complexity – and true potential – of institutional change in achieving *HC25* and HEiAP’s shared vision of health equity for all Chicagoans.

2. GENERAL EVALUATION APPROACH

Our team of community-engaged researchers from the University of Illinois Chicago School of Public Health (UIC SPH) was contracted to carry out an evaluation of the initiatives encompassed by the Health Disparities Grant. Like other HEZ evaluations, the scope of our work focused primarily on process evaluation activities. And, through our collaboration with CDPH, our evaluation approach was intentionally aligned with, and built upon our common understanding of, *HC25* principles of antiracism, asset-based, equity-focused, trauma-informed, capital-building, and community-led action.

Our intention was to conduct an evaluation that was formative in nature, investigating the unfolding of the initiative’s implementation and its alignment with the articulated vision in order to provide information to CDPH’s CPEZ and HEiAP offices that will guide improvement of their health equity initiatives going forward. Furthermore, we have striven to incorporate principles of developmental evaluation into our work, aiming for tight integration between evaluators and program staff, and allowing for methodological flexibility. Developmental evaluations are best suited to interventions that are complex and dynamic in nature, where understanding of goals and vision can evolve over time as participants reflect on actions and results (i.e., what works and what is difficult or challenging). This contrasts with traditional formative evaluation which concentrates on fidelity to a model (Patton, 2011). As noted above, the models for national HEZ and HiAP are not set – they are widely under development beyond Chicago and there is not a well-defined set of standards or best practices to measure accomplishments against. Developmental evaluations are flexible in their design and thus best suited to be responsive to the changing needs of a community, and changing perceptions of those needs (Patton, 2011). This is particularly useful for a multi-level community encompassing both the concerns of City leaders and the actors at the local neighborhood and regional levels.

Developmental evaluation necessarily involves participatory evaluation, as the evaluators develop the aims and processes for the evaluation in partnership with their clients, providing expertise but not imposing their own views. This evaluation has incorporated the principles of participatory evaluation, including the establishment of a multi-partner advisory group (AG), which included CDPH staff, contractors, and individuals representing community organizations involved in HCEZ and HEiAP. Participatory evaluation calls for shared leadership and the mutual engagement of all partners and evaluators in the design and implementation of every aspect of the evaluation (Fetterman et al., 2018). The AG’s contributions to the evaluation were

facilitated by the UIC Evaluation Team in consultation with CDPH staff. The main role of the AG was to review our evaluation plans and proposed methodology and offer suggestions related to internal validity, feasibility, and acceptability through the lens of key partner groups. In the spirit of participatory evaluation, the UIC Evaluation Team collaborated with CDPH staff to respond to AG feedback and recommendations. This general arrangement enabled our research design and methods to shift when UIC, CDPH, and our partners mutually agreed that doing so would enhance the overall evaluation.

Our evaluation work was originally planned to begin in August 2022 and continue through the end of May 2024. Our contract with CDPH, however, was not fully executed until January 2023. While we were able to conduct some pre-evaluation planning activities during the period between August-December 2022, we were not fully staffed and able to begin our evaluation work until January 2023.

2.1. Participatory Evaluation: What and Why?

Participatory evaluation is an evaluation approach that centers meaningful partnership between trained evaluators, program leadership and staff, and other invested individuals to produce “action-oriented knowledge” about the program under study (Cousins & Earl, 1992; Garaway, 1995, p. 86). In a participatory evaluation, evaluation experts – in this case UIC – partnered with others – CDPH and AG members – to define the scope of the evaluation, develop research questions, design the study, collect and interpret data, and develop actionable results (Springett, 2017). When our teams at UIC and CDPH first came together to conduct this evaluation, it became evident that a participatory evaluation approach would be most appropriate. This was apparent for three reasons. First, both CDPH and the team of evaluation researchers at UIC are oriented towards community power-sharing as a public health strategy and more participatory and community-engaged research methods. Second, the vast size and complexity of the two health equity initiatives under study meant that UIC could not fully understand their theory of change without close partnership with those responsible for planning and implementing the initiatives. Third, the two initiatives being evaluated were born out of the *HC25* report which was guided by six core principles: antiracism, asset-based, equity-focused, trauma-informed, capital-building, and community-led action ([CDPH, 2020](#)). Given these guiding principles, our evaluation required an approach that embedded community decision-making power into the evaluation work. Participatory evaluation emerged as the perfect methodological fit as it allowed for more equitable working relationships between evaluators (UIC), clients (CDPH), and the program staff, community partners, and community members the initiatives intend to serve.

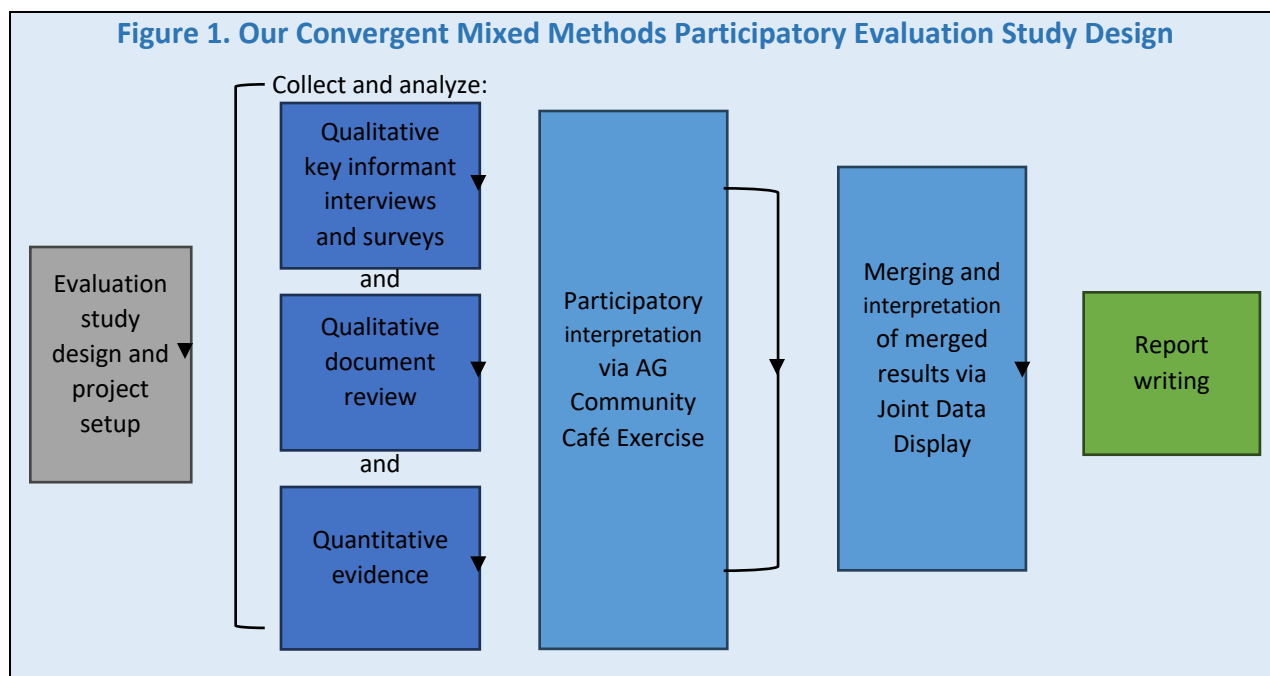
Our mixed methods participatory evaluation began several months after the CPEZ/HCEZ and HEiAP initiatives started. First, we decided on the purpose and scope of the evaluation and formed the AG made up of academic, government, and community partners to provide insight and guidance on all steps of the evaluation process. Next, we developed a program theory to explain how and why the combined HCEZ and HEiAP interventions were expected to address structural racism and a logic model to connect the intervention activities with desired outcomes.

We then finalized a working draft of our program theory and logic model with the AG and co-developed evaluation research questions. We continued to refine our evaluation questions as we analyzed secondary qualitative and quantitative data and collected and analyzed new qualitative data. We engaged in participatory interpretation of preliminary quantitative and qualitative evaluation results with our AG and invited them to share preliminary findings with their networks as well, which helped to support the validity of our interpretations. Finally, our UIC team engaged in mixed methods interpretation to reach more complete and cohesive conclusions. In this report, we communicate our most current answers to the evaluation questions posed.

Each step of our participatory evaluation is described in the sections of this report that follow. For more detail on how we conducted this participatory evaluation, we invite you to read our article in the *Journal of Participatory Research Methods* (Ford et al., 2024). The article was co-authored by evaluation partners at UIC, CDPH, and members of the AG. The published paper is both a work product of our participatory evaluation and a manifestation of the participatory processes that we engaged in during the life course of this project.

2.2. Mixed Methods Approach: Why and How?

We opted to conduct a mixed methods evaluation study because there were both qualitative and quantitative sources of evidence that would best answer the evaluation questions for this project. The data collection, analysis, and interpretation in our participatory evaluation relied on a convergent mixed methods design (see **Figure 1**). A convergent mixed methods design is one where the quantitative and qualitative data are collected and analyzed concurrently and come together – or converge – in the interpretation phase of the project (Creswell & Plano Clark, 2011). A convergent design helps to triangulate results from different methods and provides multiple perspectives on the same topic.



The decision to conduct a convergent mixed methods study design was also a pragmatic one. After developing an AG, program theory, logic model, and defining the evaluation questions, UIC had a limited timeline to report evaluation results for the two complex citywide initiatives and thus opted to divide into teams to simultaneously collect and analyze various sources of available evidence to inform the evaluation questions. One team focused on collecting new qualitative evidence through key informant interviews and open-ended survey responses. Another gathered existing documents and engaged in document review. The third team focused on collecting quantitative evidence from various sources, including monthly reports and surveys. Each team did their own preliminary analysis, began interpreting their results in partnership with the AG, and participated in mixed methods interpretation. The sections that follow provide more detail on the sources of our qualitative, document, and quantitative data.

2.3. UIC Evaluation Team Process

The UIC Evaluation Team has been led by Dr. Benjamin Shaw, Professor and Director of the Division of Community Health Sciences (CHS) at the University of Illinois Chicago School of Public health, and managed by Diana Ghebenei, MPH. The five faculty co-investigators on the team each hold appointments in CHS, and each have lived and/or worked in Chicago for at least eight years. The team has been further supported by multiple graduate students (both MPH and PhD students) and a post-doctoral research fellow. The CHS business manager has provided additional administrative support to the team.

The UIC Evaluation Team's work was carried out through a set of regular weekly working meetings and is facilitated by the file sharing and messaging services of Box and Microsoft Teams. The primary weekly meetings included:

- Project management meetings between the project leader and project manager
- Research assistant meetings between the project manager and the graduate student assistants
- Mixed methods subgroup meetings
- Full team meetings

At various times during the project, the UIC Evaluation Team also broke into smaller working groups (e.g., a “program theory/logic model workgroup”; an “evaluation questions workgroup”, and an “AG engagement workgroup”), with ad hoc meetings of these smaller groups scheduled as needed.

In addition to the regular team meetings, UIC Evaluation Team representatives met weekly with our CDPH partners (see **Appendix Table A.2.** for a roster of the larger UIC-CDPH team), and quarterly with our AG.

3. KEY STEPS OF EVALUATION

3.1. Engagement of Advisory Group

At the outset, we identified the need for an engagement method to assure that the evaluation was grounded in the priorities of the key partners involved. We identified the use of an advisory group (AG) to serve as a rudder for our work. AGs are typically used to assure representation of community perceptions, preferences, and priorities (CDC, 2011; Norris et al., 2007). Our AG was conceived as a body of partners who could guide all steps of the Health Disparities Grant Evaluation. We formed the AG with representatives from the multiple partner groups including UIC, CDPH, and HCEZ and HEiAP partners. AG membership was carefully selected based on a thorough assessment by partners involved, ensuring that members, resources and capacity were carefully evaluated with a focus on equitable representation. We met on a quarterly basis.

Facilitated by our partners in CDPH, we engaged a multi-partner AG (see **Appendix Table A.3**). The AG met virtually with the UIC Evaluation Team on a quarterly basis, with meetings lasting approximately 1.5 hours. These meetings provided the UIC Evaluation Team with an opportunity to give status updates and allowed AG members to offer feedback and suggestions.

During the initial meeting, we used a consensus building technique called the “[Fist to Five](#)” strategy to develop a group contract and engaged the AG in a discussion of their potential roles in the evaluation process (Ford et al., 2024; Lucid, 2024). Establishing a group contract within the AG accomplished several things. First, it helped to clarify the roles, responsibilities, and expectations of each member, ensuring a shared understanding of the project's purpose and objectives. Second, a community contract fostered a sense of ownership and commitment among the group members, promoting their active participation and engagement throughout the evaluation process. Lastly, the contract served as a guiding framework for effective communication, collaboration, and conflict resolution, creating a respectful and inclusive environment that maximized the group's collective expertise and potential.

In engaging the AG about their roles, we felt it was important to discuss not just how the AG might function, but more importantly, why we needed their input. We explained our desire that their roles are not only about collaboration, but more importantly, a significant emphasis was placed on this AG being participatory, encouraging a strong sense of empowerment and involvement in the evaluation process.

Our initial meeting with the AG led to us establishing the following eight-point group contract:

1. Clarifying questions are always welcome.
2. Explain acronyms, when possible; there are many in public health.
3. Step up, step back.
4. Be explicit about inequities, power, and privilege.
5. We will work together to create a space where participants are comfortable asking for what we need.
6. Challenge assumptions.
7. To support each other in our risk-taking, we agree to respect the privacy and confidentiality of this space.
8. Make it a safe space to try new ideas.

Following the initial meeting, we used a brief survey to solicit the AG's input on their potential roles. The potential specific roles highlighted by the AG include:

- Serving as strategic thought partners and advisors
- Providing honest input and feedback including on evaluation design
- Reviewing program impact data
- Providing feedback on HCEZ activities and outcomes
- Supporting RLs and provide context on grant
- Supporting CDPH to implement what is working and adjust where needed
- Contributing to knowledge of authentic and intentional community involvement
- Facilitating information gathering from people who are not on the AG including communicating information to CLs (note: each RL had a representative on the AG)
- Sharing main challenges and solutions regarding regional-specific program implementation/outcomes
- Helping to identify evaluation criteria/design and make suggestions for gap in data collection and reporting tools/processes including adaptation for effectiveness of services
- Gathering perspectives and evidence to strengthen/validate evaluation
- Contributing to interpretation, implementation, and dissemination of evaluation results
- Providing context/local and unique perspective/guidance that is distinct from academic perspective
- Showcasing the success of evaluation through people and trust

The AG was asked for input on the drafts of our logic model, evaluation questions, and proposed methodologies. These drafts were shared in late June and feedback was provided by July 7, 2023.

After the July deadline, meetings in the Fall of 2023 focused on discussing the evaluation questions that would guide the data collection process. Members provided insights on the relevance of the questions, suggested reframing, and identified questions not applicable to their communities. Open dialogues addressed pressing questions from AG members, such as, “How will these approaches truly benefit the community members involved in these initiatives?” Besides providing feedback on our deliverables, some AG members contributed as co-authors on a collaborative manuscript between UIC and CDPH (Ford et al., 2024), discussing the challenges and opportunities of participatory evaluation.

From January 2024 onward, AG meetings focused more on the findings from our data collection process. Each subgroup (e.g., Doc Review, Qualitative, Quantitative) provided updates on their data collection and preliminary findings. AG members reviewed these findings and offered feedback and insights. Many suggested creative ways to format and share the final findings with community members and partners to ensure the information is both intelligible and easily digestible.

Most meetings were conducted virtually, but we held our first in-person meeting on March 11, 2024. This was an opportunity for AG members to meet us in person, build camaraderie, and discuss important matters. In this meeting, we engaged community members in discussions centered around our preliminary findings and receiving their guidance on the interpretation of the data. This was primarily done through a “Community Café” approach (see **Section 3**). We collectively took time to reflect on the findings and the evaluation questions and we discussed the possible meanings and potential implications. AG members were asked to share the findings with their CLs and respective partners within their community network in hopes of receiving even more in-depth feedback and thoughts about the findings at our next meeting in May, which would be our final AG meeting.

The final AG meeting was held on May 20, 2024. In this meeting, we presented our final findings and collective interpretations of the data in relation to the evaluation questions. AG members were asked to reflect on the findings, provide feedback, and answer our team's clarification questions. They discussed achieving consistency and a unified voice when continuing important conversations among partners and CDPH. AG members also provided guidance on drafting the final report and indicated their preferred level of involvement in the reporting process. One suggestion was to share the findings in a town hall event to receive live feedback from community members. Members who could not attend the final meeting would then be invited to share their thoughts and feedback about the findings via email as a way of contributing to the final reporting process.

In keeping with our values of equity, all eligible members of the AG were compensated \$100 for each quarterly meeting attended.

3.2. Development of Program Theory and Logic Model

Our aim was to develop a comprehensive shared understanding of the Health Disparities Grant that: a) integrated CDC requirements with HCEZ and HEiAP strategies and objectives; b)

provided a theoretical framing for how these efforts align to confront social and environmental factors that contribute to racial inequities in health; c) informed the evaluation questions to be addressed; and d) guided our refinement of methods to be used. To achieve this shared understanding, we developed a program theory and logic model primarily by compiling and reviewing informational resources about the Health Disparities Grant provided to us by CDPH and by conducting informal interviews with key CDPH staff and CPEZ/HCEZ and HEiAP representatives.

3.2.1. Distinguishing Between Program Theory and Logic Model

Program theories and logic models are two evaluation tools used to better understand a program. These tools detail the relationships among a program's inputs and outcomes and the mechanisms involved in moving from one to the other. While a program theory and logic model serve a similar purpose, they differ significantly in their approach and together allow a more comprehensive view of a program.

- The goal of a program theory is to provide an explicit explanation of how and why a program's activities are understood to contribute to the results by delineating a series of intermediate steps, or the mechanisms that lead from activities to outcomes. A program theory is not intended to be linear; instead, a program theory is conceptual and “concerned with testing the plausibility of a program's design and establishing the elements that make the program sound or otherwise” (Markiewicz & Patrick, 2016, p. 74).
- The goal of a logic model is to communicate a logical and linear explanation of how a program's specific activities fit together and how they produce the desired outcomes. A logic model identifies the main elements of the program and connects them in a way which describes how they work together to reach the result. In doing so, a logic model links a program's activities and outcomes in a linear format that is more operational (Markiewicz & Patrick, 2016). A logic model is intended to identify “an intentional and sequential progression from a program's actions to its results” (Markiewicz & Patrick, 2016, p. 74).

3.2.2. Development Process: Program Theory

The initial program theory was created using an iterative process that involved reviewing documents provided by CDPH, discussions between the members of the program theory/logic model working group (i.e., a working group within the larger UIC Evaluation Team), and feedback from the UIC Evaluation Team and staff from CDPH.

To begin with, one team member reviewed the CDC grant proposal and drafted a detailed program theory based solely on that document. As the need to create a model that was inclusive of both CPEZ and HEiAP and not limited to outcomes related to COVID-19 became apparent through conversations with the CDPH team, other members of the team joined in. In a meeting between the UIC Evaluation Team and CDPH staff in early January 2023, CDPH shared their preliminary thoughts on the goals, objectives, and outcomes related to CPEZ and HEiAP.

Following that meeting, a subgroup of the UIC Evaluation Team crafted an updated version of the program theory. Multiple documents shared by CDPH were reviewed, including historical documents that laid the groundwork for the initiatives. The subgroup met several times to workshop drafts, asking the question “If this [*activity, process, etc.*] happens, then what is intended to change?” after each stage was defined. After receiving feedback from the full UIC team, a draft version was shared with CDPH ahead of a late February 2023 meeting with the goal of getting input from CDPH, correcting errors, and identifying strengths and weaknesses of the program theory draft. The meeting revealed that while UIC was gaining a better understanding of how and why CPEZ/HCEZ and HEiAP was intended to work, there were some remaining gaps in understanding that could be best filled with CDPH staff knowledge.

In late March 2023, members of the UIC Evaluation Team met with CDPH staff to finalize a working draft of the program theory. In that meeting, UIC and CDPH staff updated the program theory draft; defined what short, intermediate, and long-term outcomes meant in years; and discussed potential short-term outcomes for this work.

The process described above informed the version of the program theory shared in our Year 1 progress report. Since then, a subgroup of the UIC Evaluation Team has worked to incorporate edits from CDPH staff and develop the version of the program theory presented in this report. The primary documents that inform this version of the program theory include: Healthy Chicago 2.0, Health in All Policies Resolution, HiAP Task Force Report, Racism as a Public Health Crisis press release, *HC25*, CDPH’s Draft Logic Model Exercise, CDPH’s Health Disparities Evaluation Frameworks, HEiAP Evaluation, HCEZ Request For Proposal (RFP), Draft 2023 HEiAP Workplan, and HCEZ Scope of Work.

The program theory lays the groundwork for the logic model, in that it captures many of the key activities that were part of both HCEZ and HEiAP initiatives, as well as broad outcomes that were not specifically tied to a single initiative.

3.2.3. Development Process: Logic Model

While we were developing and refining the program theory, the program theory/logic model subgroup began working on the beginnings of the logic model. They began by developing a “cross-walk” between the outcomes identified in the program theory and the activities that made up the HEiAP and HCEZ work, ensuring that each activity could be linked to at least one outcome, and vice-versa. At this point, they also began to identify possible outputs of these activities – or measurable ways to determine that activities had happened and what the scope had been (e.g., counts of trainings held, number of reports published).

Given the iterative nature of this theory-building work, each major development in the logic model resulted in changes being made to the program theory, and vice versa. For example, in the logic model, outcomes became phrased in terms of change that happened as a result of the activities (e.g., “increase in X” or “decrease in Y”), and these changes were reflected in newer drafts of the program theory.

A preliminary draft logic model was shared in a UIC Evaluation Team meeting in May 2023 and a follow-up meeting was scheduled to develop a more complete draft. After refining the outcomes and tentatively classifying them as short-term (i.e., in 1 year), intermediate (i.e., 1-5 years), and long-term (i.e., 5-10 years), the team began to see where there might be overlap between the intermediate and long-term outcomes from the HCEZ and HEiAP initiatives.

Since then, the team has worked through multiple iterations of this logic model. The logic model should be considered a “living” document that can be modified as further information emerges or new perspectives are incorporated.

3.2.4. Overview of Program Theory

The [program theory](#) (Figure 2) that we have explicated describes CDPH’s efforts to improve the living conditions of Chicago residents and to promote health equity in order to advance the *HC25* vision of closing Chicago’s racial life expectancy gap. The CDPH initiatives that are specifically delineated in this program theory are HCEZ and HEiAP.

Importantly, the information in the program theory came from CDPH public reports and internal documents shared with the UIC Evaluation Team. The reports and documents are hyperlinked in the PDF version of the program theory and named in different colored boxes in the long black box at the top. The colored boxes of text in the program theory correspond with the report or document from which the information was sourced. White boxes indicate that the information was gleaned through informal conversations with CDPH staff. This program theory is intended to communicate conceptually to CDPH and the UIC Evaluation Team what CDPH has communicated to others about how these initiatives are intended to function.

In the section below, we describe what this program theory says about how and why HCEZ and HEiAP are understood to lead to advanced health equity and a reduction in the racial life expectancy gap in Chicago.

Program Theory Logic. The team goals and the problem statement which informed this program theory were born out of work that began years ago. As such, the program theory begins with detailing Healthy Chicago 2.0, a 2014 CHA and report developed by CDPH and a coalition of local organizations referred to as the Partnership for Healthy Chicago. The program theory also describes the historical context that laid the groundwork for the HEiAP office: the 2016 Health in All Policies City council resolution, the findings from the 2017 Health in All Policies intergovernmental Task Force report, and the 2021 mayoral declaration of racism as a public health issue.

The strategies that came out of Healthy Chicago 2.0 and the sociopolitical environment in City government at the time served as an important precursor to *HC25*, a 2019 CHA and report. The findings from *HC25* demonstrated that social and institutional barriers must be removed to close Chicago’s life expectancy gap and identified supporting community power and transforming systems that oppress people as actions that would get at the root causes of health inequities.

This key finding informed the goals of this work (i.e., improve systems of care for populations most affected by inequities; further the health and vibrancy of neighborhoods; strengthen community capacity and youth leadership; transform policies and processes to foster anti-racist, multicultural systems) and the following problem statement:

Because Chicago's life expectancy gap is only a symptom of systemic racism in the city, there is a need to address unequal systems and structures, recognizing health equity as both a process and an outcome. This means a focus on the following actions for Black, Latinx, and low-income communities, as well as communities burdened by pollution, disinvestment, and gentrification:

- 1. Being civic trauma-informed, civic love-oriented, and centered on structural care*
- 2. Addressing systemic inequality in political power*
- 3. Addressing income inequality and inequity in material living conditions*
- 4. Building capacity within the City of Chicago's governmental agencies and communities to prepare for the next public health emergency*

This understanding of the problem led to a series of activities for HCEZ and HEiAP. Progress on these activities is made with the oversight, guidance, and recommendations of the Partnership for Healthy Chicago.

CDPH endeavors to create the conditions for structural and institutional change by building community power to organize and lead change efforts via the HCEZ. HCEZ builds community capacity to reach disproportionately affected populations by contracting/working with RLs as delegates of CDPH, and RLs subcontract/coordinate CLs to engage residents impacted by health and racial inequities and adapt and disseminate culturally and linguistically tailored public health messages (see **Appendix Table A.4.** for listing of all RLs and CLs). CLs do work in and with communities most impacted by health and racial inequity and have experience coordinating across the local public health system and leading planning, outreach, education, engagement, and policy change efforts. As such, they are understood to be well-positioned to develop and coordinate hyperlocal neighborhood networks of partners, including other community groups, health providers, schools, faith-based organizations, and businesses that are focused on addressing health and racial equity priorities in their neighborhoods. CDPH supports RLs and CLs in these efforts by identifying existing networks and partners, addressing gaps, and cultivating cross-sectoral collaboration and partnerships. Collectively, these hyperlocal neighborhood networks contribute relevant lived experience and inform CDPH's thinking about key issues, tailoring health communications, outreach, and interventions. RLs and CLs work together with the hyperlocal neighborhood networks to provide training and resources to conduct local health assessments, develop and implement action plans, disseminate health information, evaluate their efforts, and engage in policy and systems change work.

CDPH endeavors to create the conditions for structural and institutional change by preparing City government, including CDPH, to engage in structural change processes co-led with communities most affected by inequities via the HEiAP office. The HEiAP office advances decisions, practices, and policies across City departments and sister agencies to eliminate health

and racial inequities and benefit the health of all Chicagoans through capacity-building, organizing and mobilizing, and engaging in policy and systems change work within City government. The HEiAP office's capacity-building work can be seen in their training and technical assistance efforts across City government. The HEiAP office supports other City departments and sister agencies in conducting and publishing health and racial equity impact assessments, utilizing equity tools, and participating in co-governance projects. Their organizing and mobilizing work can be seen in the cross-sectoral partnerships they build and maintain to address SDoH, including the leadership, technical assistance, and strategic support offered to issue-specific community co-led tables. The HEiAP office's systems change efforts can be seen in the practices, procedures, and protocols that they develop that are intended to result in more equitable outcomes (see **Appendix Table A.5.** for a listing of HEiAP-affiliated staff linked to their area of HEiAP-related work).

These HCEZ and HEiAP activities are expected to lead to a series of outcomes over time (see **Figure 2**). According to this program theory, these collective outcomes will ultimately create conditions that are conducive to hyperlocal networks of neighbors channeling their collective power to work in collaboration with the City to improve their living conditions and make progress in advancing health equity for historically oppressed communities and to help close Chicago's racial life expectancy gap (impact).

3.2.5. Overview of Logic Model

Our draft logic model (**Figure 3:** [Word Version](#), [PPT Version](#)) describes how the core activities carried out through HCEZ and HEiAP, with the support of CDPH, are linked to measurable outputs and short, intermediate, and long-term outcomes that improve the living conditions of Chicago residents and promote health equity in order to help close Chicago's racial life expectancy gap.

Consistent with the program theory, the logic model is broken into components that fall primarily into the HCEZ and HEiAP arenas. We recognize that some activities overlap; however, based on our understanding of the efforts and feedback from the AG, we chose to present the specific activities and their aligned short-term outcomes in two separate tracks that merge with shared intermediate and long-term outcomes. One exception to this is for activities related to sustaining work accomplished in the first 24 months, labeled in a separate track as "Sustainability." We understand that these efforts are likely to be undertaken jointly by HCEZ and HEiAP staff, CDPH, and local organizations and that the specific focus will likely depend on the funding source as well as the community-identified priorities. Finally, we recognize that each activity box includes multiple smaller activities or steps that happen as part of the overall strategy. We selected to focus on the high-level activities for the sake of simplicity in a visual model.

In the section below, we walk through the details of this logic model which communicates how HCEZ, HEiAP, and the sustainability activities align with their desired outcomes.

HCEZ Activities. Below, we summarize the primary activities that are part of the HCEZ efforts, categorizing them into four strategy types.

CDPH-provides resources, guidance, and support to communities. This includes the provision of prevention resources and services, and the facilitation of regional and local network planning. This may also include assistance to regional or community leads that is provided by organizations contracting with CDPH, such as the Public Health Institute of Metropolitan Chicago (PHIMC) and the Illinois Public Health Institute (IPHI). The outputs related to the provision of this TA include descriptive information on the numbers and types of meetings that are held, as well as descriptions of the specific types of resources (e.g., COVID testing and vaccination resources which are highlighted in CDC performance measures 1.1, 1.4, and 1.5) and support (e.g., data office hours) provided. This type of assistance and support should result in the short-term outcome of increased capacity at the regional and local level to prepare for and address those threats to public health that have been identified as important by the community, including but not limited to COVID-19. Here, increased capacity is conceptualized to include both data use capacity (highlighted in CDC performance measure 2.1) and general infrastructure improvements (highlighted in CDC performance measure 3.1).

RL and CL agencies are selected. The outputs of these activities will paint a comprehensive picture of who is directly funded to work on HCEZ-related efforts across the city, including the number and type of organizations that are included in these efforts, and specifically where they are located vis-à-vis the 77 community areas (CAs).

RLs and CLs organize and develop regional and local neighborhood partnerships and networks. Whereas the previous set of activities were focused on recruiting and engaging people and organizations to do the work of the HCEZs, this set of activities refers to the actual work of developing and nurturing those partnerships and networks. Outputs include the numbers of meetings held between partners at the regional and local levels, the structure of partnerships, and the work that comes out of them. We will also count and characterize new hyperlocal networks and partnerships that result from these efforts, and evidence of inter- and intra-regional partnerships. The short-term outcomes resulting from the selection and organization of RLs and CLs are shared. One of the short-term outcomes of this work is to increase the number of new and expanded partnerships that can be mobilized to advance health equity and address SDoH (highlighted in CDC performance measure 1.4). Other short-term outcomes include an increase in the number of neighborhood and regional networks as well as the strengthening of relationships between community organizations at the regional and neighborhood levels.

Neighborhood networks develop and implement local action plans. There are numerous smaller activities that fall into this broader activity, including the collection of data for the local health assessments, the development of action plans, and the implementation of those plans. The multiple activities that fall within this larger activity are represented by outputs, including the number of data collection and consensus-building activities and the types of partners involved in these processes. We also expect to see evidence of rigorous and inclusive approaches to data analysis, and evidence that the action plans are reflective of community input. By doing the work of analyzing data and developing and implementing local action plans, short-term outcomes include an increase in the amount

of hyperlocal data available to planners, the number of action plans that are reflective of community priorities, and the local capacity to implement data collection and analysis efforts and engage in evidence-based local public health planning.

HEiAP Activities. Below, we summarize the primary activities that are part of the HEiAP efforts, categorizing them into three strategy types.

Capacity Building. HEiAP staff, with support from IPHI, provide leadership, TA, training, strategic support, and equity-related tools such as the Equity Decision Support Tool and Racial Equity Action Plans to a variety of partnerships and communities, including the Partnership for Healthy Chicago, City departments, and sister agencies. We propose that these outputs can be quantified as the amount of time and number of events that are spent associated with these activities (e.g., the time and activities that go into conducting and publishing racial equity impact assessments). By engaging in these activities, we anticipate seeing increases in knowledge within City departments and sister agencies about how to center health and racial equity in decision-making, the capacity of community co-led groups (or “tables”) to make decisions using health and racial equity lenses, and CDPH’s own capacity to respond to community-identified health threats.

Organizing. HEiAP staff build and facilitate reciprocal learning relationships within and across multiple arenas – including communities, the Partnership for Healthy Chicago, and other City departments – to advance health and racial equity. Outputs from these efforts can be seen in the form of the time and number of reciprocal learning relationships formed, and in the establishment and continued support of a community accountability committee that can guide CDPH and other systems’ approach to and accountability for their health and racial equity work. By facilitating these partnerships and implementing methods of accountability, we believe that there will be an increase in systems and community members that see themselves as participating in reciprocal learning relationships with the HEiAP office, the number of new and expanded partnerships within the local public health system and within City departments and sister agencies that can be mobilized to advance health equity and address SDoH (highlighted in CDC performance measure 1.4), the number of co-led collaboratives and co-developed products that are focused on health and racial equity, and the number of City departments and sister agencies who champion the HEiAP framework.

Operationalizing. HEiAP staff support the implementation of new practices and structures – such as the development of processes to evaluate how new initiatives promote health equity amongst City departments and sister agencies or the creation of REAPs within City departments – that result in more fair and just health outcomes. The outputs that are seen will depend on the specific initiatives undertaken, but could include the number of: new practices, structures, and policies that are created because of capacity building and organizing efforts; City departments/sister agencies who conduct and publish racial equity impact assessments or action plans; government staff that use the Equity Decision Support Tool; and products that are co-developed and/or shared across the city. The most immediate outcomes from this would likely be increases in health and racial equity impact assessments, use of equity tools, and/or action plans across City

government, the number of co-led collaboratives and co-developed products that are focused on health and racial equity, and sources of accessible and democratized data for use in health and racial equity planning.

Sustainability Activities. As mentioned earlier, we anticipate that sustainability activities will be coordinated across the CPEZ/HCEZ and HEiAP program boundaries. These activities can be summarized in four categories, all of which lead to the outcomes of increased funding to support local health equity work, and ongoing support from CDPH to RLs and CLs to continue this work.

CDPH identifies possible funding sources. We see this as being in the form of an assessment of the funding landscape, with outputs taking the form of its production and distribution to RLs and CLs.

CDPH submits grant applications. These outputs would be counted as the number of grant proposals submitted.

CDPH realigns existing work plans and revenue to support ongoing HEiAP and HCEZ efforts (including utilizing the HCEZ structure to facilitate programs from other offices). This output might be seen in the amount of existing funding that is allocated to HEiAP and HCEZ efforts.

CDPH provides grant writing and similar assistance to RLs and CLs to sustain health equity efforts outside of CDPH contracts. These outputs would be measured in terms of hours and types of TA provided, and the number of health equity-related grants submitted by RLs and CLs agencies

Shared Intermediate and Long-Term Outcomes. In this narrative, we will not restate each of the shared intermediate and long-term outcomes or the ultimate impacts that we see these activities reaching in the years that follow. What we see emerging, however, is a substantial increase in the amount of local community investment in public health and health equity efforts, a broader understanding of the variety of services and efforts that are related to public health, improved trust and communication between CDPH and other City agencies and residents/agencies from local communities, and a more evident integration of health equity and racial equity into the work of all areas of Chicago government.

It is only through these outcomes that we will see improvements to and commitment for services protecting the health of those communities most affected by racism and disinvestment. And, it is through achieving these outcomes that we will see communities themselves with the sustained capacity – and the power – to implement community-driven strategies to address health threats and improve equity.

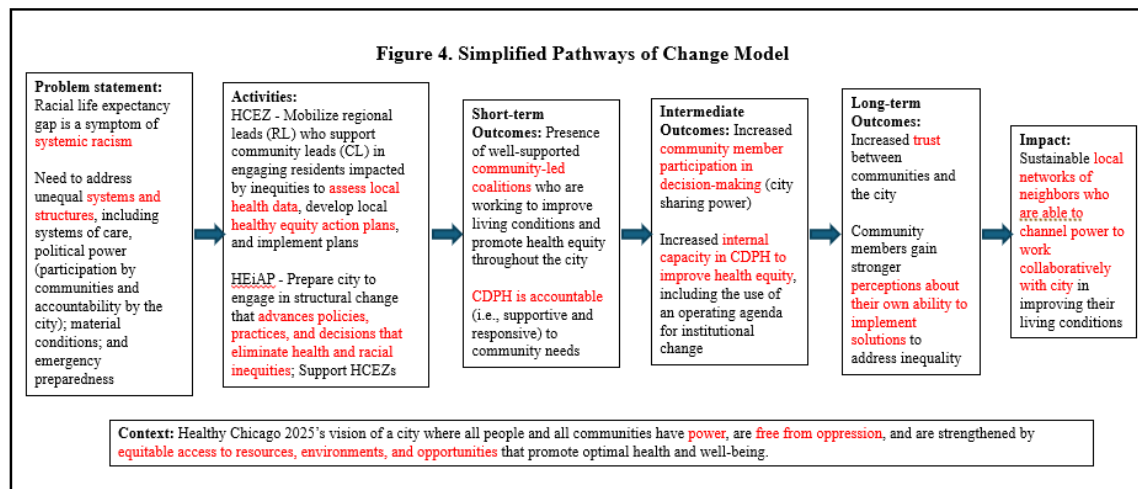
And finally, the long – and lasting – impact of this would be “improved conditions that are conducive to hyperlocal networks of neighbors channeling their collective power to work in collaboration with the City to improve their living conditions and make progress in advancing health equity for historically oppressed communities to advance *HC25*’s mission to close Chicago’s racial life expectancy gap.”

3.3. Co-Development of Evaluation Questions

3.3.1. Generating Initial Draft Questions

Simultaneously to explicating the program theory, our team established a working group charged with drafting an initial set of evaluation questions. To ensure a close connection between our program theory and our evaluation questions, our program theory/logic model working group and our evaluation question working group shared the evolving drafts of their work at least weekly, via team meetings, file sharing, and email exchanges.

An important first step in drafting a set of evaluation questions was to review the draft program theory carefully and generate a more simplified version, which we called a “Simplified Pathways of Change” model (see **Figure 4**). With this simplified model, we sought to highlight the most important, innovative, and potentially impactful elements of the larger program theory. Our assessment of what was most important, innovative, and impactful was based solely on the CDPH resources available to us at the time.



Next, we reviewed this simplified model and realized that three key domains were emphasized throughout our program theory, and we used these domains to organize our initial thinking about evaluation questions. Those domains are:

1. Hyperlocal capacity building¹
2. Power-sharing and CDPH accountability and responsiveness to community²
3. Sustainability³

¹ Capacity building refers to the “development of sustainable skills, resources, and organizational structures in the affected community” (McCloskey et al., 2011).

² Using a continuum of community engagement (Agency for Toxic Substances and Disease Registry, 2022), power-sharing represents activities and outcomes that demonstrate the strongest levels of community engagement, including for example evidence of shared decision-making with community members and groups, the formation of strong partnership structures, and strong bidirectional trust (ATSDR, 2022).

³ Sustainability refers to “the continued use of program components at sufficient intensity for the sustained achievement of desirable program goals and population outcomes” (Shelton, Cooper, & Wiltsey Stirman, 2018).

A set of draft evaluation questions for each of these domains were generated and presented to CDPH on February 24, 2023. Evaluation questions focused on both process and outcome evaluation were included in this initial set. Furthermore, each draft evaluation question was linked to our initial ideas regarding data needs and methods that would be required for answering the question. Initial discussion was guided by a set of questions that we posed, including whether the domains were relevant, and whether the questions – individually or collectively – reflected evaluation priorities.

3.3.2. *Evaluating Questions Matrix*

While our presentation of the initial set of draft evaluation questions was organized around two dimensions (domains of activity and impact; and process vs. outcome evaluation), further discussion with our CDPH partners revealed an important third dimension that captured and distinguished between three key levels of activity and impact. These three levels included:

1. Local communities (e.g., CBOs and community residents)
2. Local public health system (including CDPH)
3. Multiple sectors of City government

With this new understanding, we created a 3x3x2 Evaluation Question Matrix (see **Table 1**) that represented all three constructs (i.e., domain, level, and evaluation type) and attempted to place each of our draft questions in the appropriate cells of this matrix. Where we found gaps in our inventory of questions, we referred to the comprehensive program theory and early versions of our logic model to facilitate the generation of additional questions. We understood this exercise as a way of assessing the breadth of coverage of our draft evaluation questions, recognizing that our final set of high-priority evaluation questions for use in the near term would not necessarily need to address every single cell of this matrix.

Table 1. Three-Dimensional Evaluation Question Matrix						
	Capacity Building		Power-Sharing		Sustainability	
Community	process	outcome	process	outcome	process	outcome
Public Health System	process	outcome	process	outcome	process	outcome
Across Government	process	outcome	process	outcome	process	outcome

3.3.3. *Linking Evaluation Questions with Community Input and Data Needs*

Through discussions with CDPH and our AG, we continued to add to and modify our draft set of evaluation questions. We focused on identifying the evaluation questions that were of the highest priority to CDPH and our AG. As such, we solicited written feedback on the draft evaluation questions from our AG by sharing our draft evaluation questions (see **Appendix Table A.6.**) with the AG virtually, and asking them to respond via email within a two-week period, with a focus on the following prompts:

1. Which questions do you think are most important?

2. Rank the three most important questions in each category.
3. What's missing?

This input helped us to develop an initial proposal containing high-priority evaluation questions.

Linking Evaluation Questions to Data. A key factor in prioritizing our evaluation questions was the feasibility of answering the question with available data. To assess this, we created a Data Inventory document posted on our UIC/CDPH Teams channel that any member of the UIC or CDPH evaluation team could review and modify over time. The Data Inventory consisted of an organized list of available data sources and included hyperlinks to each data source and notes about which data elements in each source were likely to be most useful to our evaluation. The key data sources that were included in the Data Inventory were divided into four main sections:

1. CDPH-generated with ongoing data collection (e.g., Quarterly reports from CPEZ to CDC)
2. CDPH-generated background data (e.g., HCEZ Community Vulnerability Index)
3. Community partner-generated with ongoing data collection (e.g., Monthly Status Reports by HCEZ Region)
4. Community partner-generated background data (e.g., CHA Reports by Region and CA)

Also included in the Data Inventory was a section focused on potential new data sets that could be developed such as: an UIC-generated Database of HCEZ CLs linking CAs to their health priorities and other key variables such as funding tier and CBO typology of each CL; a Partnership Database, including a record for each partner identified in each Partnership Tracking Tool report submitted by RLs, to potentially be used to conduct a network analysis of partnerships; and a Funding Source Tracking Database capturing information about funding sources supporting both HCEZ and HEiAP now and into the future.

Finally, we created an additional section of the Data Inventory that linked each of our draft evaluation questions to a data source documented in the inventory. If a particular evaluation question could not be addressed with currently available data, this showed us that new data collection would likely be necessary to address the question. As we worked to identify the highest priority evaluation questions, we used this Data Inventory tool to help assess the feasibility of addressing each question, based on the availability of data and/or the time and resources needed to collect new data. This process helped to ensure that we would be pursuing evaluation questions that were both high-priority and feasible within our existing time and resource constraints.

Following these processes, we arrived at the following final list of seven evaluation questions (see **Table 2**).

Table 2. Final Evaluation Questions	
1.	With guidance and support from CDPH, what new skills and resources (i.e. capacity) have been developed within community-based organizations (CBOs) involved in the HCEZ initiative? Have there been inequities in the transmission of support or the development of capacity? How are CBOs integrating newly developed skills or resources into their work?
2.	In what ways has the capacity for improving health equity around community-identified health priorities increased within CDPH and other City agencies and departments?
3.	What interorganizational partnerships have been developed or have strengthened with support of the HCEZ or HEiAP initiatives (e.g., among orgs in same community/region, between orgs in different communities/regions, and between community and City)? What have been key barriers and facilitators to partnership or relationship building and reaching shared leadership?
4.	What are key factors that have influenced – both positively and negatively – the development of shared leadership and trust between organizations involved in the HCEZ or HEiAP initiatives (e.g., between individual CBOs; between CBOs and CDPH or the City in general)?
5.	To what extent do key partners – including community organizations, representatives from the local public health system, and City departments and sister agencies – have shared goals around health equity? To what extent have these partners been learning from one another (i.e., engaged in reciprocal learning)?
6.	Is there evidence across the public health system of changing practice to address health equity? What systems changes are being worked on or have been accomplished as a result of the CDPH health equity initiatives?
7.	What do communities, CDPH, and other City departments or agencies feel they need to continue progress in addressing health equity?

3.4. Data Collection and Compilation

For this evaluation we collected and analyzed new data and also analyzed existing data provided to us by CDPH. While some of these data were collected from individuals, the unit of analysis for this evaluation is the organization in that every piece of data that we collected or analyzed is meant to represent an organization, not an individual. As such, the UIC Institutional Review Board determined this work to be non-research.

3.4.1. Key Informant Interviews

Key informant interviews are conversations between a researcher (i.e., the interviewer) and a participant (i.e., the key informant) that are guided by questions related to a study topic (Saldaña & Omasta, 2022). For this work, interview data was collected to inform our understanding of the implementation, ongoing processes, and immediate outcomes associated with the HCEZ and

HEiAP initiatives and intended to supplement the qualitative documents and quantitative evidence to be described in the next sections.

We used a sampling strategy for key informant interviews that fit within our project timeline. This involved selecting a subset of key informants who could offer insights into distinct aspects of the citywide health equity efforts. We conducted 16 individual and 6 group interviews (a total of 32 individuals were engaged in interviews). Participants were purposively selected, which means that they were selected to participate in an interview based on their expertise with one or both initiatives and/or their role in health equity-related efforts within the City of Chicago. Our interview participants had varying degrees of knowledge about the specific internal functioning of the initiatives. The specific participant categories included: (1) representatives of community organizations who are implementing the health equity initiatives (i.e., RLs and CLs), and (2) CDPH staff involved in implementing or leading these initiatives.

We developed tailored interview guides for each participant category, guided by the evaluation questions that frame this project. Prior to beginning the interview, UIC assured participants that the interviews were voluntary and anonymous, and participants gave their verbal consent to be interviewed. Interviews were conducted with audio recorded on Zoom and transcribed using Rev transcription services. The transcripts were cleaned to ensure accuracy. Audio recordings and cleaned transcripts were uploaded into password-protected UIC Box folders. The interviews ranged from 30 to 90 minutes. Participants who did not work for the City of Chicago were compensated \$75 for their time spent conducting the interview.

3.4.2. HEiAP Documentation

Part of the evaluation process involved a review of HEiAP-related documents. This section describes all the documents provided to us by HEiAP officials to help support and further inform our understanding of the initiative and its potential impact.

HEiAP Presentations and Workplan. The HEiAP presentations and workplan are introductory materials with an overview of the apparatus of HEiAP. It covers the primary objectives of HEiAP, which include addressing priority areas such as housing, institutional change, environment, food, and neighborhood planning and development through the creation of tools such as the EDST and Race Equity and Health Impact Assessments. It describes their proposed strategies for capacity building; organizing and mobilizing community while impacting policy; and researching and providing TA to operationalize health equity in decision-making related to policy and systems change.

The workplan is a working draft for the office that discusses the plan for HEiAP in great detail to accomplish its objectives in a step-by-step format. It describes its proposed deliverables, plans, and activities to engage community partners in strategies that develop capacity and increase cross-sector partnerships/collaborations.

Community Co-Led Table Documents. The HEiAP office supports several community co-led tables. We learned about this work mostly through HEiAP presentations. One key product created from this work was the Equitable Transit Oriented Development (ETOD) Health and

Racial Equity Impact Assessment (HREIA). This document was a product of listening sessions conducted with community partners to identify ways to advance racial equity, build community wealth, improve climate resilience and achieve other public health goals. HREIA looks at three ETOD policy components and a critical companion strategy. The findings of this assessment led to the provision of 10 recommendations for ETOD Policy, including to: (1) require on-site affordability for developers to access TOD density bonuses and incentives; (2) incentivize small and local businesses to locate within ETOD areas; and (3) put in place pedestrian, bike, and ADA compliant and wheelchair-friendly design guidelines within ETOD areas to promote safety for sidewalk and transit users.

Cumulative Impact Assessment Documents. Cumulative impact is defined as the aggregated exposure to pollution through air, water, and land that – supplemented by health conditions and social factors – can affect individuals and communities in both positive and negative ways. HEiAP shared with us various documents that describe their contributions to the first Cumulative Impact Assessment (CIA) for the City. The CIA is made up of several documents, including:

- *Chicago CIA 2023 Summary Report* – This summary report provides an overview of the CIA goals, process, findings and recommendations, and next steps. The CIA was a collaborative effort with CDPH, the Department of Environment, and community partners from the Environmental Equity Working Group. The report provides data on how environmental burdens and other stressors vary in impact across the city.
- *City of Chicago 2023 Environmental Justice (EJ) Action Plan* – The EJ Action Plans are City department's actionable policy changes that can be implemented now based on current municipal codes and authorities. Departments created these actionable plans/commitments with their respective teams guided by OCEE and CDPH. These strategies represent improvements to their departments policies, processes, and practices to promote environmental justice. The EJ Action Plans are living documents that will be regularly updated by City departments to reflect ongoing community input, policy and technology advancements and data collection efforts. OCEE and CDPH collaborated to create a template that each department would use to outline their strategy. The template adapted elements and guiding questions from Chicago REAP, and EDST, including questions to help departments evaluate benefits and burdens.
- *Community Input Provided During Assessment: Chicago Cumulative Impact Assessment* – This document describes the various methods that were used to engage communities for input and feedback on the CIA. From April-August 2023, community input was collected through a range of events and mechanisms such as town halls, listening sessions, tabling sessions, survey distribution, and email. This ensured the work was truly informed and guided by community members.

Racial Equity Action Plans (REAP) Documents. REAPs are multi-year strategic plans that describe how a City department endeavors “to improve equitable outcomes in their core work (service-delivery, policy, programs, finance, HR, etc.)” ([City of Chicago, 2024b](#)). A total of 28 City departments and offices completed REAPs and they are available on the City website: ([Racial Equity Action Plans \(chicago.gov\)](#)). Also available on the City website is a description of the REAP methodology and a summary of priority areas chosen across multiple departments and offices. The REAPs do not describe the contributions that CDPH made to this work.

Equity Decision Support Tool Documents. A toolkit designed to help City employees and elected officials work together with CBOs and advocates to center equity in government decision-making. The EDST can be used in whole or in part, depending upon the needs. Each component provides a fillable table to guide thinking and help maximize decision-making. This is a working draft.

In addition to these documents, we also examined the 2023 [Report on the Recommendations of the Systems Change Collaborative to Improve the Health of People Experiencing Homelessness](#) as an example of the institutional change work supported by HEiAP ([CDPH, 2023e](#)).

3.4.3. HCEZ Community Health Assessments and Action Plans

In addition to interviews, another important source of data for understanding the work of the HCEZ RLs and CLs were their community health assessment (CHA) reports and their action plans. CDPH provided the UIC team with access to each region's CHA reports. The formatting and methodology of the CHAs varied by region, with most consisting of a written report of approximately 20 pages. We also reviewed the brief summaries of each CHA that was conducted at the CL level in the HCEZ Playbook ([CDPH, 2023c](#)), a published resource that summarizes the community-led milestones achieved by HCEZ RLs and CLs through April 2023.

As of the close of our contract (May 31, 2024), RLs and CLs are in the process of finalizing their action plans. Therefore, a comprehensive review of these action plans is beyond the scope of this evaluation. However, CDPH has shared a summary document that describes some key aspects of the 30 action plans that have thus far been submitted and we have used this summary to gain a general understanding of the types of interventions that are being proposed.

3.4.4. HCEZ Monthly Reports/CDC Quarterly Reporting

HCEZ Regional Lead Monthly Reports. As part of their contracts, HCEZ RLs were responsible for submitting monthly reports to CDPH documenting progress on their scopes of work. This reporting started in June 2023, with RLs reporting these measures on behalf of both their own organization and CLs in their region. RLs initially sent their monthly reports to their Project Coordinators (PCs) who sent them to the Program Analysts. Beginning in early 2024, RLs were asked to email their reports directly to Program Analysts (PAs). This shift was prompted by feedback from RLs through multiple meetings with the PAs, and helped to show RLs the importance of the reports, and thereby increase compliance and responsiveness. The PAs also began sending reminders to RLs approximately one week before a report was due. They also followed up with RLs who had not submitted their report approximately one week after a report was due. After reviewing these reports, the PAs were able to provide the RLs with recommendations for reporting improvements.

In the monthly reports, RLs provided quantitative updates with respect to several key performance measures associated with four main strategies:

- *Strategy 1* – Expand existing and/or develop new mitigation and prevention resources and services to reduce COVID-19-related disparities among populations that are at higher risk and underserved.
 - Performance measure 1.1: Number of COVID-19 mitigation and prevention resources and services delivered in support of populations that are underserved and disproportionately affected by type.
- *Strategy 2* – Increase/improve data collection and reporting for populations experiencing a disproportionate burden of COVID-19 infection, severe illness, and death in order to guide the response to the COVID-19 pandemic.
 - Performance measure 2.1: Number of improvements to data collection, quality, and reporting capacity for recipients, partners, and agencies related to COVID-19 health disparities and inequities.
- *Strategy 3* – Build, leverage, and expand infrastructure support for COVID-19 prevention and control among populations that are at higher risk and underserved.
 - Performance measure 3.1: Number of improvements to infrastructure to address COVID-19 health disparities and inequities.
- *Strategy 4* – Mobilize partners and collaborators to advance health equity and address SDoH related to COVID-19 health disparities among populations that are at higher risk and underserved.
 - Performance measure 4.1: Number and proportion of new, expanded, or existing partnerships mobilized to address COVID-19 health disparities and inequities.

Each region utilized a common data dictionary to collect this performance measure data from their CL partners. This data dictionary included the CDC description of each performance measure, as well as additional examples of how to count different measures and what may be included in each. CDPH did not require that each example of a performance measure improvement be reported. Instead, they simply asked that RLs reported the aggregate quantitative counts of each performance measure that was collected for their entire region.

RLs also provided narrative updates on their work through two broad questions about their successes and challenges, as well as additional questions derived from their scope of work. While these narrative sections were focused on RL-specific scopes of work, RLs were encouraged to highlight examples of CL work where relevant. The UIC Evaluation Team reviewed the qualitative content of these narrative updates for information relevant to the evaluation questions (described below).

CDC Quarterly Reporting. A requirement of the Health Disparities Grant was for CDPH (CPEZ and HEiAP) to report their progress to CDC on a quarterly basis. These quarterly reports were required to include data on the specific performance measures being tracked by RLs and CLs. Each quarterly report to CDC also allowed for the inclusion of narrative sections – referred to as Workplan Progress Reports – that provided context and nuance to the quantitative performance measures reported.

3.4.5. HCEZ Community Lead (CL) Survey

In January 2024, the UIC Evaluation Team, in partnership with CDPH, developed a Qualtrics survey that was distributed to 42 HCEZ CLs, including 9 Far South CLs, 5 Near South CLs, 7 Southwest CLs, 7 West CLs, 7 Northwest CLs, and 7 North/Central CLs. The CL Survey asked key questions that align with our overall evaluation questions, including five open-ended and five closed-ended questions. The survey was structured so that an open-ended question was followed by a closed-ended question related to the topic area. For the closed-ended questions, participants were asked to rate their level of agreement with the statement on a Likert scale where response options included *Strongly Disagree*, *Somewhat Disagree*, *Somewhat Agree*, and *Strongly Agree*. The list of questions is as follows:

- *Question 1 (open-ended)* – In a couple of sentences, please describe one or two ways in which your participation with the Healthy Chicago Equity Zones (HCEZ) initiative has impacted your ability to serve your community. For example, did you or people in your organization learn new skills or tools that help you do your job better? If so, what were they?
- *Question 2 (closed-ended)* – People in my organization have developed useful new skills through our involvement in the Healthy Chicago Equity Zones (HCEZ) initiative.
- *Question 3 (open-ended)* – How has your work with the Healthy Chicago Equity Zones (HCEZ) initiative impacted relationships between your organization and other organizations or individuals? For example, do you feel like your organization works more closely (or less closely) with others because of the work you have done as an HCEZ community lead?
- *Question 4 (closed-ended)* – My organization, my regional lead, and CDPH are working collaboratively towards shared goals related to health equity.
- *Question 5 (open-ended)* – How would you describe the level of trust between your organization and your regional lead organization, and between your organization and CDPH? Did your work with the Healthy Chicago Equity Zones (HCEZ) initiative increase or decrease this level of trust?
- *Question 6 (closed-ended)* – As a result of our work with the Healthy Chicago Equity Zones (HCEZ) initiative, I believe there is a greater level of mutual trust between my organization and CDPH.
- *Question 7 (open-ended)* – Do you have any current recommendations for the regional leads or CDPH that could improve the work of the Healthy Chicago Equity Zones (HCEZ) initiative or your community's participation in it?
- *Question 8 (closed-ended)* – I am satisfied with current progress in the Healthy Chicago Equity Zones (HCEZ) initiative.
- *Question 9 (open-ended)* – Is there anything else that you would like to share about your experience working in the Healthy Chicago Equity Zones (HCEZ) initiative?
- *Question 10 (closed-ended)* – I am confident that my organization will be able to sustain our health equity work that has been supported by the Healthy Chicago Equity Zones (HCEZ) initiative.

The CL Survey was open for approximately three weeks – from January 19, 2024 to February 8, 2024 – and three reminder emails were sent over this time period. A \$20 gift card was provided

as incentive for those who completed the survey. While 35 CLs began the survey, only 33 provided responses to at least one question; therefore, the two respondents who began the survey but did not answer at least one question were excluded from analysis. Included in the 33 respondents were 9 Far South CLs, 3 Near South CLs, 5 Southwest CLs, 5 West CLs, 5 Northwest CLs, and 6 North/Central CLs.

3.5. Mixed Methods Analytic Approaches

3.5.1. Qualitative Analysis of Interviews

All 22 cleaned interview transcripts were uploaded into Dedoose, a qualitative analysis software approved for use by UIC IRB. Dedoose aided in our ability to manage and analyze the data. Our qualitative analysis of the interview data included both deductive and inductive approaches.

Deductive Analysis. Prior to beginning analysis, the UIC team developed a codebook that identified the main codes we expected to emerge from our data. This codebook was informed by our interview guide and evaluation questions. We developed several main codes, such as partnerships and capacity building, along with several sub-codes like barriers and facilitators.

Inductive Analysis. Each research assistant in our subgroup was trained to identify any additional codes that emerged from the transcripts that were not part of our deductive process. Research assistants created notes (i.e., memos) in Dedoose to facilitate discussion and reach consensus amongst the subgroup on the emergent codes and their definitions. Research assistants then returned to any previously coded transcript to apply the emergent codes. An audit trail of all coding decisions was maintained in Dedoose during our qualitative analysis process.

Before coding our full sample, we established reliability between coders (i.e., different subgroup members all coding in the same way). To do this, each team member independently coded one transcript. We then met as a team to review that same transcript line by line. In this meeting, we reviewed the applied codes and refined code definitions via a consensus discussion on any areas of disagreement. The remaining transcripts were divided among the team and coded independently. Throughout our coding process, we used memos to annotate any areas of uncertainty. These memos were discussed, and discrepancies addressed, in weekly analysis meetings.

After all transcripts were coded, we created summary reports of each code in the form of documents which summarized findings from the text segments attached to each specific code. These summary reports informed our preliminary qualitative interview findings and our subgroup's contributions to answering the evaluation questions. The qualitative interview results communicated in this report reflect multiple rounds of analysis and interpretation with team members within our UIC subgroup, across our entire UIC Evaluation Team, at CDPH, and with our AG. All interview findings in this report are reported in aggregate form only and direct quotes are only shared when the anonymity of the participant can be maintained.

3.5.2. Document Review of CPEZ and HEiAP Reports and Presentations

Qualitative analysis of documents provides valuable sources of information for the evaluation of programs or initiatives that can complement data from interviews and other sources (see Bowen, 2009 on value of documents for qualitative research, Patton, 2014 on use of qualitative analysis of documents in evaluation). Program or initiative documents are not co-produced by an interviewer and therefore can provide data less biased by the desires of the evaluator. They are less intrusive data sources as they don't require the time of participants like focus groups or interviews. However, available documents may not be rich sources of available information for specified evaluation questions. We reviewed available documents from CDPH (see **Section 3.4**) for information pertinent to our evaluation questions and found that the HCEZ Monthly Update Reports from RLs were rich sources of information, including some details about what was happening at CA levels that was otherwise inaccessible to us. Due to this, we concentrated on those reports as data sources.

Our procedures for document analysis followed the processes of “data condensation” outlined in Miles et al. (2020). After a preliminary review of the reports' contents for relevance to our evaluation questions, we sent up an initial matrix in Excel with categories of information to be extracted relevant to those questions (see **Table 3**). Two research assistants assigned to the document review subgroup reviewed the monthly reports and summarized the information in the Excel spreadsheet, organized by these categories, the month of each report, and the region reporting. For the final two quarters of 2023, data was shared in quarterly reports. The research assistants discussed the data among themselves, and their work was reviewed by faculty members on the document review team. The data was further summarized in a more general matrix across all the regions, organized by topic as relevant to the evaluation questions where document-derived data was useful. It was then pulled into the matrix to be discussed by the evaluation team, which covered the evaluation questions and all data sources.

North	Quarter or month in 2023	New skills & resources within communities (examples of activities reported)	Comments relating to evidence of increase in capacity around improving health equity through strengthening public health network (see HCEZ network)	Other comments relating to evidence of increase in capacity around improving health equity	Partnerships - comments relating to developing, strengthening, or maintaining partnerships	(concrete) Other outcomes showing increased capacity - quantitative (from performance measures)	(concrete) Other outcomes showing increased capacity - qualitative (e.g. comments on performance measures)	Challenges	Summary	Reviewer
	Q3 (June-Sept)									
	Oct									
	Nov									

3.5.3. Quantitative Analysis of Performance Measure and Survey Data

Our analysis of the quantitative performance measure data, and the data from the CL Survey, relied on descriptive statistics. For example, because much of the performance measure data

were in the form of counts (e.g., number of new partnerships), we used frequency distributions to compare different types of performance measures with one another and across regions. Data from the CL Survey were in the form of responses to questions on the Likert scale. With these data we were able to compute means (i.e., a measure of central tendency) and standard deviations (i.e., a measure of variability) to help find the average response across all respondents as well as understand the extent to which responses varied from this average response.

3.6. Approaches to Interpreting of Findings

3.6.1. Advisory Group Community Café Exercise

One interactive approach we took with the AG to discuss and interpret our preliminary findings was through an interactive “Community Café” exercise. Community Cafés are asset-based community development community engagement approaches which use discourse with broad kinds of partners to make meaning of emergent issues (Aldred, 2011; McKnight, 2017). Discussion is facilitated by pre-determined prompts and iterative discussion groups, building on ideas that are shared in real time in response to synthesized information.

During our AG Café, food and beverages were provided and a relaxed, casual atmosphere was created to set the tone for the meeting. The primary goal of this exercise was to engage the AG members in a discussion about the emergent findings, presented by the UIC team, and guided by two key interpretive questions: “What does it all mean?” and “What do we still need to know?”

The AG Community Café had two components and participants could choose to participate in one or both:

1. *Group Discussion Tables* – Participants chose one of two tables to discuss the findings that had been presented. They had eight minutes per table to reflect and share their responses (which were recorded on poster boards) before moving to the next table.
2. *Individual Reflection* – Participants who opted out of group discussions could reflect individually. They shared their responses to the questions on sticky notes and placed them on flipchart paper.

At the end of the meeting, those who participated in group discussions could also provide individual reflections on the flipcharts, after reviewing them in a gallery walk style. AG members were then asked to share the meeting’s findings and content with their community members and networks. The discussion-based results were shared with AG members for additional reflection and feedback.

3.6.2. Joint Data Display and Review

A defining aspect of mixed methods study designs is the integration of qualitative and quantitative sources of evidence. In a convergent mixed methods design, the data are brought together after analysis to answer the question: “To what extent do the qualitative and quantitative results converge?” (Creswell & Plano Clark, 2011). This final analytic step of merging and

interpreting data (see **Figure 1**) requires researchers to engage in a strategy to make merged data comparisons, of which there are several.

Our team chose to develop a joint data display table to compare our results and develop mixed methods interpretations (see **Table 4**). A joint data display is a figure or table in which qualitative and quantitative data are brought together to support direct comparisons across specified dimensions (Creswell & Plano Clark, 2011). We used our evaluation questions as the dimensions to organize and compare our qualitative and quantitative data and preliminary results. Once the framework for the joint data display table was created, each subgroup (qualitative, quantitative, document review) added their data and preliminary interpretation for each evaluation question to the column associated with their data sources in a shared document.

Table 4. Example of Joint Display Table					
	Evaluation Question	Interview & Survey Data	Quantitative Data	Document Review Data	WORKING Integration/ Interpretation
1	With guidance and support from CDPH, what new skills and resources (i.e. capacity) have been developed within community partners involved in the HCEZ initiative?				
1a	Have there been inequities in the transmission of support or the development of capacity?				
1b	How are community partners integrating newly developed skills or resources into their work?				
2	In what ways has the capacity for improving health equity around community-identified health priorities increased within CDPH and other City agencies and departments?				
3	What interorganizational partnerships have been developed or have strengthened with support of the HCEZ or HEiAP initiatives?				
3a	What have been key barriers and facilitators to partnership or relationship building and reaching shared leadership?				
4	What are key factors that have influenced – both positively and negatively – the development of shared leadership and trust between organizations involved in the HCEZ or HEiAP initiatives (e.g., between individual CBOs; between CBOs and CDPH or the City on general)?				
5	To what extent do key partners – including community organizations, representatives from the local public health system, and City departments and sister agencies – have shared goals around health equity?				
5a	To what extent have these partners been learning from one another (i.e., engaged in reciprocal learning)?				
6	Is there evidence across the public health system of changing practice to address health equity? What systems changes are being worked on (or have been accomplished) as a result of the CDPH health equity initiatives?				
7	What do communities, CDPH, and other City departments or agencies feel they need to continue progress in addressing health equity?				

The joint data display allowed our team to view our evidence side-by-side. Our next step was to interpret these combined results to assess how our mixed methods analysis answered the research questions. The final column (WORKING Integration/Interpretation) was initially left blank by each of the subgroups so that it could be completed in a synchronous virtual meeting with the entire UIC Evaluation Team.

When we came together to complete the final column, we began with silent reading time. Each subgroup familiarized themselves with the preliminary conclusions, or “answers” to the evaluation questions, from the other two groups. Before we began a deeper discussion about our mixed methods conclusions, we paused for reflection. Several members of the team shared excitement and surprise at how congruent our findings were across data sources. We felt that our data flowed well – from qualitative interview data, which was supported by quantitative evidence, to document review data, which provided confirmation and detail to our interpretations. In this way, we were able to integrate the data via connecting and embedding, two of the most common ways to mix findings (Fetters et al., 2013; Meissner et al., 2011).

After general reflections, we began the process of going question by question to develop interpretations that all team members in all three subgroups agreed with and had the data to support. For each question, one person volunteered to tell the story they thought the evidence was communicating. Other team members added or edited until we reached consensus. When points of confusion arose during this discussion, we noted action items that would allow for clarification and confirmation of our conclusions. For example, member checking with CDPH or re-reading a document or interview transcript. This question-by-question discussion and interpretation process took two virtual meetings and resulted in the findings communicated in the next section.

4. INTEGRATED FINDINGS AND INTERPRETATIONS

In the section that follows, we present our interpretations of the findings that have emerged from our mixed methods analyses of multiple data sources collected and compiled to address each of our evaluation questions. To begin this section, we first present a brief summary (see **Table 5**) of findings as they relate to each of the evaluation questions developed in partnership with CDPH. Following this, we present a more expansive look at our results and interpretations that support our findings.

Table 5. Summary Table of Key Findings by Evaluation Question		
	Evaluation Question	Summary of Key Findings Sources: I (interviews), D (project documents, from qualitative and/or quantitative analysis), S (Community Lead Survey)
1	With guidance and support from CDPH, what new skills and resources (i.e. capacity) have been developed within community partners involved in the HCEZ initiative?	All regions agree or strongly agree that they developed new skills (S). New skills reported from CLs included grant writing, data collection and analysis for the community health assessments, and event planning (D). Some RLs spoke highly about the community resources, tools, and TA provided to them by CDPH and/or independent contractors. CLs learned new skills from the RLs (I).
1a	Have there been inequities in the transmission of support or the development of capacity?	Some regional variation but no evidence of large-scale systematic inequities between regions; some interview or survey results indicated perceived differences within N and W regions (I, S). Technical assistance provided to RLs seen by some as fragmented, unhelpful, or inappropriate (I). At CA level, and in relationship between RLs and CLs, barriers to equity included: challenges in managing fiscal requirements and transitions, delays in payment, need for multilingual information, social conditions and communication barriers in certain neighborhoods. (S, D)
1b	How are community partners integrating newly developed skills or resources into their work?	Previous narrow scope of work of some RLs & CLs broadened to include health equity. Improvements in data capacity, multisector coordination, and trainings. Training in data collection and analysis especially helpful for CLs, enabling them to better respond to community need going forward. (I, S, D)
2	In what ways has the capacity for improving health equity around community-identified health priorities increased within CDPH and other City agencies and departments?	Development of the Equity Decision Support Tool increased the capacity to conceptualize health equity, but the limited staffing and resources given to the HEiAP office were barriers to effective implementation of the tool. Increased capacity of the CLs and RLs to supply data to CDPH increases the potential for response to community need going forward (I, D).
3	What interorganizational partnerships have been developed or have strengthened with support of the HCEZ or HEiAP initiatives?	Despite major challenges in staffing, HEiAP office extended partnerships across City departments and built new partnerships through co-led community tables, including organizations/ departments focused on transportation, environment, food equity, etc. (I, D). HCEZ reported over 700 new partners across a variety of sectors, not just clinical health or health promotion (D).
3a	What have been key barriers and facilitators to partnership or relationship building and reaching shared leadership?	CDPH's slow contracting process was a core barrier to relationship building and reaching shared leadership with RLs and CLs, and the root cause of many other barriers (e.g. trust, staff turnover). Lack of resources and staff have been major barriers in HEiAP. Trust, individual personal connections, and shared organizational missions facilitate relationship building and reaching shared leadership. (I, S, D)
4	What are key factors that have influenced – both positively and negatively – the development of shared leadership and trust between organizations involved in the HCEZ or HEiAP initiatives (e.g., between individual CBOs; between CBOs and CDPH or the City on general)?	Additional factors negatively influencing shared leadership and trust include: hierarchical, top-down nature of organization of HCEZ, where CLs had limited direct contact with CDPH or with each other across regional boundaries; turnover (e.g., change in RL in W region); challenges around communication on accountability, including of RL scope of work; funding/stipends and uncertainty about future funding (e.g., funding for Community Action Plans) (I, D, S). Timely, open, honest communication between project coordinators (PC) and RLs builds trust (I).
5	To what extent do key partners – including community organizations, representatives from the local public	Survey of CLs, and review of HEiAP documents, shows recognition of shared health equity goals across organizations (S, D). However, delays and limitations in needed resources and

	health system, and City departments and sister agencies – have shared goals around health equity?	staffing raise questions about how statements of shared goals relate to commitment to implementation (I, S, D).
5a	To what extent have these partners been learning from one another (i.e., engaged in reciprocal learning)?	Low capacity and lack of shared understanding have limited ability of HEiAP to support policy change needs of RLs (I). Top-down structure of HCEZ limited reciprocal learning across regions and between CLs and CDPH (I, D). Improved data gathering helped RLs and CDPH learn from CLs, other CBOs, and community residents (I, S, D). PCs learned from RLs, but saw limited evidence that learning was influencing CDPH organization wide (I).
6	Is there evidence across the public health system of changing practice to address health equity? What systems changes are being worked on (or have been accomplished) as a result of the CDPH health equity initiatives?	CLs only agree "somewhat" that they are satisfied with the progress of the HCEZ work and expressed skepticism of its sustainability (S). Regular ("3:05") meetings between RLs and CDPH represented a change in practice (I). Improvements seen in data dissemination and use: access of diverse communities to health information, completeness of Chicago Health Atlas, utilization of data for community-driven decision-making (S, D).
7	What do communities, CDPH, and other City departments or agencies feel they need to continue progress in addressing health equity?	Uncertainty on continued funding and limitations in support for HEiAP staff and Action Plan implementation resulted in calls for ongoing and consistent funding (i.e., not subject to grant dollars), and in general prioritization from City administration, as well as more direct communication between CLs and CDPH and opportunities to learn across regions. Ensuring shared accountability for tailoring technical assistance and executing reciprocal communication strategies would also help (I, S, D).

In conducting our analysis of the findings for each evaluation question, we found overlaps and reiteration of key themes. Hence, we feel is most useful to focus the remaining presentation on what we have concluded are this evaluation's "5 Key Findings":

- *Key Finding #1* – The HCEZ and HEiAP initiatives have helped to build Chicago's "health equity muscle." As such, the city is now better prepared to protect its most vulnerable residents from both emergent and chronic health challenges.
- *Key Finding #2* – Together, these initiatives have led to increases in the extent to which CBOs and City departments have adopted new health equity goals into their long-term work plans.
- *Key Finding #3* – Growth in trust among community organizations and between communities and the City is evident but has been slowed by structural barriers.
- *Key Finding #4* – The buildup of skills, collaborators, and trust with (and within) communities is strongly linked (i.e., correlated) with progress in action planning and sustainability.
- *Key Finding #5* – Now is a critical moment for the sustainability of HCEZ- and HEiAP-related work.

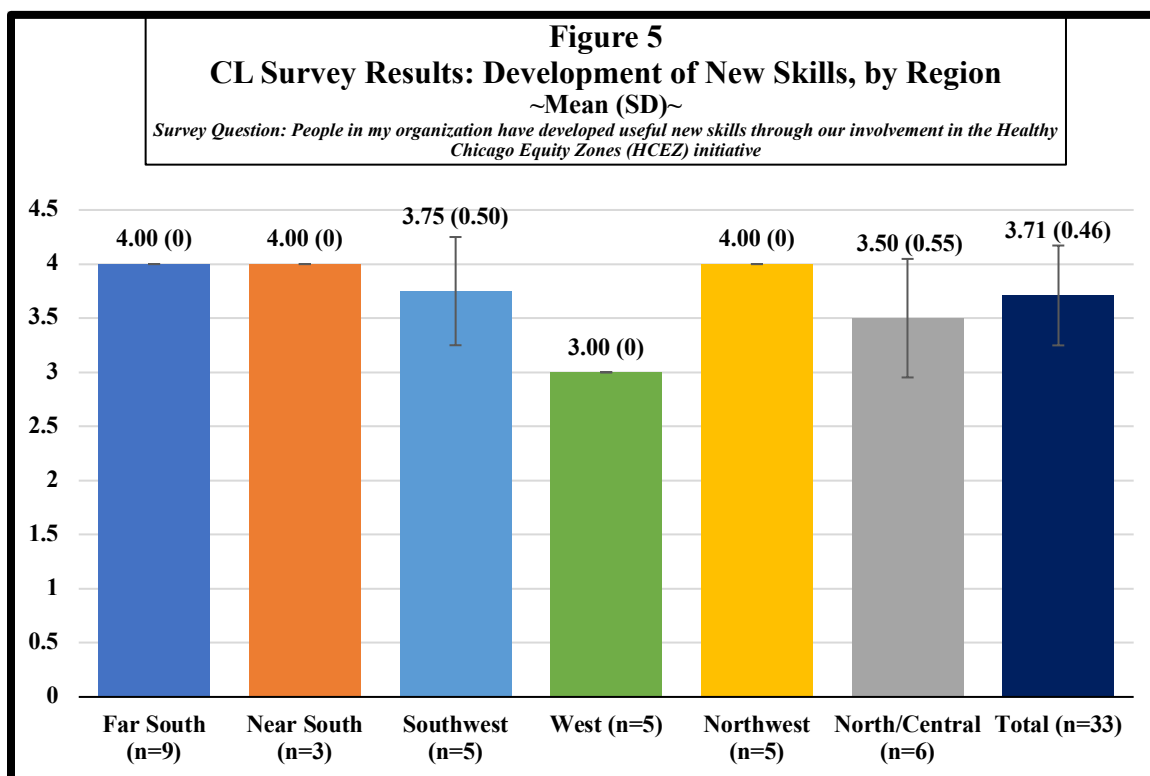
4.1. The Buildup of Chicago's "Health Equity Muscle"

After analyzing our data and reviewing all of our findings from multiple perspectives, as described above, we believe that there is strong evidence to suggest that the HCEZ and HEiAP initiatives have strengthened the City of Chicago's preparedness for protecting its most

vulnerable residents from both emergent and chronic health challenges. As such, these initiatives are responsible for meaningful progress towards closing long-standing racial gaps in life expectancy. In particular, we have observed clear signs of improvement in several areas that can be considered fundamental contributors to, or determinants of, health inequities. These areas include: a) the buildup of skills and resources (i.e., capacity) for identifying and confronting health priorities within communities; b) the buildup of awareness and prioritization of health equity within CDPH and other City departments; and c) the increase in meaningful and diverse partnerships between and among CBOs and the City. Collectively, we refer to these fundamental areas as Chicago’s “health equity muscle.” Our specific findings with respect to each of these areas are described below.

4.1.1. New Capacity within Communities

Our evidence and analyses indicate that CDPH’s health equity initiatives have led to meaningful improvements in the capacity and skills of Chicago communities in important areas that contribute to health inequities. In HCEZ, this increase in community capacity is evident in the results of the CL survey. As shown in **Figure 5**, on average, CL respondents report “somewhat” or “strong” agreement with the statement about developing “*useful new skills*” through their involvement in the HCEZ initiative (mean = 3.71 out of 4). CLs in the Far South, Near South, and Northwest were unanimous in their “strong” agreement about development of new skills.



Note a - Response options - [4=Strongly Agree; 3=Somewhat Agree; 2=Somewhat Disagree; 1=Strongly Disagree]

Our qualitative interview data provide us with additional insight into how these new skills were developed. For example, these data highlight the perception of CLs that their new skills and capacity were developed in large part following the TA provided by RLs.

“[T]he initiative enabled our team members to gain valuable skills in community engagement and health advocacy, equipping them with advanced tools for effective outreach and health education. Secondly, the collaboration fostered through HCEZ broadened our network of resources and partnerships, allowing us to access and disseminate a wider range of health services and information, which directly benefits the community we serve.”

“[The HCEZ initiative] has opened doors for community members to receive the information and benefits they otherwise might not have had access to...”

“As an organization, we learned how to use the Chicago [Health] Atlas tool and have been able to use this tool ... to inform the work that we do based on the data we collected for our service area.”

While some RLs spoke highly about the community resources, tools, and TA provided to them by CDPH and/or independent contractors (e.g., new centers offering free vaccinations), others did not find them helpful – particularly in the early days.

“It hasn't been useful support... I think at one point there were four different organizations that were acting as consultants on this work, and that was the main way that CDPH intended to provide support to regional leads, but it wasn't aligned... so naturally it felt fragmented and incoherent and so didn't have that great of an impact on the work on the ground.”

One RL staff person described feeling like the TA provider was there to tell them *how* to do the work, rather than support them. Others said that the support felt out of sync with the activities they were doing. One RL representative described needing training *“on the front end that we got in the middle.”*

“So by the time we got those things, we had already kind of reached the burnout situation, you know what I mean? Now you're adding another meeting to our schedule once-a-month for three hours, and it's like, 'Yo, ain't nobody got time for that.'”

Some of these struggles were likely the result of a slow CDPH contracting process (e.g., by the time the contracts were implemented, RL needs had shifted) and some may have been due to ongoing transitions between TA providers.

CLs report having learned new skills from RLs in the areas of grant writing, CHA, asset mapping, data collection and analysis, event planning, and knowledge dissemination. The Monthly Update Reports included positive feedback from RLs on the effectiveness of TA sessions. Following TA from RLs, CLs were able to broaden the scope of their work to focus on health equity more broadly (e.g. addressing SDoH) and expand across neighborhoods in the region.

Gaining access to tools like the Chicago Health Atlas was mentioned in numerous CL Survey responses across the city and CLs were able to use the new skills and capacity learned from the TA provided by RLs immediately.

“I learned and conducted surveys, focus groups, and workshops. This was all new to me. I also learned about Chicago [Health] Atlas when seeking data ... about my community.”

“Through workshops and collaborations within HCEZ, we’ve gained access to innovative tools and resources, enhancing our capacity to address health disparities and promote equitable access to healthcare services in our community.”

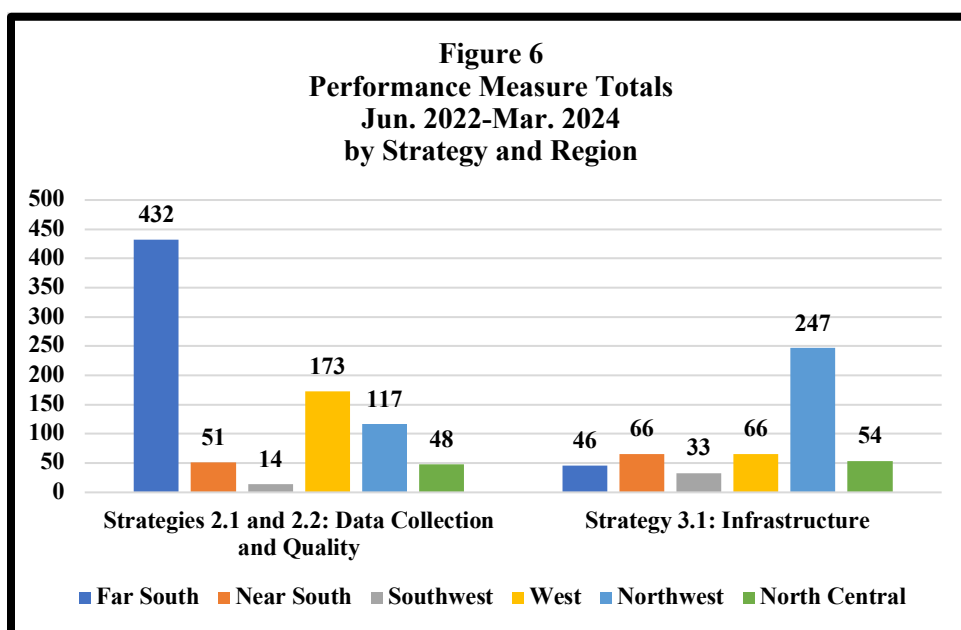
Our document review work also shows that there was measured improvement in data capacity, engagement, and infrastructure for multi sector coordination. The training on data analysis and collection was especially helpful for CLs, as noted in the Monthly Update Reports. This allowed them to adjust their programming to better meet local community needs. More specifically, evidence from the reports show that the initiative has strengthened data collection and reporting capabilities in the regions and CAs, leading to broadened community understanding and participation in data collection and information sharing that supports health equity efforts.

Document review data from the Monthly Update Reports also show some challenges experienced by RLs and CLs in the development and use of the HCEZ Playbook. These experiences highlight the need for clear communication and resources to support data sharing but also demonstrate the value and impact of having comprehensive shared resources.

Additional findings related to increased organizational capacity from the review of Monthly Update Reports include evidence of:

- Increased availability of COVID-19 data at the neighborhood level.
- Greater participation in providing community information (e.g., the priority needs analysis process).
- Positive feedback on data-driven decision-making.
- Improved understanding of health disparities at the CA level.
- Increased participation in data analysis and dissemination efforts at the CA level.
- Higher engagement levels at community events and health fairs.
- Positive feedback on improved communication and understanding of health data.
- Increased accessibility of health information for diverse communities.
- Improved data completeness and accuracy in the Chicago Health Atlas.
- Increased utilization of data tools for community-driven decision-making.
- Positive feedback on the usability and effectiveness of data infrastructure.
- The completion of the Communications Plan and the integration of HCEZ outcomes into regional assessments and reports.

Performance measures data (see **Figure 6**) show the amount of improvement reported in key areas, as well as the variation across region.⁴ While differing levels of organizational/staff capacity likely impacted the level of data collection between RLs and CLs, each region reported dozens of improvements in data collection and reporting capacity (Performance Measure 2.1) within their organizations between June 2022 and March 2024. The number of data collection and quality improvements reported in the Far South is particularly impressive. Multiple improvements to more general infrastructure (e.g., multisector coordination and trainings/education) for addressing health inequities (Performance Measure 3.1) are also evident in each region. Further details about the regional distribution of these performance measures can be found in **Appendix Figures A.8.-A.10**.



With the new skills and capacity developed through the HCEZ program, RLs and CLs who previously had more narrow scope of work were able to broaden the scope of their work to include attention to the role of SDoH in creating or maintaining health inequities. RL and CL involvement in CHA is a prime example of how their newly learned skills and capacity have helped them to become better positioned to address health equities within their communities. Conducting regional CHAs was a key HCEZ activity, with each of these assessments resulting in a report that identified the top health priorities within each CA or region.

Table 6 presents a summary of the methodologies used by each region to gather and analyze data for their CHAs. As this table shows, each region conducted surveys of residents, with several regions also conducting additional focus groups, other types of interviews, and workshops in order to solicit community input. In total, this data gathering effort resulted in over 7,000

⁴ It is important to note that these data are self-reported by CLs and RLs, and although each region was working with a common data dictionary, it is possible that some of the variation observed is due to regional differences in definitions for each performance measure.

Chicagoans providing input into the CHAs; significant improvement in civic participation in health planning

Table 6. Summary of Community Health Assessments (CHAs) by Healthy Chicago Equity Zone (HCEZ) Region							
HCEZ Region		Far South	Near South	Southwest	West	North/Central	Northwest
Regional Lead (RL)		Phalanx Family Services	Greater Auburn Gresham Development Corporation	Southwest Organizing Project	West Side United (Former RL)	Swedish Hospital	Northwest Center
Methodology (N)	Surveys (+/-)	+(N=2,087)	+(N=Not reported)	+(N=974; 1,479)	+(N=974; 1,479)	+(N=Not reported)	+(N=426)
	Focus Groups (+/-)	+(N=39; 235+)	-(N/A)	+(N=25)	+(N=25)	+(N=Not reported)	+(N=Not reported)
	Interviews (+/-)	-(N/A)	-(N/A)	-(N/A)	-(N/A)	+(N=Not reported)	-(N/A)
	Workshop/Coalitions (+/-)	-(N/A)	-(N/A)	+(N=Not reported)	+(N=Not reported)	-(N/A)	+(N=3)
Top Health Priority Area		Violence, Mental health	Neighborhood safety	Health and healthcare access	Mental health	Mental health	Housing, Neighborhood safety
Footnotes: Methodology: (+) denotes that methodology was used; (-) and “N/A” denotes that methodology was not used.							

Several new insights can be drawn from a close examination of the specific health priorities chosen by communities and regions. Our analyses and conclusions related to this are presented in **Appendix A.7**.

4.1.2. Increased Capacity within CDPH to Support Community-Identified Needs and Prioritize Health Equity Work

CHAs carried out by the HCEZ RLs and CLs have likely also played a role in increasing CDPH capacity to understand and support community-identified needs. This is evident in the HCEZ Playbook ([CDPH, 2023b](#)). While the Playbook is primarily intended to serve as a resource for community leaders to inspire future collaborations and action planning that addresses local health priorities, it also has potential to serve as a communication tool within CDPH and City government to help increase internal awareness of community priorities.

At present, other CDPH bureaus and offices are trying to connect with RLs with the intention of engaging in hyperlocal work to address community health priorities. This reflects CDPH's efforts to more fully utilize RL expertise but is also part of a larger effort to continue funding of HCEZ efforts.

One avenue through which CDPH's health equity capacity has been strengthened by these two initiatives can be seen in the work of the HEiAP office. Through the creation of the new EDST and their role guiding the CIA, HEiAP has laid the groundwork for future health equity work

across the department and the City. “We’re really trying to normalize health equity framing,” observed one member of the HEiAP office. Indeed, the creation of the EDST has been central to HEiAP’s capacity building efforts.

“Individuals [in City government], I would say, are not really understanding the full concept of health equity. And this is where we come in as capacity building. So one of the examples is the creation of the equity decision support tool that we’re working on. So it helps City employees, government officials, elected officials to kind of understand how to work with community members on decision-making together and uplifting the work that community members are doing, but how to bring equity into their projects, their policies. So that’s where we are trying to work on the capacity building.”

Another HEiAP office member explained how they are able to help other departments or groups use the tool.

“[W]e’ve had a couple of projects where we’ve been able to see, actually be able to work through that tool so we’re better able to communicate that and how that might be supportive.”

While not yet publicly available, it is clear that the EDST influenced the direction of the CIA, which contained strategies (e.g., environmental justice action plans) that different City departments will take on to support environmental justice.

“So those action plans, they actually used some of the questions that are part of our equity decision support tool. So our support tool was the template that help[ed] develop those strategies.”

The CIA itself is another example of HEiAP’s capacity building and technical assistance efforts.

“A major part of the cumulative impact assessment was both the policy recommendations, and the EJ [environmental justice] action plans, which the dividing line there is on the action plans. What do these departments already have the authority to do and can just do now and what is new legislation that needs to be in place before they can do that. So providing that sort of bigger structural idea of what are more equitable high level policy ways to go about things is sort of the more focus part of it of me for that.”

Finally, at least one HEiAP office member appears to be primarily focused on policy analysis, which includes keeping up with what is happening within the City government itself and looking at other jurisdictions’ policies and best practices related to priority areas so that CDPH/HEiAP’s work can be informed by best practices and/or learn from implementation challenges.

While the work that has been done by HEiAP to date offers the potential to increase CDPH and City government’s capacity to engage in systems-level health equity framing across the department and the City, our evidence indicates that the HEiAP office’s capacity to support the

use of those tools is questionable without the additional commitment of resources and personnel from CDPH leadership. The members of this team are dedicated, knowledgeable, and extremely talented, but since 2022, their numbers have decreased from at least eight to four. After losing or leaving vacant the positions of Director, Project Manager, Project Coordinator, one of the two Senior Policy Analysts, and the Deputy Commissioner who oversaw them, they are left with little clout within the larger department.

“[W]e don't have a deputy commissioner. No one's saying that HEiAP team's name in other rooms and stuff when it comes to leadership topics, it is the whole thing” said one team member. *“My experience so far is that seniority can be really helpful, tenure can be really helpful when navigating relationships within the City.”*

In many ways, this has eroded the trust that staff have with CDPH leadership.

“We've had a lot of CDPH leadership changes in our time here and I feel like the level of trust has not always been clear between leadership. I know when we had our deputy commissioner while she was still in that role, I feel like there was significant trust and a lot of support there; higher up, then that's less clear.”

Institutional commitment to both staffing and support is required for the potential benefits of the HEiAP work to be fully realized.

Also, in 2022, it became necessary for the HEiAP office to shift their focus from the development of tools and processes intended to help City governmental departments apply a health equity lens, and from supporting multiple health equity-centered community co-led projects (called “Community Co-Led Tables”), to completing one very complex health equity assessment – the CIA. While nobody we spoke with expressed regret about the time and energy spent on the CIA, they acknowledged that other efforts, including providing support to other community-led initiatives and partnerships, took a back burner due to limited staffing.

“[S]omething that could have helped us move or have been improved is if we had more team, more team members, because not only was our community members' capacity, small or limited, it was also ours. In terms of the HEiAP team during that time, basically only four members drove the CIA, the cumulative impact assessment.”

Similar to HEiAP, evidence suggests that the work by the HCEZ initiative to date has potentially increased the capacity of CDPH to collect and use community-generated data, needs, and priorities in public health planning, but that long-term success depends on continued support. It is possible that increased capacity at the community level to integrate and provide data may improve CDPH's capacity to improve health equity around community-identified health priorities. However, the strained staff capacity of HEiAP – which was partly due to few staff members and partly due to existing staff being pulled in various directions – calls into question whether CDPH's capacity to center equity in their work has increased, as the HEiAP office has limited bandwidth to both develop tools like the EDST and support CDPH and other City departments in applying these tools.

4.1.3. Partnership Development

Our data also show clear evidence that interorganizational partnerships were strengthened with the support of the CPEZ and HEiAP initiatives. Some of these new or strengthened partnerships were a direct result of community organizations coming together to work on CPEZ/HCEZ or HEiAP activities. However, indirect connections were also made. For example, some RL organizations reported that their involvement in HCEZ has raised their profile as health equity champions within Chicago and this has often led other organizations to approach them for collaboration on non HCEZ-related activities or initiatives.

It was clear through the interviews that many RLs and CLs already had strong partnerships built with other organizations in the region, which factored into why they were selected in the first place. One PC described their RL coordinator as knowing “*literally every single person*” in that region and was able to “*connect everyone to everyone.*”

Another RL stated:

“All of our community leads were already embedded into the community grassroots. And so that's always key. And so, we didn't have to go through all of that about mistrust or any of that because the people in their community knew who they were.”

This RL goes on to say:

“I would like to think reasons that we were chosen as a regional lead was already because of the trust and our impact in the community already. And community leads were chosen for that same reason, because they already were embedded in the community. They already had to trust of the community. And then us coming together ... being responsive to their needs, to their requests to challenges. We didn't just listen, but we actually tried to come up with solutions. And that could have been through other partnerships, that could have been through connectivities [sic] that we had with government, local government, state government. And so I think for us, it's us being responsive to one another that helped build those relationships. And not just hearing and say, ‘Okay, I heard you,’ but then nothing happened as a result of me hearing you.”

For some organizations, participating in the HCEZ initiative and having to reach out and make new partnerships, both at the regional and hyperlocal levels, pushed them to expand their networks, and in general, CLs felt supported by RLs in this effort. For example, one CL told us:

“I would say our regional lead was a help in that, I can't speak to necessarily CDPH being help in that. But also at the same time, I feel like it [the HCEZ initiative] also forced us, in doing certain events, it also forced us to make other friends or get acquainted with other organizations who that is their niche in order to bring it to our people.”

Having resources to share in the community directly helped CBOs build and strengthen their networks at the local level. One RL remarked that they had strengthened relationships in 11 communities, bringing in nearly half of their partners, and that being part of the HCEZ initiative enabled that to happen.

“I’ve been able to help organizations double their budget, hiring staff members, continuing to build trust and communities.”

When asked how HCEZ allowed them to accomplish this, the RL was direct: *“By giving us the finances to bring these organizations on board. You know money talk out here in these streets.”*

Meetings, such as the weekly gathering of RLs and PCs (referred to as “3:05”s because they are held each Tuesday at 3:05pm) also enabled RLs to build connections with one another. In fact, many of RLs found it helpful to meet with one another outside of the official meetings with CDPH.

“We call them the alternate 3:05 with just the regional leads where we sit and we talk. We talk about how we can collaborate, what's working for you, what's not working for you, ways that we can build events that we can collaborate on the things that they've learned. It's just a shared learning space.”

One PC described their RL organization making a large-scale effort to leverage the networks of all CLs in the area, creating *“one big network”* throughout the region.

The experiences that both RL and CL CBOs had with improving their partnerships with CDPH are reported in greater depth in the following chapter on trust. While the partnerships between CDPH and CBOs were more complicated to accurately summarize, overall, there was a strong sense that the efforts made by CDPH and CBOs across the city paid off in terms of stronger, more reciprocal partnerships and greater levels of trust. This was evidenced by a round of applause for CDPH staff leadership at the End Phase Meeting in June 2024. It was also summed up by one RL:

“I think for me, the trust level is great. I have met several amazing individuals within CDPH. Is everything a success? No, I don’t think so. However, it’s a great effort. So I think that trust level is definitely there, but I think that anything can get better.”

HEiAP, too, can point to a number of interorganizational partnerships that have been created or strengthened because of their work. Prior to being shifted to focus almost entirely on the CIA, team members collaborated with OERJ and other departments in racial equity learning cohorts. The team also sat in on several city-wide Community Co-Led Tables that were focused on a number of public health or public health-adjacent issues. Specific partners that emerged from our interviews include those involved in the Chicago Food Equity Council and Elevated Chicago tables, as well as multiple environmental justice community-based organizations from the Environmental Equity Working Group through the CIA initiative. They also provided support to other departments within the City of Chicago.

“So we supported [another City department] by providing narrative around what we understood as not just the impacts of the COVID-19 pandemic, but also how much of a reflection on what additional outcomes were exacerbated from that response or lack of response in the communities that we were supporting applications for.”

In particular, the CIA appears to have deepened relationships between CDPH and organizations in the environmental justice space.

“We’ve had other community partners like Neighbors for Environmental Justice, PCR (People for Community Recovery), along with many other organizations that came to that particular assessment. So they were part of other groups as well. But there were many times where we were all together in the room discussing the assessment process, the next steps, what should we do next?”

“So all of the materials that we created, everything was co-designed. They came to us and they said, ‘Okay, you’re missing this particular piece. Make sure this is part of the material that you’re going to be sharing.’ When it came to our in-person engagements, they themselves created slides as well. They said this was a partnership. ... We didn’t have to ask them to come to us. We were going to the community. So we wanted to make sure that we weren’t just really asking them to listen and just to say, this is what the City’s going to do. It was more about these have been the issues in the community, and that was what our community partners shared because they’re out there and it’s part of their lived experience.”

As the CIA wound down, these relationships have continued to be sustained.

“I think for the environmental justice space in particular, I think we’re starting ... to see more connections and being built and with different initiatives going on in the city. So ... building relationships with community partners has been helpful to at least understand what is the larger landscape of what’s going on in Chicago and how best to ensure that I, there’s a needle being thread between different projects. So for instance, how can [we] best support the Chicago Air Monitoring Network strategy, understanding what were the lessons from the Cumulative Impact Assessment to bring in. So I think that has been going well.”

As the HEiAP office shifts their focus away from the CIA and back to the larger mission of their office, several team members expressed a strong desire to return to some of the partnerships that they were unable to maintain as effectively while working on the CIA.

The need for a continued focus on partnerships and inclusivity – making sure that information is shared in multiple directions, top-down as well as bottom up (i.e., back and forth from CBOs and CLs through RLs to/from CDPH) – came up in the document reviews as well as interviews. However, there was more opportunity for shared information among CBOs across regions (e.g., identifying and sharing promising practices) than we realized. For instance, in their Monthly Update Reports, the Southwest region discussed the development and utilization of public health

ambassadors to connect with communities, but some felt that this innovation was not broadly shared and discussed sufficiently with other regions.

In HEiAP, sharing information and discussions across, rather than just within, individual City agencies; challenging the status of silos; and efforts to connect areas such as transportation and urban agriculture to health were a key part of documented efforts and will require continued commitment to sustain and develop.

In terms of numbers and types of partnerships formed, RLs and CLs have identified over 600 partnerships that have been built or strengthened through their involvement in the HCEZ initiative. For each partner reported, we asked them to identify the “partner type” as well as the partner’s closest alignment with *HC25*’s priority areas. As shown in **Figure 7**, these partners come from a wide variety of sectors, most commonly from health-related organizations, CBOs, or social service providers, but also several from schools, local governments, and faith-based organizations.

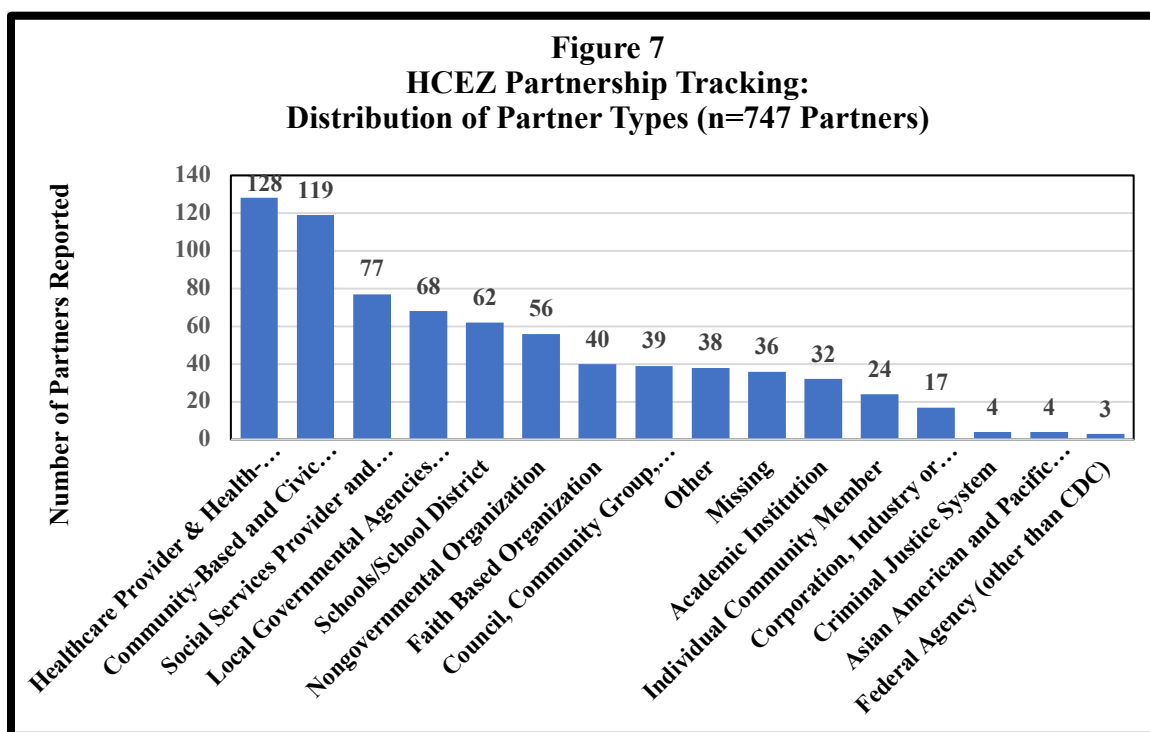
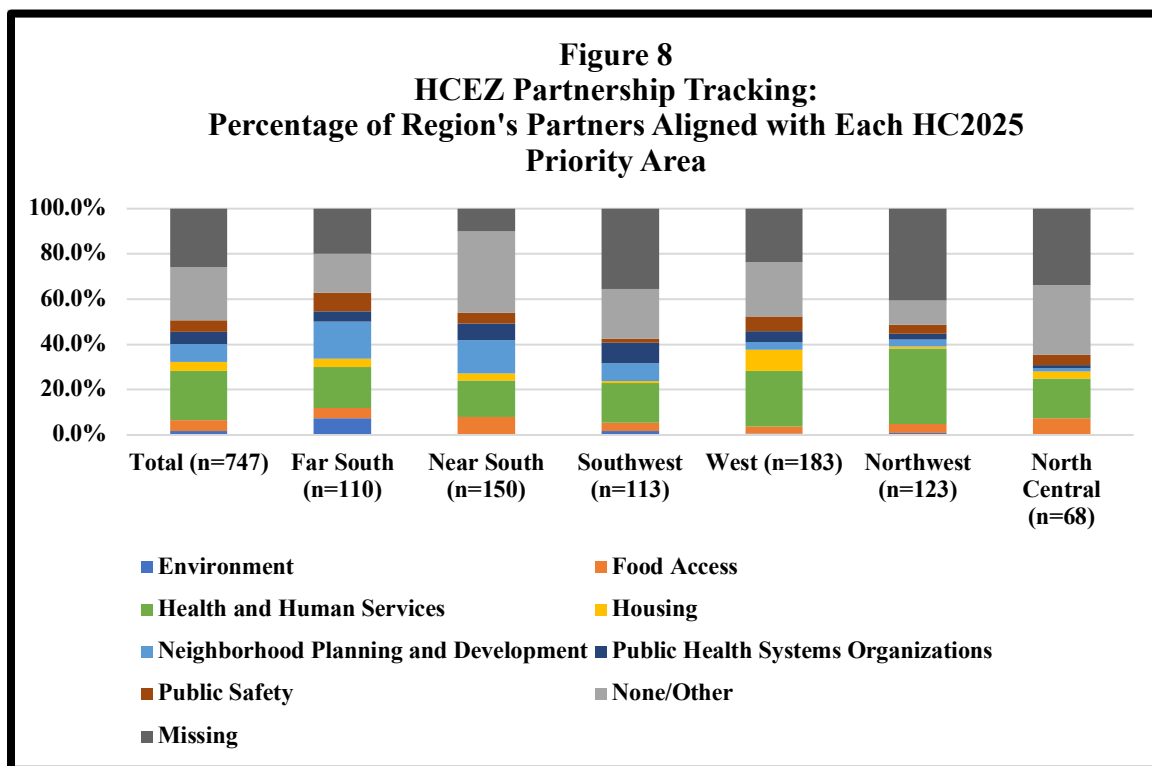


Figure 8 provides further insight into the characteristics of partners that were established and nurtured within the HCEZ network, by region. As this figure shows, about half of partners are aligned with HC2025 priority areas, with the most common area of partner organizations aligning with the HC2025 priority area of *Health and Human Services*. Importantly, it is also clear that partnerships have been made and maintained with organizations aligned with other HC2025 priority areas that are likely to be important collaborators for work addressing SDoH. For example, **Figure 8** shows substantial representation of organizations focused on *Food Access, Housing, Neighborhood Planning and Development, Public Health Systems, and Public Safety*. At the same time, close to 50% of all reported partners do not show clear alignment with HC2025 priority areas as indicated by a response of “None/Other” or missing data for this

variable. We should note, however, that these responses do not necessarily indicate that a partner is not aligned with HC2025 priority areas. Indeed, some RL or CLs may have skipped this step in their reports simply due to time constraints, while others may have chosen “Other” because they did not feel comfortable choosing just one HC2025 priority area for a given partner. Additionally, beginning in 2024, RLs and CLs were no longer asked to report HC2025 priority areas for their partners, with the goal of reducing response burden.

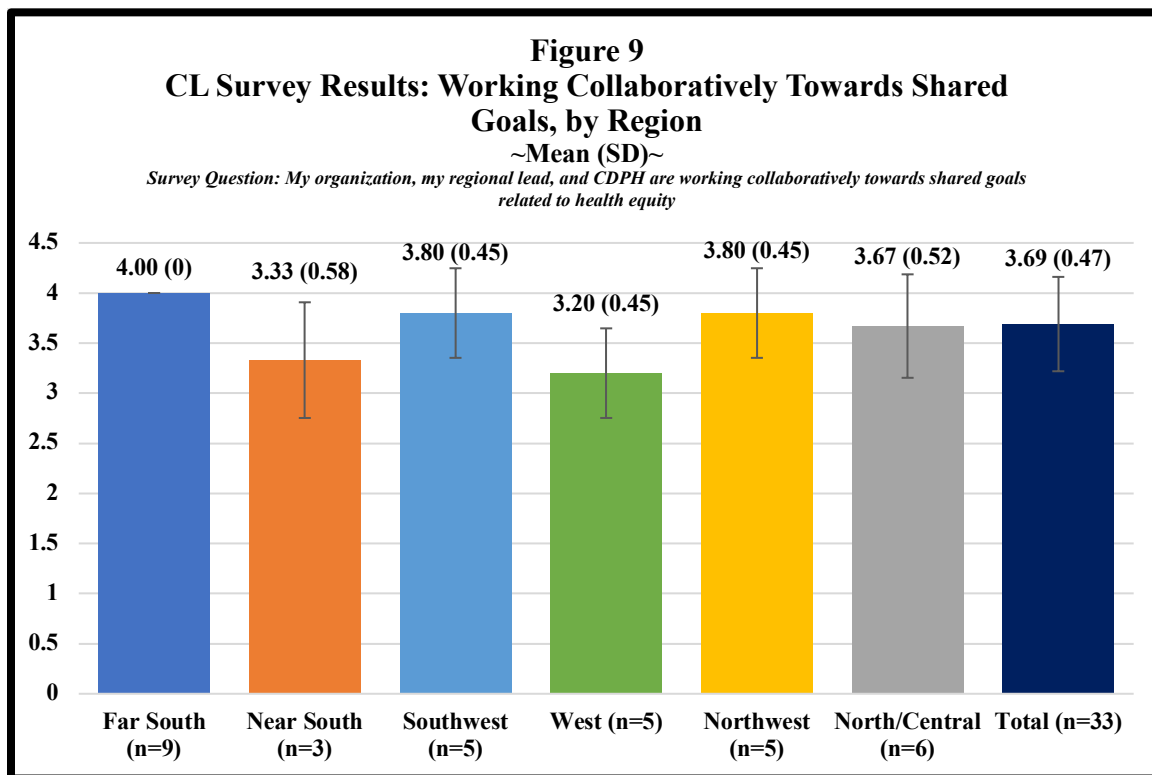


Finally, the Evaluation Team noted both in the 2023 HCEZ Showcase (August, 2023) and in the End Phase Meeting (June 2024), the organic, recurring chant “*Six Regions, One Chicago!*” demonstrated clear evidence of partnership and solidarity.

4.2. Increase in Organizational Adoption of Health Equity Goals

Our analysis of available data also reveals that together the HCEZ and HEiAP initiatives have led to increases in the extent to which CBOs and City departments have adopted new health equity goals into their long-term work plans. For example, our data show strong evidence that the HCEZ initiative provided the overarching framework for RLs, CLs, and CDPH to develop shared goals around health equity. Not only did RLs and CLs learn from CDPH and its contractors, but CDPH leadership also report feeling that the HCEZ structure and process facilitated their ability to engage in reciprocal learning with RLs and CLs. Learning of community-identified health priorities that emerged from the CHAs is one example of CDPH learning from RLs/CLs (also referenced in Section 4.1.2). Consistent feedback loops between CDPH and the RLs also allowed for ongoing dialogue and learning by CDPH about what aspects of the HCEZ work were not being adequately captured in regular reporting.

This sense of communities and the City working collaboratively towards shared goals related to health equity is evident from the CL Survey (see **Figure 9**). Similar to the other questions in the CL Survey, respondents report substantial agreement with the statement about “*shared goals*,” with average responses closer to “strongly agree” than “somewhat agree” (mean = 3.69 out of 4). Again, we also see potentially meaningful regional variation, with CLs in the Near South and West regions indicating a bit less agreement about the collaborative nature of the HCEZ work.



Note b - Response options - [4=Strongly Agree; 3=Somewhat Agree; 2=Somewhat Disagree; 1=Strongly Disagree]

As stated by one RL, the work with HCEZ really “*expands*” their organization’s core mission. Prior to the onset of the pandemic, the organization had not had a strong focus on health, but since then, they received grants and joined in COVID-related projects.

“And so HCEZ then became very much a big part of our work around public health and continues to be a big part of our work around public health. It allowed us the opportunity to get community residents’ feedback on what was important for them or their family and or community. We have established, I think five or six core areas around social determinants of health that is still guiding our work today around public health. ... [HCEZ] has definitely impacted our agency and as a result had had a direct impact in our communities. And we’re seeing the results of that voices now being raised where they may in the past feel like they didn’t even have a voice.”

The shared framework that HCEZ provided for RLs and CLs helped them to better understand how their work is connected to health equity. For a number of RL and CL organizations, their

introduction to health equity and SDoH came as a result of the pandemic, and now they have woven them into the fabric of their missions. RLs, PCs, and CLs expressed shared goals in incorporating health equity into their work long-term. One PC observed:

“I think in my perspective and totally from my opinion, although I would imagine some of the other people on my team would agree with me, in particular really passionate about health equity and they actually see long-term investment from their organization in this area of work. So health equity was not something that [redacted] was involved in at all prior to 2020. Their first introduction to doing any sort of public health work was really the vaccination efforts in 2020. And that's how they kind of got started within the equity zones. And so, they've quickly sort of adopted this area of work into their organization and do plan on building out health equity work long term.”

Another PC observed how they see the work of their RL organization shift.

“I think there will be long lasting changes just because now their work is kind of already oriented towards health equity. Even some of these community organizations, because a lot of them were not even health organizations or things like that. ... a lot of them weren't really in tune to what public health was. So I feel like now that they have this mindset, they always want to, I feel like their goals, even just in their personal organizations are towards public health. So I think there will be long lasting changes in the fact that a lot of these organizations have a mindset shift, so they're incorporating public health and everything they do. But I don't know when the funding ceases. I do think things will stop.”

A CL noted that this work really expanded their partners to include others who are also working in this area.

“By serving as an HCEZ Community Lead, our organization has developed stronger, more collaborative partnerships with a diverse array of local agencies, healthcare providers, and community leaders. This enhanced cooperation and networking have facilitated a deeper understanding of shared goals and challenges, fostering a spirit of unity and collective effort.”

Another CL wrote:

“As an HCEZ Community Lead, we've established collaborative partnerships with diverse stakeholders, leading to enhanced coordination and collective action towards addressing health disparities. Through shared goals and mutual support fostered by HCEZ, our organization now works more closely with others, fostering a network of support and amplifying our impact on health equity initiatives.”

Our interpretation of the data also suggests that PCs were learning from RLs about best practices in implementation of HCEZs, but that PC learning was not always incorporated into CDPH's ongoing work at the organizational level. RLs and CLs advocated for clear communication pathways with CDPH.

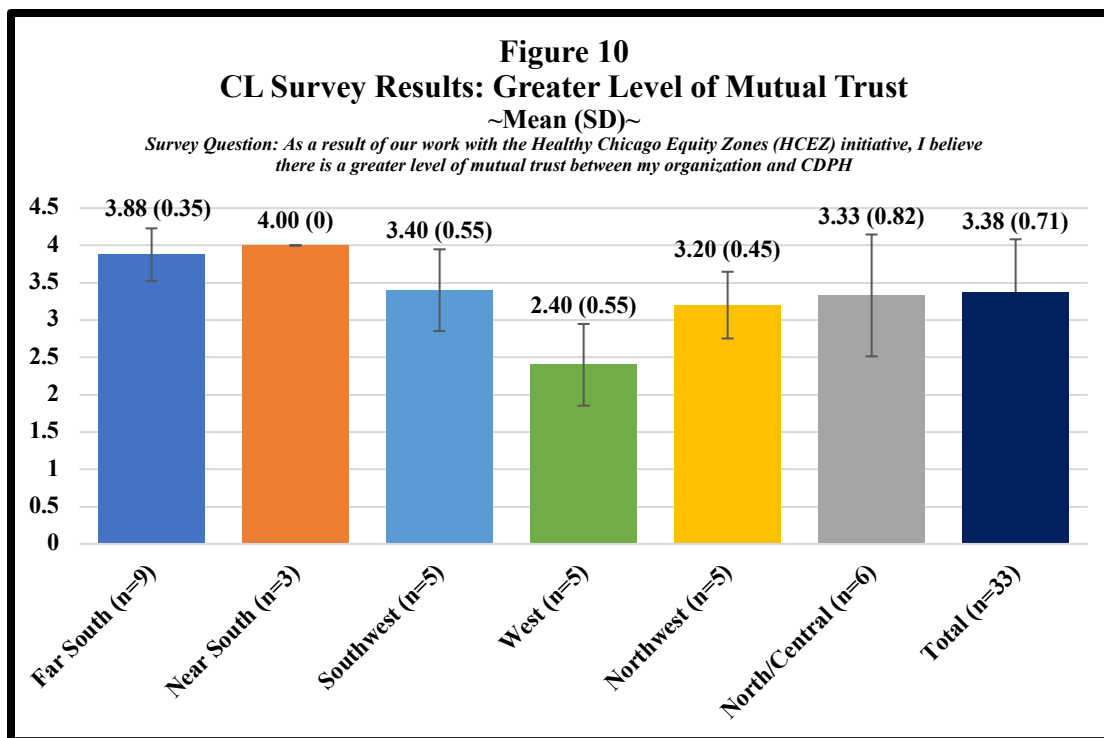
Because of the strict hierarchical structure of HCEZ, RLs and CLs felt that their ability to engage in reciprocal learning with CDPH and across regions was limited. The reciprocal learning between RLs (i.e. across regions) was meant to happen in the 3:05 meetings; however, RLs did not feel that this was sufficient opportunity for regions to learn from each other. Within each region, CLs learned from one another and from their RL.

HEiAP efforts have also focused on helping other City departments to become aware of how their work may impact health equity in Chicago. For example, with support from HEiAP, almost all City departments and sister agencies published REAPs where they communicated their goals on improving racial equity. Each of these plans makes clear that they were built upon the shared goals of *HC25*, and many of these plans address overlapping priorities. HEiAP's support of the EJ Action Plans (see section 3.4.2) also exemplifies their role in centering health equity in the work of diverse City departments.

4.3. Growth in Trust; Barriers to Trust

Importantly, our findings suggest that the CDPH health equity initiatives have fostered a sense of shared leadership and growing perceptions of trust, both within and between communities, and between communities and City government. However, due to several structural challenges, growth in these important areas has progressed more slowly than growth in capacity/skills or growth in partnerships.

Data from the CL Survey show overall agreement that mutual trust between communities and CDPH has grown. As shown in **Figure 10**, on average, CL respondents report “somewhat” or “strong” agreement with the statement about greater levels of “*mutual trust*” between their organization and CDPH (mean = 3.38 out of 4). However, it is meaningful that this mean score is closer to “somewhat” than “strong” agreement. We also note the presence of substantial regional variation in responses, perhaps a reflection of specific and potentially resolvable, rather than widespread and intractable, challenges.



Note: c: Response options - [4=Strongly Agree; 3=Somewhat Agree; 2=Somewhat Disagree; 1=Strongly Disagree]

When it comes to shared leadership surrounding HCEZ, our conversations with RLs, CLs and PC revealed some ambiguity. One RL described shared leadership as “*equity in decision-making and agenda setting.*”

“And I think at the regional lead level, I think CDPH was very intentional and deliberate in that. I don't know if we were given the level of leadership that they expected us to have because of our limited capacity, but I think they at least tried to provide space in terms of always respecting what regional leaders saying.”

One person in CDPH leadership shared that the biggest success in this project was “*literally shifting power to community and doing true co-governance development.*”

While some in CDPH leadership believed strongly that they were implementing an innovative, collaborative decision-making process, it didn’t always feel that way in action. One RL said bluntly:

“I believe that there is shared leadership within our region that is important. I do not necessarily feel like there is shared leadership between community and government through the HCEZ model at this moment in time.”

A PC gave an example of needing to conduct a community health assessment survey by the end of the year, but the process start had been delayed, giving a very tight timeline for the survey to be administered. This resulted in a situation in which power was not shared.

“And so, I think there were frustrations from the Community Leads that it felt like... CDPH is telling us we have to do this and that we only have two weeks.”

While the PC acknowledged that the delays happened on both CDPH’s and the community’s sides:

“But either way, the feeling of we are doing this because CDPH is telling us we have to do this felt present towards the end of the assessment, towards the beginning, there are a lot of optimism, a lot of collaboration, a lot of collective decision-making. As we came up against the deadline more, please make sure you're checking all the boxes and I think felt less shared and collective.”

Building and maintaining trust was a central theme in our conversations with RL, CL, and CDPH staff, including HCEZ and HEiAP. As reflected in the conversations about partnerships between RLs and CLs (described above), there was an overall sense that trust was established between RLs and CLs. One PC remarked:

“I think the relationship between the PCs again and the regional leads is a really good model for that trust building that is necessary for shared leadership. Without trust, I don't think that there can be authentic relationship building and authentic thought partnership with the two parties that you are sharing leadership with. I think part of that too is the department needs to recognize its maybe missteps or mistakes in the past from the perception of our community partners. While we may not think we might've done something wrong or maybe not have made the best decision, at the end of the day, our community partner felt that way and they perceived it to be a mistake or a misstep. And so, we need to recognize that in order to move forward. That is, I think a foundational part of trust building is recognizing what has gone well, what hasn't gone well. And once we can do that, then I think there can be authentic shared leadership and we can begin to, as leaders.”

However, the siloed structure of human services work and community organizing did emerge as a barrier to building and sustaining partnerships at the regional level:

“While we haven't yet extensively collaborated with other organizations, there's potential for future partnership building. However, it's worth noting that at times, it felt like each organization was pursuing its agenda independently. It may have been helpful to work on [our] action plan together to support and build on our resources.”

Multiple RLs mentioned feeling strong, positive connections to the CDPH staff with whom they worked most closely. One RL commented:

“There are some fantastic people in the department who are doing the good work and it's not easy.”

Another noted:

“I mean, I trust CDPH. I believe every individual that is working [as] part of CDPH is absolutely working at their capacity within the system. I don’t think we have ever been led astray.”

While it was clear that at an individual level, a great amount of trust was established between CDPH staff (especially PCs) and RLs, there remained a substantial lack of trust in the systems and processes of CDPH. Several key barriers to growth in trust were identified in our data, including the slow contracting process with the City and communication challenges. On the other hand, the HCEZ and HEiAP initiatives provided many opportunities for City staff to work closely with staff of CBOs and this often resulted in the strengthening of individual personal connections which helped to facilitate the development of trust at the organizational level. The recognition of shared organizational missions was also identified as a major facilitator to trust building and reaching shared leadership. CDPH's slow contracting process was the core barrier to relationship building and reaching shared leadership with RLs and CLs. This was the root cause of many other barriers (e.g. trust, staff turnover). Communication – and, specifically, a lack of shared understanding of language used during these initiatives – also seems to have led to misunderstandings about the RL Scopes of Work, which led to some sense that shared leadership was not reached. Another major barrier to shared leadership with CLs was that they did not have direct communication with CDPH. This was a limitation of the strict hierarchical structure of HCEZ.

To CLs, CDPH was seen by some as a “*silent partner*.” Trust between organizations is very organization- and individual-specific and thus varies citywide. In most regions, trust was strong between RLs and CLs; in the West region, poor communication and the change in RL organization limited trust between the organizations.

Findings from document review of HCEZ Monthly Update Reports supported the challenges documented from interviews in building trust. Challenges to improved relationships included issues around shared accountability, availability and timeliness of funding/stipends, sustainable space for HCEZ-related work, and unanticipated changes due to the Mayoral transition.

More details about these challenges were revealed in our interview data. Despite biweekly 3:05 meetings, which offered a time for regular communication between CDPH and RLs, some RLs still felt unclear about what was expected of them. One RL wished CDPH had been “*a little bit more decisive around what they wanted to see from us*.”

“I think there were times where we had an expectation that they were going to provide something for us and their expectation was for us to go out and just build it and just kind of make it up. And that is not exactly, that doesn't help when you're trying to figure out a relationship as well.”

Another RL described taking on activities that they believed had been approved, but when they described what they did in a voucher, the language they used was flagged as not being approved. “*It really has more to do with the language than it has to do with the activity itself.*”

PCs also acknowledged that there were times when their RLs felt frustrated or left out of the decision-making processes. *“Oftentimes, we’re looping people in way after we’ve made a decision or we’ve chosen a particular path for said grant activity.”*

Feedback from CLs suggests that their relationships with CDPH were mixed, likely because of the hierarchical HCEZ structure that required CLs to reach out to RLs with questions or concerns, rather than ask CDPH directly.

Many CLs agreed that trust between their organization and CDPH had improved.

“As far as level of trust between our organization and CDPH, it’s also increased. They’ve helped us gain access to the skills needed to perform our job, but also provided us access to PPE and vaccines.”

Others felt differently. One CL remarked:

“[W]e still don’t have a deep relationship with CDPH. If that was one of the goals of the initiative, then we should probably have more direct contact with staff there and hear from them directly/share with them directly about the alignment of our goals regarding health equity. It may be something that because of capacity, the regional leads hold the relationship. That’s fine too, but it doesn’t allow for our individual community organizations developing that level of trust and shared vision with CDPH as if we were working together a bit more directly.”

Another CL noted:

“This [trust] has been ever-changing and it has been difficult to be able to form a strong level of trust because it has seemed like every time we are comfortable and begin to trust, then there is a drastic change. We currently have been working on being a bit more openness and trusting with our regional lead as well as CDPH.”

Across the board, there was a sense that CDPH may need to show up differently to enhance partnership and shared leadership. Both PCs and RLs advocated for the inclusion of CLs in team meetings as a strategy to achieve this. They emphasize how incorporating CLs into the meetings would improve communication and collaboration, and many CLs echoed the same sentiment of wanting to have more direct communication with CDPH.

“I believe that the community leads should be incorporated in our 3:05 meeting. I believe that they should be a part of the conversation. Not all conversations, but most of the conversations because the meetings that we are having, it includes them, but they are excluded. So, they are actually doing the work. They need to hear what CDPH has to say as well. And I think that by them being included, it can bring in better ideas, just different perspectives on how we can do things in maybe different ways or better ways, or how they can be more supported or whatever it is. But I definitely think that they need to sit at the table sometimes.”

These frustrations did not go unnoticed by CDPH employees. One CDPH leader acknowledged, *“I think I can say that it never felt like we had quite cracked the nut of what is the relationship between CDPH and community lead organizations.”* This may have been made even more challenging for RLs and CLs because the PCs themselves differed on the level of involvement they have with CLs. Some insisted that communication take place primarily through the RL agency and others tried to make themselves slightly more available to CLs without destroying the hierarchy that was laid out on contracts.

Universally, RLs, CLs, and even CDPH staff expressed frustration with the process of getting contracts approved, payments received, and activities reimbursed. This was an issue that came up time and again, and that threatened not just the work but also the trust that had been established. One PC summed it up:

“I feel like because of our more recent funding issue, taking a really long time to award them for the next grant cycle and all these things, it has not only eroded trust with us and the regional leads, but also I think at the community level to the regional leads.”

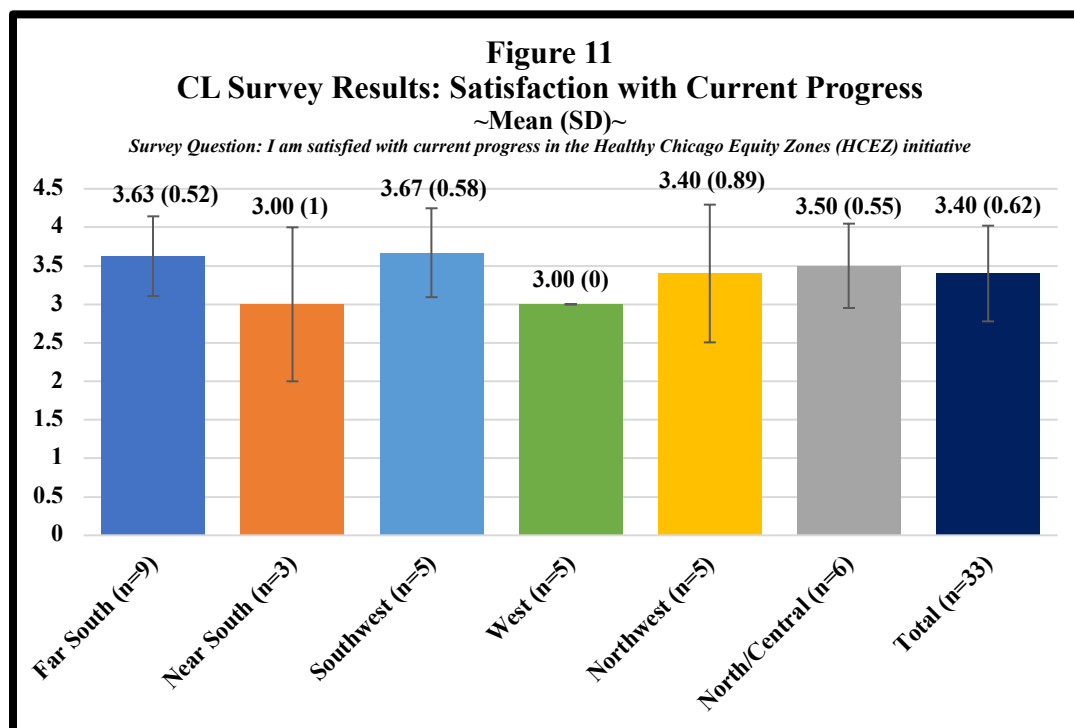
In some regions, when RLs did not get reimbursed, they could not pay CLs, and the work stopped. While there was some recognition that the problem was not caused by any one individual or even with CDPH, it nevertheless caused harm and interfered with trust building. *“[We] can’t front thousands of dollars,”* said one RL. *“That is actually taking a hit against communities.”*

As an evaluation team, we witnessed efforts on the part of CDPH to both acknowledge and attempt to remedy many of the issues that threatened partnership-development between CDPH and RL and CL CBOs. For example, at the End Phase Meeting attended by both RLs and CLs, a member of CDPH’s leadership team spoke with “radical transparency,” explaining that as the CDC grant and COVID-19 funding wound down, the agency did not presently have additional funding that could be directed towards the HCEZ initiative – as much as they wished they did. *“The continuing work is not going to live on the back of the City of Chicago, but on the backs of the community”* whether that was how anyone felt it should be or not. In our assessment, this illustrates commitment to openness and transparency with CBOs that CPEZ and HEiAP staff and partners have advocated for over the course of this initiative.

4.4. Correlates of Progress and Sustainability

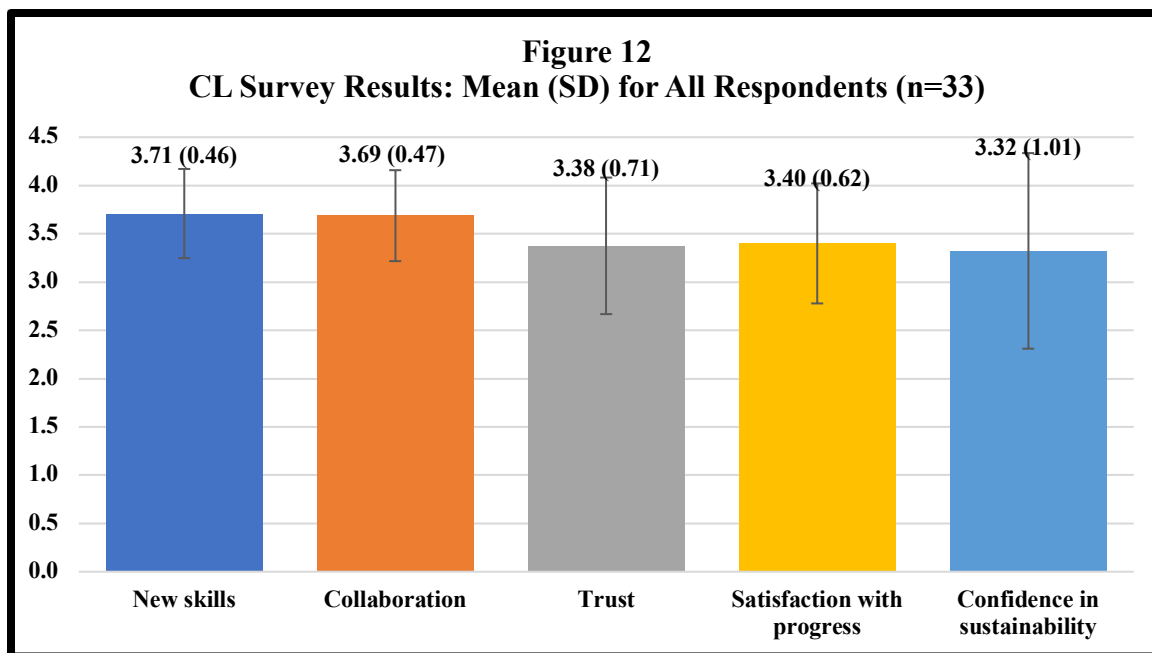
Several additional findings provide further insight into both the potential value and current challenges of CDPH’s health equity initiatives. For example, one key finding from our analysis is that the buildup of skills, collaborators, and trust with and within communities is strongly linked (i.e., correlated) with a community’s perceived progress in action planning as well as their perceived ability to sustain (their) health equity work (i.e., sustainability). In the CL Survey, we asked whether CLs were *“satisfied with (their) current progress”* in the HCEZ initiative. As shown in **Figure 11**, average responses were similar to the averages found for the questions

about “*trust*” and “*sustainability*,” indicating that CLs were only “somewhat” satisfied with their progress (mean = 3.40 out of 4).



Note: d: Response options - [4=Strongly Agree; 3=Somewhat Agree; 2=Somewhat Disagree; 1=Strongly Disagree]

In addition, **Figure 12** presents the mean response scores of all CL respondents (n=33) for each of the five main questions in the CL Survey. As this figure shows, the CLs largely agree that they have experienced growth in terms of new capacity/skills, and they also largely agree with the sentiment that the HCEZ work has been collaborative and centered around shared goals related to health equity. At the same time, however, the CL respondents as a whole report less agreement with regard to perceptions of mutual trust with CDPH, satisfaction with progress, and confidence about sustainability of the work.



Note: e: Response options - [4=Strongly Agree; 3=Somewhat Agree; 2=Somewhat Disagree; 1=Strongly Disagree]

We have also examined the relationship between responses to the CL Survey, as shown in the correlation matrix presented as **Table 7**. Each cell of this table presents a bivariate correlation coefficient, between 0 and 1.0, that can be interpreted as an indicator of the level of correspondence between responses to the two questions from the CL Survey that are represented as rows and columns. Statistical significance tests were not computed due to the small sample size; as such, we do not claim that these correlation coefficients are representative of CLs who did not respond to the survey. We consider these findings as preliminary and suggestive of a set of patterns, rather than conclusive. For convenience, each correlation coefficient that is greater than or equal to 0.50 (with rounding) is marked in bold.

Table 7. Correlation Matrix from Community Lead Survey					
	New Skills	Working Collaboratively	Mutual Trust	Satisfaction with Progress	Sustainability
New Skills	1.00	0.38	0.22	0.46	0.28
Working Collaboratively	0.38	1.00	0.64	0.59	0.46
Mutual Trust	0.22	0.64	1.00	0.54	0.31
Satisfaction with Progress	0.46	0.59	0.54	1.00	0.46
Sustainability	0.28	0.46	0.31	0.46	1.00

From this correlation matrix, we draw several exploratory insights. First, it appears that skills development is strongly associated with progress (correlation coefficient = 0.46), but relatively

weakly associated with trust (correlation coefficient = 0.22) and sustainability (correlation coefficient = 0.28). This means that CLs who perceived the most growth in skills and capacity were also those who perceived the most satisfaction with their progress. On the other hand, perceiving growth in skills and capacity did not necessarily correspond to growth in trust or confidence in an organization's ability to sustain the work. Our exploratory interpretation of this finding is that skill building within communities is likely to be a necessary foundational building block for progress in community-led health equity work, but that simply building up the practical skills and capacity of CBOs (e.g. in the areas of data collection) is not sufficient for contributing to growth in trust among partners or ensuring the sustainability of a health equity initiative.

On the other hand, it appears as if the sense of working collaboratively with multiple partners around shared goals related to health equity may have a broader, and potentially longer term, impact. As shown in **Table 6**, this sense of collaboration is relatively highly correlated with growth in trust (correlation coefficient = 0.64), satisfaction with progress (correlation coefficient = 0.59), and confidence about sustainability (correlation coefficient = 0.46). This means that CLs who perceived the strongest sense of collaboration and shared goals within their HCEZ work were also those who reported the highest levels of trust, the most satisfaction with their progress, and the most confidence in their ability to sustain this work. If this is true, it would suggest that any work that can be done to further support this sense of collaboration among partners would be beneficial because these efforts are likely to build trust, facilitate progress, and encourage confidence about the sustainability of the work.

The remaining correlation coefficients in **Table 6** may also be informative. For instance, relatively strong correspondence between trust and progress (correlation coefficient = 0.54) may suggest that growth in trust leads to progress, or even that progress leads to growth in trust. Similarly, the relatively strong correspondence between progress and sustainability (correlation coefficient = 0.46) may suggest that these two factors influence one another (i.e., when progress is made, the belief that it can be sustained strengthens; or perhaps confidence in the long-term sustainability of the work may facilitate more short-term progress).

Taken together, these cross-cutting findings from the CL Survey provide compelling evidence that the HCEZ initiative has had positive impacts on some of the most fundamental factors that contribute to inequities within communities, particularly the building of skills/capacity and the strengthening of collaborations. Growth in trust has been slower but is still evident. Each of these fundamental factors (i.e., skills, collaboration, and trust) is directly associated with perceived progress with respect to the HCEZ work, and perceived progress is associated with confidence in the sustainability of this work. We infer from these findings that continued investments in building skills, a sense of collaboration, and trust with and within communities is necessary in order to achieve continued and long-term progress in addressing health inequities in Chicago.

As such, identifying communities and community partners where growth in these fundamental factors is lagging should be prioritized. To support this growth, systems change within CDPH is likely necessary. One PC spoke at length about how partners in that region wanted greater connection to other offices within CDPH and City departments, but that was not the role that this PC felt they were supposed to play and they weren't in a position of power to have the existing connections or adequate time.

“I think they [RLs and CLs] want to know that we are embedding this work across the city and that it's not just a siloed program that will fizzle out in our office. They want partnership with other offices, they want partnership with other departments, and they want investments from those departments because they understand that in order for something to be sustainable, it needs to be people from other, it can't just live in one office, it has to have commitment across the board within cities.”

We also note that CDPH may have struggled to balance shared leadership and flexibility for RL and CLs with not providing enough direction. One CL described it well when they said that the scope of HCEZ was co-developed between CDPH and RLs and was intentionally generic to allow for individual variations.

“The part where I think we got stuck was more of the follow-up documentation to provide guidance. For example, with the community assessment, we provided a whole community assessment guide that said, you need to check off these steps. ...There's been this goal of flexibility that I think has caused operational inefficiencies, and I understand that that's maybe part of doing equity and collaborative work. I often felt that the regional leads were looking for more explicit instructions. We thought we were being empowering to say, you can use our partner tool, but you can use whatever you want. We understand you might have already had these, we don't want you to make you do it our way. But I don't think it was actually as helpful as we thought it was to have that type of approach. So rather than being a little bit more prescriptive, where I more explicitly spelled it out of you need to have a regional plan and you need to have a community lead plan, it was definitely not clear on the scope. And then I also don't think it's necessarily clear on the follow-up documents. And then the other actually more main thing I would point to that was fairly obvious to me fairly early that there was this gap when I would ask questions of my leadership and my manager, my director would be like, ‘Hey, is this right? Should I tell them not to do this?’ I would get fuzzy answers.”

This PC went on to describe that they also lacked clarity in their role and in how prescriptive they should be.

“If I saw that they were probably not meeting what we want them to do, should I say something? It was unclear to me if that was in my role or not, I would ask questions. I would say, ‘Hey, I think you guys should consider this.’ But if they just kept not doing it, I would just let it go.”

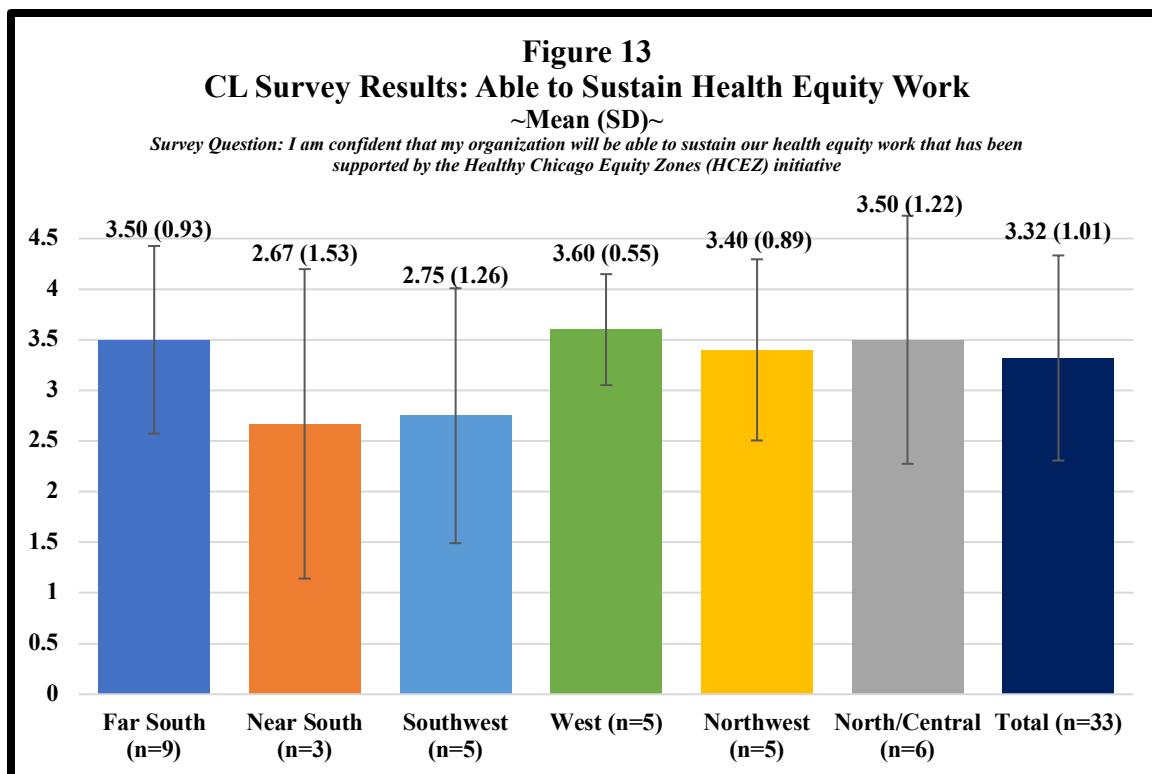
Stronger integration between the actions of HCEZs and HEiAP was also mentioned as an opportunity for further supporting the fundamental factors that lead to progress and sustainability of this work.

“Even within our office, there's this constant feedback about connecting our work to the HEiAP team because at some point, I guess we had mentioned that HCEZ and HEiAP would be working together, and we've kind of gone back on that a few times and we totally have created some confusion with our regional leads about what our relationship to HEiAP is. And HEiAP does play an important role within the work that the Health Disparities grant is doing, but we just haven't been able to make the connection between how does HEiAPs work fit into HCEZ and how does HCEZ support HEiAP's work?”

4.5. Sustainability Considerations

Our findings indicate that we are currently at a critical moment in the life course of CDPH's health equity initiatives. To continue progress in addressing health equity, both through CPEZ and HEiAP, there is an urgent need for continued investment in support of these initiatives. From the start, a goal of the HCEZ has been to build the capacity of RLs and CLs to secure their own independent funding to continue their health equity work; however, it is clear that communities remain somewhat skeptical with regard to their capacity to find and successfully compete for new funding. Meanwhile, due to changes in leadership and competing interests, it remains unclear if staffing capacity within CDPH will be able to remain strong enough to provide the level of support that communities will likely need in order to obtain additional funding. Without adequate support from CDPH, and with limited additional funding, it is also possible that the new partnerships that have been built and strengthened through the CPEZ and HEiAP offices will also weaken, and much of the fundamental progress that has been made with respect to Chicago's readiness to address health inequities with community-led approaches will be lost.

The findings from the CL Survey are a good indicator of the skepticism perceived by communities with respect to the sustainability of the HCEZ work (see **Figure 13**). Of the five main questions asked in the CL Survey, the question asking organizations how confident they are about their ability to sustain the work received the lowest level of agreement. On average, CLs reported a level of agreement with the statement about their ability to “*sustain (their) health equity work*” that is closer to “somewhat agree” than “strongly agree” (mean = 3.32 out of 4). In two regions – Near South and Southwest – even more skepticism was reported, with means scores in these regions falling below 3, an indication that some CLs in these regions do not believe that this work will be sustainable.



Note f: Response options - [4=Strongly Agree; 3=Somewhat Agree; 2=Somewhat Disagree; 1=Strongly Disagree]

There appears to be some disagreement or confusion with regard to whose main responsibility it should be to secure additional funds, with CDPH generally stating that the communities need to be responsible for securing funds and RLs feeling that CDPH should take a more active role. Some also suggested that there needs to be a Health Equity Office within CDPH that is funded by consistent City (not grant) dollars.

Of the employees working directly on HCEZ and HEiAP, it seems as if only three positions are funded through the City; the rest are/were grant-funded. One CDPH staff person we interviewed expressed that:

“Some in leadership are struggling to understand the benefit of the transformative work” that the initiatives were promoting and expressed concern that because both projects were funded through COVID-era grants and operated outside of the standard structure of the department, they would be ended as ‘COVID-era’ initiatives.”

As it currently stands, leadership in CDPH have been working tirelessly to keep both the CPEZ and HEiAP offices afloat by juggling grants and moving valuable staff on and off projects in order to retain them. And, the reality is that a combination of private (e.g., from the business community or philanthropy) and public funding is likely needed to sustain this work. HC25 calls for a generation (20-30 years) of sustained efforts to combat health inequities and come to an "ideal state" of equity. This would most likely take billions of dollars in investments, a funding scope that is well beyond the limits of the City's capacity.

4.6. Concluding Insights and Interpretations

As evaluators, we have been charged with looking at these initiatives not only at a microscopic level, but also from the 10,000-foot vantage point. This view allows us to consider the larger environmental contexts in which HCEZ and HEiAP exist and identify those factors that influence the implementation and sustainability of these projects.

CDPH's health equity initiatives are rooted in an understanding that racist policies and practices are deeply embedded within our institutions and society and that true health equity cannot be realized without intentionally identifying and dismantling these structural artifacts. The *HC25* report calls this out as one of its primary tenets: "To transform policies and processes to foster anti-racist, multicultural systems." Both the HCEZ and HEiAP initiatives show a deep commitment to implementing anti-racist principles moving forward, particularly in how CDPH interacts with communities most impacted by racial and health inequities. Yet as evaluators, we also observed that there are ways in which the current internal practices and processes of the department may be out of alignment with these principles. Specifically, we wish to highlight our observations surrounding the workload and level of power among junior Black and brown CDPH staff working on the initiatives.

Among the many strengths of HCEZ, one is that the frontline CDPH staff who engage most directly with community-based organizations were (in many cases) of the same minoritized groups which they were tasked with supporting. We felt that, at times, this racial concordance created a more trusting environment between the junior staff of City government and representatives of community-based organizations. However, we also viewed this feature of the HCEZ initiative through a more critical lens: the staff with the lowest levels of power over CDPH bureaucratic structures and timelines were the ones required to directly communicate with the staff of the community-based organizations who were being actively harmed by those government structures and norms. Another strength of HCEZ are the passionate and mission-aligned people it brought together. This strength can also be a burden: we observed the commitment of junior HCEZ staff to meet the demands of their role as government representatives to local community organizations while facing under compensation and constrained institutional power.

We saw a similar trend in the HEiAP office. Despite HEiAP's ambitious mission focused not only on internal CDPH policy and systems change, but on transformational policy and systems change efforts across City government, we began the evaluation with a HEiAP office that was woefully understaffed. In the few years since the office was established, the team leader transferred to a new position, multiple staff members left, and the Managing Deputy Commissioner who supervised the team in the interim resigned, with no indication that any of these positions would be filled. The remaining staff members, many of whom are Black and brown professionals, were left with no direct support and little access to CDPH leadership. One person was assigned to take on many of the management and leadership responsibilities for this office – a young Black woman who, to our knowledge, was not offered additional compensation, a commensurate title change, or given additional power within the department. With no direct supervisor, this person relied on support provided by other colleagues who had greater access to the decision-makers within CDPH.

It is clear how the conditions described above, if left unchecked, can lead to burnout, poor retention of CDPH staff, and a resulting loss of institutional memory. Further, in the case of HEiAP, we hope to communicate how the understaffing of the HEiAP office serves as the key barrier to the entire City of Chicago government building its capacity to transform their way of doing business to incorporate health and racial equity into their daily activities. The work of changing internal culture requires extensive relationship building and support. It cannot happen through one-time trainings or one-off consultations, and it cannot occur without a fully staffed team with the capacity to carry out the core aspects of its work.

Although the decisions that led to the conditions described above were not intended to be discriminatory, the result created an unjust situation in which Black and brown staff were assigned responsibilities well above their job classification, while receiving no additional compensation for taking these tasks on, and in the case of HEiAP, with no assigned supervisory support. This reality limits the transformational impact of these health equity initiatives and is in direct misalignment with the stated anti-racist values of the *HC25* report. To fully embody the *HC25* ideals, the department must transform its own internal policies and practices to create anti-racist systems that support its own staff in doing their anti-racist work.

5. LIMITATIONS

The findings reported here are supported by a methodologically rigorous process that sought to maximize the reliability and validity of our data. As such, we are highly confident about the accuracy of our findings.

That said, like any complex evaluation, our findings should be interpreted in light of this study's limitations, many of which were outside the control of our team. Below we highlight six limitations that we feel are the most important to consider when drawing conclusions from our findings.

1. Perhaps most importantly, the evaluation activity did not commence in tandem with the development and initial implementation of the HCEZ and HEiAP initiatives. This is not in line with best practices in program planning which suggests that evaluation planning should begin in step with program planning and implementation. The delay in evaluative activities may have resulted in missed opportunities in the evaluation, such as assessing changes between the pre-implementation and post-implementation periods. Relatedly, as evaluators we identified discrepancies between the vision of this work articulated in the original CDC grant narrative and CDPH's written description of the work and the work that was actually completed. This is not unusual, but we found it led to tensions among partners likely rooted in uncertainties of roles and expectations. Better articulation and communication of the vision and the roadmap may have reduced that tension and clarified the best direction for the evaluation.

2. Our ability to closely examine regional and community area level variation was somewhat limited due to the relatively small sample size of HCEZ RLs and CLs, and our commitment to upholding the confidentiality of interviewees.
3. We did not conduct interviews with every relevant actor within every aspect of this health equity work (e.g., we did not interview individuals from other City agencies or any other HEiAP partners). Instead, we interviewed a subset of key informants that offered insights into distinct aspects of the citywide health equity efforts.
4. Some of the key work products of the HCEZ (e.g., action plans) and HEiAP (e.g., EDST) initiatives have not yet been finalized. As such, we were not able to assess how these products are being used by partners, or what impacts these products may have on health equity in Chicago. Future process and outcome evaluation focused on these products is necessary.
5. All of the key data that informed our evaluation – including, for example, the key informant interviews, the CL survey, and the performance measure and partnership data – was self-reported. This is a common practice in public health evaluation, and much can be learned from self-reported data. However, we also recognize that these types of data can include unpredictable biases that can weaken reliability and validity.
6. Finally, while document review methodology is a commonly utilized systematic approach to organize and analyze existing documentation on a case in order to answer evaluative questions (Bowen, 2009; Rapley, 2018), this approach can be challenging due to issues of data completeness and data quality. A robust dataset is contingent on data availability and transparency.

6. KEY RECOMMENDATIONS (CONSIDERATIONS FOR THE FUTURE)

Our review of the literature as well as our own evaluation work has shown that both conceptually and practically, the public health practice models illustrated by HCEZ and HEiAP offer promising mechanisms for improving health equity in Chicago. Moreover, there is enthusiasm in Chicago communities for continuing power-sharing initiatives with CDPH. We have observed that these approaches have led to better relationships and improved trust between CDPH and community partners, greater skills and understanding of public health and health equity among community members and residents, and an expanded network of hyperlocal public health partners. Our findings offer compelling evidence that in a large and diverse city like Chicago, the use of a hyperlocal lens to support equity-centered and people-powered efforts that champion resilient communities to make their own decisions on health priorities and utilize local assets to solve health problems is both innovative and impactful. However, these gains are at great risk of being lost. To prevent this from happening, we offer the following recommendations for consideration:

- 1) *Ensure adequate support for HCEZ and HEiAP staff, as well as RL and CL partner agencies, as part of City-funded public health services*
 - a) Ideally, this would entail finding additional revenue from the City of Chicago, rather than relying on one-off grant support. This does not mean pulling resources from other essential public health services, although it could mean that CDPH finds ways to integrate HCEZ and HEiAP within existing public health services (e.g., RLs and CLs participate in developing and implementing community-tailored infant vaccination campaigns in collaboration with the Maternal, Infant, Child, and Adolescent Health division).
 - b) Additionally, we recognize the need for institutional commitment to maintaining adequate internal staffing and support for both HCEZ and HEiAP. The CDPH staff who have been responsible implementing these initiatives – from project coordinators to directors – are well-trained, highly competent, and dedicated to improving health equity in Chicago. Fully realizing the enormous potential of these initiatives requires adequate support of the staff and offices that prepared to carry out the work. For example, while the work that has been done by HEiAP to date offers the potential to increase CDPH and City government’s capacity to engage in systems-level health equity framing across the department and the City, our evidence indicates that the HEiAP office’s capacity to support the use of those tools is questionable without the additional commitment of resources and personnel from CDPH leadership. The members of this team are outstanding, but since 2022, their numbers have decreased from at least seven to four. After losing a director, PC, Senior Policy Analyst, and the Deputy Commissioner who oversaw them, they are left with little clout within the larger department.
 - c) We also recognize that other, non-fiscal, support is needed for this work to continue and grow. For example, we recommend that CDPH consider ways in which it can continue to provide low-cost support for the types of generative partnership development activities that have been initiated via the HCEZ and HEiAP initiatives (e.g., providing time and space for the convening of partners).
 - d) We also believe that some cost efficiencies can be achieved by establishing mechanisms that ensure a more reliable, timely, and consistent matching of TA with the needs of community groups (e.g., RLs and CLs). Such mechanisms should also ensure that the TA offered builds upon the assets and skills that communities (i.e., RLs and CLs) already demonstrate.
- 2) *Invest in less hierarchical structures*
 - a) We recommend that CDPH continue to look for ways to reduce top-down approaches when implementing power-sharing initiatives with communities. Equity-focused work requires ensuring that all voices are heard and valued, especially those with less power and autonomy. While some hierarchical structure may be inevitable in governmental work, we have observed how hierarchy in the current model inhibits trust and the flow of information.

- b) Along these lines and with regard to HCEZ, specifically, we recommend increased communication between CLs and CDPH, thus leveling the playing field. Presently, most information continues to flow from CDPH Leadership to the PCs, to RLs, and finally to CLs. Decision-making about HCEZ work is equally top-down, with CLs often feeling cut out of the conversation. This can stifle communication and co-learning.
 - c) Similarly, we recommend that front level staff at CDPH – those in closest contact with communities (e.g., PCs in HCEZ) – be granted greater power within CDPH to make decisions and enact change. We have observed that the PCs are those who are the most well-connected to community partners, yet do not have the power within CDPH to promote institutional change based on their experience.
- 3) *Continue to deepen genuine collaboration with community partners*
 - a) Advancing health equity in Chicago requires the institutionalization within CDPH of the foundational trust that community partners have the ability to co-develop solutions. Our own evidence – as well as evidence from other HEZs (Washington Department of State, 2023; Rhode Island Department of Health & Change Solutions, 2021) – demonstrates the great potential of approaches to hyperlocal public health practice that are founded in trusting communities to know their own needs and identify solution-focused ideas. This type of hyperlocal capacity building creates relationships within CLs and RLs, across RLs, and with partner institutions that did not exist before, and is generative.
 - b) We also recommend including all levels of community partners in decision-making about the future shape of these initiatives. This includes being more transparent and more hands-on when creating scopes of work for communities. For example, while CDPH may not have the capacity to work with all CLs, allowing for more direct communication with CLs (that does not have to go through RLs) could be helpful.
 - c) Importantly, we also recommend attention to the City-wide (i.e., not unique to CDPH) structural barriers that inhibit timely compensation and contracting, as these barriers can negatively impact trust-building and partnership development. Investments in approaches to contracting with community partners that will streamline scopes of work, contracts, and timely payments are recommended. This includes the creation of avenues that allow for long-term contracts and commitments. As a tool for integrating equity into all decision making in the City, the EDST could be a useful mechanism to rethink compensation and contracting processes and practices.
- 4) *Continue CDPH's commitment to increasing community power to address SDoH while deepening civic engagement to hold policy makers accountable for policies that advance health equity*
 - a) We have learned that community organizations want more connections to City offices, and so we suggest that CDPH facilitate this by building intentional

linkages. To create accessible mechanisms for this type of engagement, we see an opportunity for HEiAP and HCEZ to work in closer concert with one another without losing the individual focus of each, as deepening collaboration with other government sectors is a key component of HEiAP. Approaches that have yielded impressive outcomes in Chicago include:

- i) The HCEZ Playbook informed by a higher level of civic engagement than any CDPH CHA effort in the past.
- ii) The promise for Citywide institutional change through the EDST and the REAPs.
- iii) The potential for policy change to reduce environmental harm to communities through the community co-designed, co-led CIA.
- iv) The norming of co-led tables to address SDoH.

5) *Raise awareness of the transformative potential of HEiAP*

- a) We see unrealized potential in HEiAP. This potential could be more fully realized with a clearer understanding of the value and impact of the HEiAP office by leaders and the public. We have observed that due to the breadth of the HEiAP mission – changing institutional culture and systems to increase awareness and prioritization of health equity – the HEiAP portfolio may appear as less cohesive than the scopes of work in most other CDPH/City initiatives. This sometimes makes their work invisible to leadership. It also makes the office more vulnerable to reductions in staffing capacity as they are often called upon to fill in gaps on CDPH projects with immediate needs, pulling them away from their primary mission. This only exacerbates the challenge for leadership to clearly understand the value and impact of the core HEiAP office. Therefore, we recommend that CDPH Executive Leadership create more transparency on the strategic direction for HEiAP to foster health equity in Chicago, as this will raise critical awareness of the transformative value of this work. A more widely recognized and accessible articulation of the strategic priorities of HEiAP is needed. We believe that HEiAP is well positioned to include among its strategic priorities the work necessary for enhancing trust, transparency/communication, and sustainability of the HCEZ coalitions.

As the evaluators of Chicago's HCEZ and HEiAP, we are deeply enthusiastic about this approach, which shifts power to the community and creates mechanisms to provide accountability for policy making that advances health equity. As with other evaluations of HEZ and HiAP, we recommend that future evaluations continue in step with the evolution of the initiatives. Moreover, we see value in the continuation of developmental, participatory evaluation approaches. Future evaluations should focus not only on the use and outcomes generated by the direct products of these initiatives (e.g., HCEZ action plans; EDST), but also on more fundamental domains such as community education, organizational capacity, systems change, and norm/culture shifting.

Finally, we would like to bring attention to the idea that this work requires a commitment at all levels of power within an organization to apply a health equity lens to all of the work that they do. That is, it cannot be only programmatic staff engaged in this work. Commitment to

considering who benefits from decisions, policies, and programs; who is harmed by decisions, policies, and programs; and the process of seeking and centering the voices and experiences of people with lived experience when decisions, policies, and programs are discussed needs to come from the top. While we saw evidence of capacity building for collaboration and the deepening of relationships required for trust-building, we have seen less evidence of the culture shift that is needed to sustain this work within organizations. Further, we observed problems arising when individuals who relentlessly push for equity leave. We encourage CDPH to engage in strategic planning that focuses on maintaining trust and transparency with communities when key staff leave.

Further, like any government agency, CDPH must reflect on the possibility of doing harm in the name of doing good. This can be avoided by investing in systems-level transparency (e.g., institutional policies that support transparency) and considering the ramifications on communities when unpredictable changes to the work occur. As power is shifted from CDPH to local communities, it's essential to reflect on whether this transition might unintentionally place an undue burden on these under-resourced communities to fund initiatives, while also potentially diminishing governmental accountability to the public's health.

7. CONCLUSIONS

Our evaluation of the CDPH's recent health equity initiatives – HCEZ and HEiAP – has provided strong evidence that Chicago is moving in the right direction towards addressing health inequities and reducing the racial gap in life expectancy. We have observed a strengthening in the amount of local community investment in public health and health equity efforts, a broader understanding among community organizations of the variety of services and efforts that contribute to community health, improved trust and communication between CDPH and other City agencies and residents/agencies from local communities, and a more evident integration and prioritization of health equity and racial equity into the work of all areas of Chicago government. This means that CDPH's health equity initiatives are having positive impacts on some of the more fundamental factors that contribute to inequities within communities. As such, we can conclude that the HCEZ and HEiAP initiatives have effectively built-up Chicago's "health equity muscle", leading to meaningful change towards closing racial gaps in high priority chronic health problems, while at the same time positioning the city well to be more prepared for the next pandemic.

Although these initiatives are moving Chicago in the right direction, we also conclude that HCEZ and HEiAP need sustained commitment from the City and CDPH in order to maintain and continue the progress that has been made. Steady funding, increased transparency, and efforts to address structural barriers to collaboration should be critical priorities for the City. With such a sustained commitment, Chicago and CDPH has a real chance to be a model for the country in how to do this type of community co-led work effectively.

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9. APPENDICES

Appendix Table A.1. Overview of Other Health Equity Zones (HEZ) Initiatives

State (year established)	Programs Funded	Activities and Interventions	Evaluation Criteria and Process	Findings
Oregon (2011)	10	- Guaranteed coalitional representation across communities of color; organized 141 education and training events. - Coalitional support from paid staff; creation health equity leadership, training, and action program. - Partnerships with regional care coordination organizations (CCOs) and healthcare systems. - Culturally-specific programs for high schoolers, college students, and their families; training and community events focused on equity, structural and historical racism, and workplace inclusiveness. - Outreach and engagement with Latino/a community members; legislative advocacy for health equity and fair housing law; coordination with CCOs for medical interpreter services.	- All regional coalitions engaged in a facilitated and collaborative process to set and prioritize three evaluation questions, five outcomes, and eight progress indicators or metrics. - Evaluation questions assessed equal representation in coalitions and programs and increased local capacity, leadership, and coordination across social services and healthcare entities. - Indicators include level of representation in coalitional bodies, quantity of support and education provided to organizations and CCOs, extent to which SDoH is connected to health equity work, and new relationships with policymakers.	- Reached 935 organizations and 6,000 individuals; representation from communities of color, AI/AN, LGBTQ, low-income, and migrant communities in leadership of all or most coalitions. - Supported 157 organizations and more than 300 individuals; 23 staff positions created. - Assessed barriers to use of healthcare interpreter services; decreased teen pregnancy rates; improved high schools graduation rates; improved self-reporting of mental health among LGBTQ high schoolers. 100% graduation rate among participating students; equity training for over 600 individuals; engagement of policy makers in racial equity training. 10 Latino community members elected to local policymaking bodies; advanced policy to address unsafe housing conditions; created new payment models for clinics with interpreters.
Maryland (2012)	5	- Creation of and operational support for patient-centered medical homes, community health centers, and medical home services. - Creation of a new transportation route for residents with transportation barriers to access healthcare. - Support for a Virtual Supermarket Program. - Community development grants for health, nutrition, and lifestyle education by community physicians for youth ages 8-18.	- Independent evaluation by the state, assessing established outcome metrics, as well as how funds were distributed and outcomes for practitioners. - Outcome metrics include rates of hospital admission and readmission, emergency department visit, diabetes, asthma, cardiovascular disease, and behavioral health, adapted for the work plan of each HEZ, as well as increased number of healthcare staff and providers and clinical care and treatment quality evaluation.	21 new or expanded healthcare delivery sites. - Over 99 full-time employees added including over 52 practitioner positions and 12 community health worker positions. \$326,985 in income tax credits and loan repayment assistance provided to practitioners. - In a three-year period, inpatient stays decreased by nearly 20,000 and emergency department visits decreased by just over 40,000. - Savings for the healthcare systems were assessed to be far greater than the cost of the initiative. - Independent scholarship has found that, while outcomes improved on average, disparities increased for Black, Hispanic, and Medicaid patients did not see any improvements in some cases. Geographic disparities were unchanged.
Rhode Island (2015)	15	- Nutrition education and food distribution. - Advocacy to promote public health policy. - New programming and coordination of existing programs to improve access to social services,	Initial community assessment through primary data collection and secondary data evaluation to identify health inequities.	- Distribution of up to 12,000 meals per HEZ, though many individual HEZ distributed far fewer; nutritional education for 3,594 individuals in single HEZ. - New policy to restrict smoking in public parks and pass green street ordinance. - Increased community engagement and participation in community programming and advocacy across multiple SDoH; increased job training.

		healthcare services, education, and training, advocacy, and leadership programs. ● Creation of a new family support center for behavioral health and wraparound services. ● Development of green and community spaces. ● Housing improvements, development of affordable housing, and advocacy for housing justice. ● Mental health, behavioral health, and sexual health education. ● Distribution of PPE and vaccinations following outbreak of COVID-19.	● Ongoing evaluation according to a predetermined evaluation plan to assess successes and challenges and document progress in coalition building. ● Implementing interventions, changes to health and health inequity status, and iterate on the initial intervention plan.	● 38-44% decrease in lead poisoning by HEZ. ● 2.0-23% decreased housing cost burden by HEZ. ● 8.3-50% decrease in deaths by overdose by HEZ. ● 24-33% decrease in teen pregnancy by HEZ. ● Behavioral health and depression screening training with more than 1,000 individuals across relevant professions. ● 4.2 million masks distributed; 207,000 individuals tested for COVID-19; 328,00 self-test kits distributed; 25,000 vaccines administered; 842,000 meals distributed; more than 500,000 residents educated and supported through outreach materials and community health workers.
Pennsylvania (2020)	5	● Improve options for rideshare and alternatives to fixed route transportation for diabetic and limited mobility individuals accessing medical and non-medical resources. ● Coordination between regional food pantries; use of primary care screenings to refer individuals to food pantries and SNAP.	No formal evaluation metrics or process for establishing these metrics.	No publicly available data to assess outcomes related to the specific activities of the initiative.
Washington (2021)	3	● Creation and formalization of coalitions in selected urban and rural HEZs. ● TA provided by Washington State Department of Health. ● Interventions have not begun; planned interventions are not publicly available.	● During program design, an evaluation team assessed partner perception of the process to build coalitions and design programs. ● Each HEZ will create an evaluation plan before beginning interventions.	● Iteration on coalition building and program design saw increased clarity from partners on the purpose and structure of the project. ● No outcomes as program has not yet entered implementation stage.
Minnesota (2022)	50	● Facilitation of training and TA between and across members to the network. ● Coordination between network members and integration of services by region. ● New models of care access and delivery, including mobile care units.	● Health Equity Data Analysis (HEDA) methodology has been used by individual network members to establish priorities. ● No specific evaluation plans or criteria are publicly available.	● Iteration and improvement on the HEDA through hyperlocal data collection. ● Improved healthcare access for people experiencing homelessness. ● Widespread availability of health equity training across networks.
Georgia (2023)	6	Selected communities do not have publicly available intervention plans or identifiable work towards coalition building and collaboration.	No specific evaluation plans or criteria are publicly available.	No publicly available data to assess outcomes related to the specific activities of the initiative.

The data in **Appendix Table A.1** show that some interventions have been unequivocal successes, regardless of context and variance across zones. In Rhode Island, Pawtucket and Central Falls HEZs saw 44% and 38% reductions respectively in the rate of children affected by lead poisoning following educational interventions (RIDOH, 2023, p. 1). Teen healthcare interventions in Woonsocket and Central Falls were followed by 33% and 24% reductions in teen pregnancy respectively (RIDOH, 2023, pp. 2-3). Aggregate impact of interventions across HEZs were credited as a major contributor to a 30% reduction in “Rhode Islanders who do not seek medical care due to cost” over the course of seven years (RIDOH, 2023, p. 3). In Maryland, emergency room visits have dropped by over 40,000 people and inpatient stays by more than 20,000 since the implementation of HEZs. They have also determined that the program has saved far more money for the healthcare system than the cost of its implementation (Chavez & Galarraga, 2020; Gaskin et al., 2018). In Oregon, youth leadership and empowerment programs have seen a 100% graduation rate for participating high schoolers (Oregon Health Authority, 2017).

Other HEZ interventions have found more variance in their results. For example, interventions in Rhode Island to promote access to affordable housing have shown very different outcomes across zones. In the West Warwick HEZ, housing cost burden was reduced by 2% in the first year of programming, while the Central Providence HEZ saw a 23% reduction in three years (Rhode Island Department of Health & Executive Office of Health and Human Services, 2023, p. 1). While compelling, comparing metrics such as these is not necessarily a good approach to understanding their success, as each initiative takes a very different approach and is working with a significantly different context. For example, while some housing interventions focus specifically on rent mitigation, others focus on development of community housing and self-sufficiency of tenants.

Community feedback from constituents and partners has been overwhelmingly positive. In Rhode Island, testimony from leaders and participants in HEZ initiatives have credited the flexible funding and support structure for improvements in both quantifiable metrics and a growing “sense of confidence” and community empowerment within the zones (Rhode Island Department of Health, 2020).

HEZs have also shown positive outcomes as units of political advocacy. Initiatives have successfully spearheaded campaigns to increase racial and ethnic representation in local government, expand fair housing laws (Oregon Health Authority, 2017), pass bans on smoking in public parks, and create new codes and ordinances to promote green streets and accessible public transportation options (Rhode Island Department of Health, 2021). The ability to leverage existing social infrastructure and connect individuals to the social capital in their community within more flexible funding structures has been identified as crucial in overcoming political structures that produce and reproduce inequity (Putland et al., 2013; Perrotte & Noorestani, 2021). Beyond their capacity to advocate for more equitable policy, HEZs have also proved their ability to collectively bargain and advocate against politically polarized funding systems. In April 2024, the RIHEZ initiative was put at risk by the state legislature’s choice to remove over \$1 million in funding from the program. All 15 HEZs and dozens of the partners in their networks have organized to advocate for this funding to be restored (Raia, 2024). Though the

outcome is still uncertain, the ability to organize such a large-scale response is a clear indicator of the political power that community-based collaborative initiatives are capable of generating.

As HEZ initiatives become more widespread, both new and continuing programs will continue to collect and analyze data that will likely make it easier to assess both intervention outcomes and structural impacts of the HEZ model. HEZ-specific evaluation plans that continue to change and develop over the course of one or many intervention implementations will be crucial to understanding the short- and long-term impacts of this model for health equity.

Appendix Table A.2. Structure of the Chicago Department of Public Health (CDPH) and University of Illinois at Chicago (UIC) School of Public Health (SPH) Health Disparities Grant Evaluation Team by Respective Name, Role, and Organization (as of July 2024)

Name	Role	Organization
Genese (Genny) Turner	Director of Health Equity and Strategic Partnerships	CDPH
Brian Edmiston	Program Analyst	CDPH
Nicole Marcus	Program Analyst	CDPH
Sydney Edmond	Projects Administrator	CDPH
Aia Hawari	Project Manager	CDPH
Octavia Tyson	Public Health Administrator III	CDPH
Isabel (Izzy) Abrams	Project Coordinator	CDPH
Stephanie Salgado	Project Coordinator	CDPH
Veeta Nowell	Project Manager	CDPH
Paula Crose	PHAP	CDPH/CDC
Rahel Woldemichael	Program Manager	CDPH/CDC Foundation
Stephanie Jara	Epidemiologist II	CDPH
Benjamin Shaw	Project Director	UIC SPH
Diana Ghebenei	Project Manager	UIC SPH
Jeni Hebert-Beirne	Faculty Member	UIC SPH
Alisa Velonis	Faculty Member	UIC SPH
Tiffany Ford	Faculty Member	UIC SPH
Kelechi Ibe-Lamberts	Faculty Member	UIC SPH
Eve Pinsker	Faculty Member	UIC SPH
Abha Mahajan	Graduate Research Assistant	UIC SPH
Ayesha Chaudhry	Graduate Research Assistant	UIC SPH
Hodan Jibrell	Graduate Research Assistant	UIC SPH
Simone Taylor	Graduate Research Assistant	UIC SPH
Maura Kelleher	Graduate Research Assistant	UIC SPH

Izziah Thabath	Graduate Research Assistant	UIC SPH
Footnote (Terminology): CDPH (Chicago Department of Public Health), CDC (Centers for Disease Control and Prevention), UIC (University of Illinois at Chicago), SPH (School of Public Health)		

Table A.3. Overview of Advisory Group (AG) Structure by Respective Name, Role, Affiliation, and Region/Project for CDPH-UIC Health Disparities Grant Evaluation (2024)

Name	Role	Affiliation	Region/ Project
Jess Lynch	Contractor	IPHI	HEiAP
Katie Morin	Contractor	PHIMC	HCEZ
Kathryn Welch	HCEZ Reg/Comm Lead	Greater Auburn Gresham Development Corporation	HCEZ- Near South
Joyce Chapman*	HCEZ Reg/Comm Lead	Phalanx Family Services	HCEZ- Far South
Tina Sanders*	Substituted for Joyce Chapman	Phalanx Family Services	HCEZ- Far South
Shannel Crump*	Alternative representative for Tina Sanders	Phalanx Family Services	HCEZ- Far South
Patrick Daniels	HCEZ Regional Lead	SCHOSP	HCEZ- North/Central
Jessica Biggs	HCEZ Regional Lead	SWOP Chicago	HCEZ- Southwest
Minyoung Do	HCEZ Regional Lead	West Side United	HCEZ- West
Nancy Valentin	HCEZ Regional Lead	Northwest Center	HCEZ- Northwest
Mike Tomas	HEiAP Community Affiliate	Garfield Park Community Council	HCEZ- West
Ruby Ferguson	HEiAP Community Affiliate	Greater Chicago Food Depository/ Mayor's Office	HCEZ- Southwest
Britney Smart	HCEZ Regional Lead	Sinai Community Institute	HCEZ- West
Sydney Edmond	HEiAP Projects Administrator	CDPH	HEiAP
Isabel Abrams	HCEZ Project Coordinator	CDPH	HCEZ- North/Central
Stephanie Salgado	HCEZ Project Coordinator	CDPH	HCEZ- Northwest

Nik Prachand	Director of Epidemiology & Research	CDPH	HCEZ
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Footnote: (*) Tina Sanders became the main Advisory Group representative on behalf of Phalanx Family Services after Joyce Chapman's passing. Shannel Crump was an additional, alternative representative on behalf of Tina Sanders.

Terminology: Illinois Public Health Institute (IPHI), Public Health Institute of Metropolitan Chicago (PHIMC), Swedish Covenant Hospital (SCHOSP), Southwest Organizing Project (SWOP), Chicago Department of Public Health (CDPH), Healthy Chicago Equity Zones (HCEZ), and Health Equity in All Policies (HEiAP).

Appendix Table A.4. Overview of Healthy Chicago Equity Zones (HCEZ) Structure by Respective Region, Community Area (CA), Community Lead (CL) Organization, and Contact Person

Near South (N=12) Regional Lead: Greater Auburn Gresham Development Corporation Project Coordinator: John Jones	
Community Area (CA)	Community Lead (CL) Organization
Auburn Gresham	GAGDC
Avalon Park	GAGDC
Chatham	Chesterfield Community Services
Douglas	Centers for New Horizons
Englewood	Teamwork Englewood
Fuller Park	Centers for New Horizons
Grand Boulevard	Centers for New Horizons
Greater Grand Crossing	The Kindness Campaign
South Chicago	Chicago Family Health Center
South Shore	South Shore Works
Washington Park	Bright Star Community Outreach
West Englewood	Teamwork Englewood
Far South (N=13) Regional Lead: Phalanx Family Services Project Coordinator: Sona Fokum	
Community Area (CA)	Community Lead (CL) Organization
Beverly*	TBD
Burnside	New Covenant Harvest Kingdom of God Ministries
Calumet Heights	Southeast Calumet Heights Homeowners Association
East Side	Youth Advocacy Foundation
Hegewisch	Hegewisch Times
Morgan Park	Morgan Park Roots Community Organization
Mount Greenwood*	TBD
Pullman	National A. Philip Randolph Pullman Museum/Roses in Roseland
Riverdale	People Community Recovery
Roseland	Kids Off the Block
South Deering	New Generation Fancy Drill Team
Washington Heights	Graduates Over Guns Corp
West Pullman	Phalanx Family Services
North/Central (N=11) Regional Lead: Swedish Covenant Hospital Project Coordinator: Isabel Abrams	
Community Area (CA)	Community Lead (CL) Organization
Edgewater	Lutheran Social Services of Illinois
Lake View	Thresholds

Lincoln Park*	Swedish Covenant Hospital
Lincoln Square	Lutheran Social Services of Illinois
Near North Side*	Swedish Covenant Hospital
Near South Side*	Swedish Covenant Hospital
North Center	Common Pantry
Rogers Park	Heartland Health Centers
The Loop	Everthrive Illinois
Uptown	APNA GHAR
West Ridge	ICNA Relief/Rohingya Cultural Center
Northwest (N=13) Regional Lead: Northwest Center Project Coordinator: Stephanie Salgado	
Community Area (CA)	Community Lead (CL) Organization
Albany Park	North River Commission
Avondale	Palenque LSNA
Belmont Cragin	Northwest Center
Dunning	Northwest Center
Edison Park	Metropolitan Family Services
Forest Glen	Metropolitan Family Services
Hermosa	Northwest Center
Irving Park	Metropolitan Family Services
Jefferson Park	Metropolitan Family Services
Logan Square	Palenque LSNA
Montclare	Northwest Center
North Park	North River Commission
Portage Park	Metropolitan Family Services
Southwest (N=13) Regional Lead: Southwest Organizing Project Project Coordinator: Isaac Marrufo	
Community Area (CA)	Community Lead (CL) Organization
Archer Heights	PODER
Armour Square	Midwest Asian Health Association
Ashburn	Southwest Organizing Project
Bridgeport	Midwest Asian Health Association
Brighton Park	Esperanza Health Centers
Chicago Lawn	Southwest Organizing Project
Clearing	Envision Community Services
Gage Park	Increase the Peace/La Casa Norte
Garfield Ridge	Southwest Collective
McKinley Park	The Resurrection Project at Casa Hidalgo
New City (Back of the Yards)	Increase the Peace/La Casa Norte

West Lawn	Envision Community Services
West (N=9) Regional Lead: Rush University Medical Center (On behalf of West Side United) Project Coordinator: Nicole Marcus	
Community Area (CA)	Community Lead (CL) Organization
Austin	Austin Coming Together
East Garfield Park	Breakthrough
Humboldt Park	Erie Family Health Center
Lower West Side	Esperanza Health Centers
Near West Side	Equal Hope
North Lawndale	Sinai Community Institute
South Lawndale	Enlace
West Garfield Park	Sinai Community Institute
West Town	Erie Neighborhood House
Footnotes: • This table includes representation for a total of 71 out of 77 potential Chicago CAs. CAs that are not represented in this table include: Near South (Oakland, Kenwood, Hyde Park, Woodlawn) and Northwest (Norwood Park, O'Hare). • An asterisk (*) next to a CA indicates the lack of a contact person. Therefore, this information has been substituted with the contact person of the respective AG member and is thereby also denoted by an asterisk (*). • Far South (Beverly and Mount Greenwood) had "N/A" for contact person and North/Central (Lincoln Park, Near North Side, Near South Side) had "Swedish Covenant Hospital" for contact person.	

Table A.5. Overview of Health Equity in All Policies (HEiAP) Structure by Respective Name, Role (Organization), Healthy Chicago (HC) 2025 Priority Area, and Community Co-Led Table (2024)

Name	Role (Organization)	HC 2025 Priority Area	Community Co-led Table
Vacant	Deputy Commissioner (CDPH) (until HEiAP office moved to IDEA bureau)	N/A	N/A
Sydney Edmond	Projects Administrator (CDPH)	All	All
Susan Martinez	Coordinating Planner (CDPH)	Food Access + Environmental Justice	Food Equity Council + Environmental Equity Working Group
Eduardo Munoz	Coordinating Planner (CDPH)	Housing + Neighborhood Planning & Development	Systems Change Collaborative* (No longer convening) + Elevated Chicago/ETOD
Clayton Oeth	Senior Policy Analyst (CDPH)	Food Access + Environmental Justice + Housing + Neighborhood Planning & Development	Food Equity Council + Environmental Equity Working Group
Mervin Dino	Assistant Commissioner (CDPH)	N/A	N/A
Aia Hawari	Project Manager (CDPH) from December 2023 to May 2024	Institutional Change	HEiAP Community Accountability Committee
Vacant	Project Coordinator (CDPH)	Institutional Change	N/A
Vacant	Senior Policy Analyst (CDPH)	Housing + Neighborhood Planning & Development	Systems Change Collaborative* (No longer convening) + Elevated Chicago/ETOD
Vacant	Director, Program Operations (CDPH)	N/A	N/A

Jess Lynch	Program Director (IPHI)	N/A	N/A
Vacant	Senior Program Manager (IPHI)	N/A	N/A

Appendix Table A.6. Draft Evaluation Questions

CAPACITY BUILDING	
Communities	
1.	What are the key characteristics of community-based organizations (CBOs) and individuals being engaged to build capacity for advancing racial and health equity at the local level? How were they selected?
2.	What communities have completed community health assessments (CHA)? Are the chosen health priorities consistent with community sentiment and available data?
3.	What communities have completed and implemented racial/health equity action plans?
4.	What skills and resources have been transmitted to CBOs, and are there inequities in the transmission of these skills/resources? How are CBOs integrating these skills/resources in their work?
5.	What have been the barriers and facilitators that communities faced when developing and implementing these plans?
6.	What benefits or unanticipated consequences have been realized by communities as a result of this work?
Public Health (PH) System and City Government	
1.	What city departments are engaged in CDPH's health equity initiatives and in what ways? How were they selected as partners? Are selected partners inclusive of and directly connected to people with lived experience of inequity?
2.	What is being done within CDPH to enhance collaboration in support of health equity initiatives?
3.	In what ways has capacity for improving health equity around community-identified health priorities increased within CDPH and other city agencies/departments?
4.	Are CDPH's health equity initiatives changing the way they engage with communities or the ways in which they work internally and with other city agencies or other partner organizations?
5.	Is there evidence of changing practice and processes across the PH system to address health equity? What systems changes (e.g., organizational practices and policies, legislative policies) are being worked on or have been accomplished as a result of the CDPH healthy equity initiatives?
POWER SHARING AND ACCOUNTABILITY	
Communities and PH System and City Government	
1.	Where in the spectrum/continuum of community engagement (i.e., outreach, consult, involve, collaborate, shared leadership) is the current collaboration between CDPH and CBOs involved in HCEZ or HEiAP?
2.	Where do most collaborations among the CBOs fit on this continuum?
3.	How has nature of engagement between these groups changed over time?
4.	Are there inequities in engagement (i.e., some CBOs or some regions being more involved than others)?

- | |
|--|
| 5. What processes or accountability structures have been established by CDPH, CBOs, or other partners to increase power sharing and accountability? |
| 6. What does successful power sharing look like to communities? |
| 7. What are key barriers and facilitators to relationship building and reaching shared leadership between communities, CDPH, and the local PH system? |
| 8. What do partners (e.g., CDPH, PH systems, CBOs, other community and intergovernmental partners) believe are the key factors that influence the development of trust between them? |

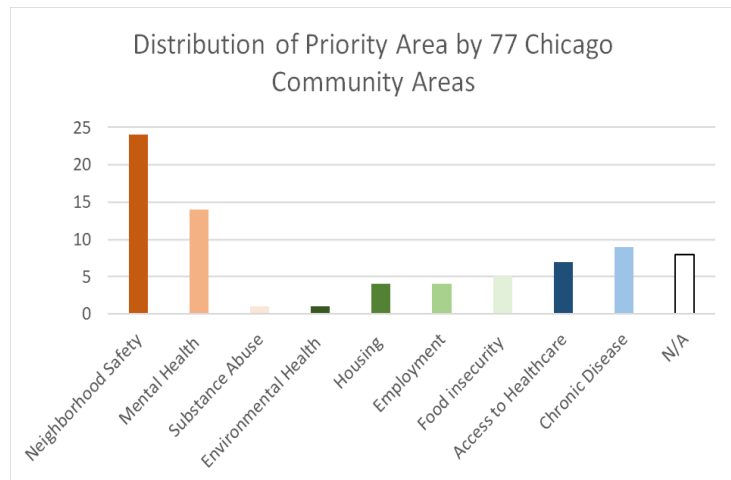
Appendix A.7. Analyses of Community Health Assessment (CHA)

We have looked at the geographic patterns of health priorities selected across communities and regions, as these could inform the types of capacity and partnership building efforts that are most needed within communities, within the public health system, or across governmental agencies. In particular, if neighboring CAs share certain health priorities, there may be value in establishing or nurturing existing partnerships between these communities to increase the effectiveness of their efforts. Moreover, identifying those key health concerns that are shared across CAs and regions will allow communities to better assess their collective capacity to address these concerns. If certain health areas are being selected as high priorities by many or most communities, this suggests the need for capacity building and partnership development efforts at the city-level and within communities that is focused specifically in areas that will help to address the most common health priorities.

In addition, with health priorities identified, communities can begin examining correlates of the health priorities chosen by RLs and CLs to better understand what factors may be driving these priorities. For example, below we have examined patterns of chosen health priorities in comparison to the following three community level factors: (1) reported violent crime rates; (2) HCEZ funding tier (which is a proxy for community vulnerability); and (3) CBO type of the CL.

For reference, see **Appendix Figure 7.1**, which presents the distribution of health priorities across CAs. For the purposes of these analyses, we have combined groups of health priorities as follows:

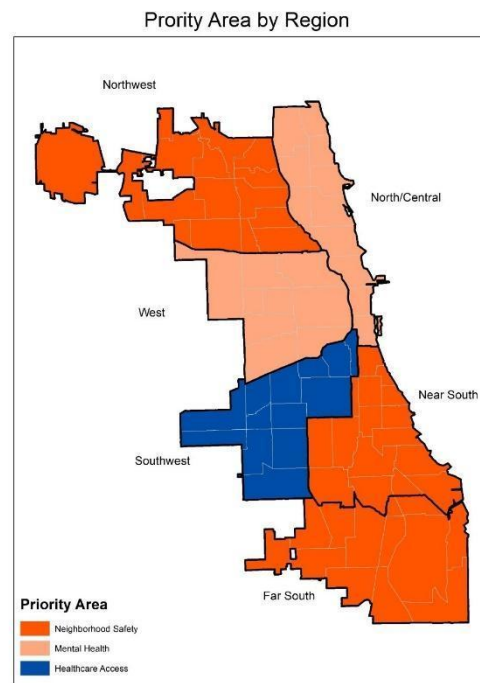
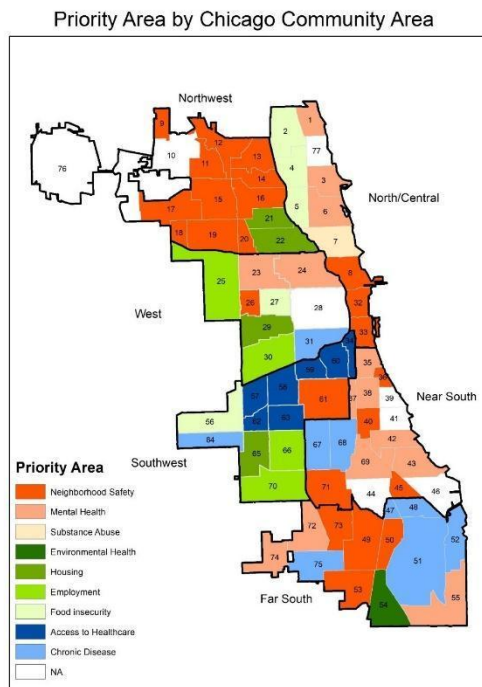
- Public safety and violence prevention = Neighborhood safety
- Mental health and mental health services = Mental health
- Access to primary care and preventative care = Healthcare access
- Food insecurity/access = Food insecurity
- Employment and economic stability = Employment
- Diabetes, smoking, and heart disease = Chronic disease



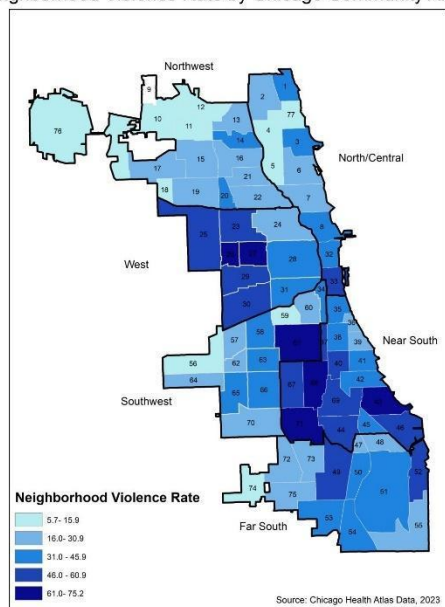
Housing, substance use, and environmental health remain as distinct categories.

Geographic patterns of chosen health priorities. We used GIS to map the spatial distribution of the top health priority areas chosen across CAs and regions (see **Appendix Figure 7.2**). As these maps show, neighborhood safety was identified as a top health priority across multiple communities and regions. The prominence of this health priority across multiple, and often

neighboring, communities suggests the need for partnerships across those particular communities and regions who chose neighborhood safety as a priority, as these partnerships will likely be key to addressing this shared priority. Furthermore, the prominence of both neighborhood safety and mental health as priorities across multiple communities in Chicago suggests the need for an assessment of efforts within CDPH and the broader city public health system to bolster resources and partnerships that are equipped to address neighborhood safety and mental health as these have not traditionally been focal points in public health.



Neighborhood Violence Rate by Chicago Community Area



Appendix Figure A.7.1. Priority Area by Chicago Community Area (CA) and Region (top panels); Neighborhood Violence Rate by Chicago CA ((bottom panel).

Correlates of chosen health priorities. Using data from the Chicago Health Atlas, we have also analyzed how RLs and CLs' top health priorities compare the available data relevant to that health area. As shown in the bottom panel of **Appendix Figure A.7.1.**, which displays a map of (reported) violent crime rates in Chicago's CAs, the highest actual violent crime rates appear to be in communities located in the West, Southwest, and Near South regions. When compared to the health priorities shown in top panel of **Appendix Figure A.7.1.**, we see some potential

discrepancies that may be worth examining further. For example, many of the communities in the Northwest region have identified neighborhood safety as their top health priority yet reported rates of violent crime are relatively low in most of these communities. On the other hand, in the CA of Austin (#25) rates of violent crime are relatively high, yet its top priority is not neighborhood safety, but rather employment. This highlights how communities can utilize their new capacity and resources related to data, and their new focus on social determinants, to better understand their local health-related priorities within context. In the case of Austin, for example, the Chicago Health Atlas data indicate that rates of violent crime are high, but the community has used additional data to understand a more fundamental problem that must be prioritized.

Our preliminary assessment of the chosen health priorities has also included an examination of the distribution of these priorities across funding tiers and across CBO types. **Appendix Tables A.7.1 and A.7.2** show the distribution of health priorities across funding type. This assessment does not reveal any strong patterns of correlation between funding type and health priority. For example, as shown in **Appendix Table A.7.1.**, regions with both relatively low levels of Tier A funding (e.g., Northwest with 40% of CAs with Tier A funding) as well as those with relatively high levels of Tier A funding (e.g., Near South and Far South with 88% and 85% of CAs receiving Tier A funding, respectively) have chosen neighborhood safety as a top health priority. A similar pattern can be seen with respect to mental health as a top priority, chosen by both the North/Central region (with only 27% of CAs receiving Tier A funding) and the West region (with 78% of CAs receiving Tier A funding). The patterns observed at the CA level, as shown in **Appendix Table A.7.2.**, also show no strong patterns of association between funding tier and chosen health priority. Regardless of funding tier, neighborhood safety was the most commonly chosen health priority, followed by mental health. These patterns, or more accurately the lack of any systematic patterning between funding tier and chosen health priority, suggest that the choice of top health priorities in a region or community was not simply a function of level of funding or baseline resources.

On the other hand, **Appendix Tables A.7.3 and A.7.4** seem to show a potential pattern of association between chosen health priorities and CBO type of CLs in a region. In particular, it seems that neighborhood safety was most likely to be chosen as a top health priority in regions that are predominated by CLs who are characterized as community-based or community development organizations (e.g., the Northwest with 93% of CLs characterized as community-based or community development org, the Near South with 81%, and the Far South with 62%). In contrast, it seems that mental health was most likely to be chosen as a top health priority in regions that are predominated by CLs who are health-based organizations (e.g., North/Central with 55% of CLs characterized as health-based orgs, and the West with 56%). This same pattern is not as readily apparent at the CA level (see **Appendix Table 7.4**), but this preliminary analysis indicates that the choice of CBO types that serve as CLs in a region may have some influence on the chosen health priorities and suggests the need for further analysis of the process used to select CLs and the extent to which important partners in the community or region were well represented by the CHA.

Appendix Table 7.1. Top Health Priority Area in Region, by Funding Distribution (2023)

	Northwest N (%)	North/ Central N (%)	Near South N (%)	Far South N (%)	Southwe st N (%)	West N (%)
Top Health Priority	Neighborhood safety	Mental health	Neighborhood safety	Neighborhood safety	Healthcare access	Mental health
Funding level						
Tier A (\$220,000)	6 (40%)	3 (27%)	14 (88%)	11 (85%)	11 (85%)	7 (78%)
Tier B (\$46,200)	9 (60%)	8 (73%)	2 (12%)	2 (15%)	2 (15%)	2 (22%)
Total CAs	15	11	16	13	13	9

Footnotes:

- Not all CAs have representation but all were allocated either Tier A or B funding.
- Health priorities were collapsed as follows:
 - Violence prevention and public safety = Neighborhood safety
 - Behavioral health and mental health/substance use treatment = Mental health
- Original classification:
 - Northwest – Public safety
 - North/Central – Behavioral health
 - Near South – Public safety
 - Far South – Violence prevention
 - Southwest – Healthcare access
 - West – Mental health and substance use treatment

Appendix Table 7.2. Distribution of Top Priority Area in 77 Chicago Community Areas (CAs), by Funding Level (2023)

Priority Area	For 77 CAs N (%)	For 52 CAs in Tier A (\$220,000) N (%)	For 25 CAs in Tier B (\$46,400) N (%)
Neighborhood safety	24 (31%)	15 (29%)	9 (36%)
Mental health	14 (18%)	10 (19%)	4 (16%)
Substance abuse	1 (1%)	--	1 (4%)
Environmental health	1 (1%)	1 (2%)	--
Housing	4 (5%)	3 (6%)	1 (4%)
Employment	4 (5%)	4 (8%)	--
Food insecurity	5 (6%)	2 (4%)	3 (12%)
Healthcare access	7 (9%)	6 (12%)	1 (4%)
Chronic disease	9 (12%)	9 (17%)	--
N/A	8 (10%)	2 (4%)	6 (24%)

Footnotes:

- Not all CAs have representation but all were allocated either Tier A or B funding.
- Health priorities were collapsed as follows:
 - Violence prevention and public safety = Neighborhood safety
 - Behavioral health and mental health/substance use treatment = Mental health
- Original classification:
 - Northwest – Public safety
 - North/Central – Behavioral health
 - Near South – Public safety
 - Far South – Violence prevention
 - Southwest – Healthcare access
 - West – Mental health and substance use treatment

Appendix Table 7.3. Top Health Priority in Region, by Community Lead (CL) Organization Type (2023)						
	Northwest N (%)	North/Cen- tral N (%)	Near South N (%)	Far South N (%)	Southwe- st N (%)	West N (%)
Top Health Priority	Neighborhood safety	Mental health	Neighborhood safety	Neighborhood safety	Healthcare access	Mental health
Organization Type						
Health-based	0	6 (55%)	2 (13%)	0	3 (23%)	5 (56%)
Faith-based	0	2 (18%)	0	3 (23%)	3 (23%)	1 (11%)
Social services provider/ SDoH-focused	0	2 (18%)	0	2 (15%)	3 (23%)	0
Other community-based/ community development	14 (93%)	1 (9%)	13 (81%)	8 (62%)	4 (31%)	3 (33%)
Total CAs	15*	11	16**	13	13	9
Footnotes: • (*) No representative for O'Hare in Northwest (14/15 classified). • (**) Near South has two CAs that are not classified (Kenwood, Hyde Park) but one CA (South Shore) that has two organizations (one community-based and one health-based). • The following categories were collapsed with these original sub-categories: ○ Health-based = Community/health, community-based/youth/mental health, health service provider, mental/behavioral health ○ Social services provider/SDoH-focused = Workforce development, community-based/housing ○ Other community-based/community development organization = NGO; community development; community-based and civic; community-based; community/prevention/youth-based						

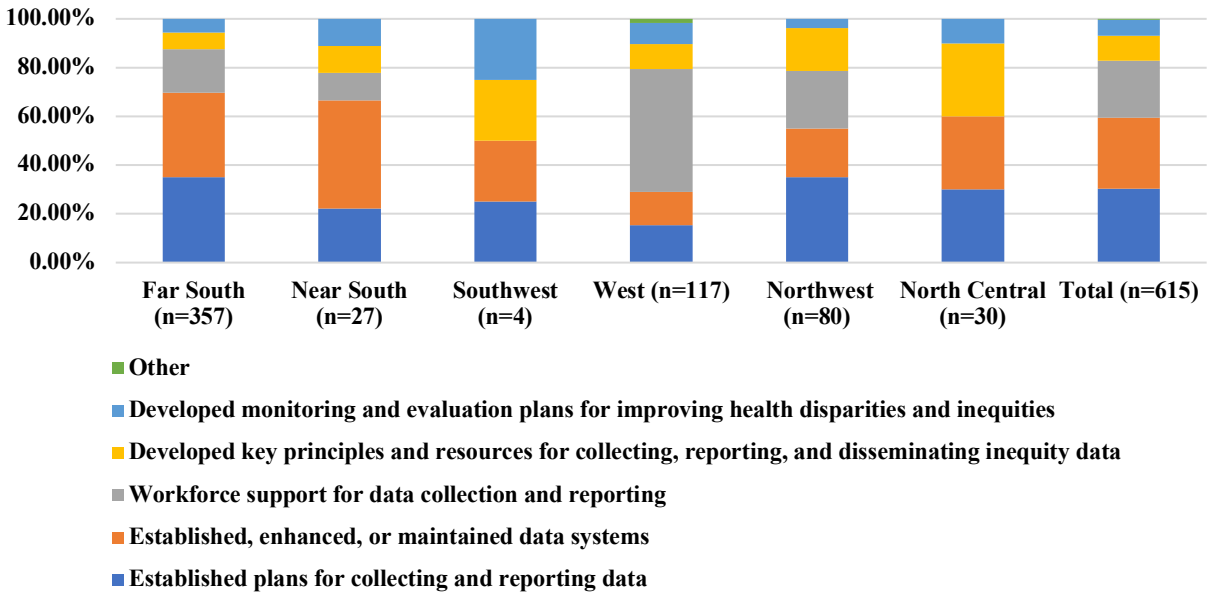
Appendix Table 7.4. Distribution of Top Priority Area in 77 Chicago Community Areas (CA), by Community Lead (CL) Organization Type (2023)

Priority Area	For 77 CAs N (%)	For 15 Health-Based Organizations N (%)	For 9 Faith-Based Organizations N (%)	For 7 Social Services Provider/SDoH-Focused Organizations N (%)	For 42 Other Community-Based/Community Development Organizations N (%)
Neighborhood safety	24 (31%)	4 (27%)	1 (11%)	1 (14%)	18 (43%)
Mental health	14 (18%)	3 (20%)	2 (22%)	1 (14%)	8 (19%)
Substance abuse	1 (1%)	1 (7%)	--	--	--
Environmental health	1 (1%)	--	--	1 (14%)	--
Housing	4 (5%)	1 (7%)	--	--	3 (7%)
Employment	4 (5%)	--	3 (33%)	--	1 (2%)
Food insecurity	5 (6%)	--	1 (11%)	1 (14%)	3 (7%)
Healthcare access	7 (9%)	3 (20%)		3 (43%)	1 (2%)
Chronic disease	9 (12%)	1 (7%)	1 (11%)	--	7 (17%)
N/A	8 (10%)	2 (13%)	1 (11%)	--	1 (2%)

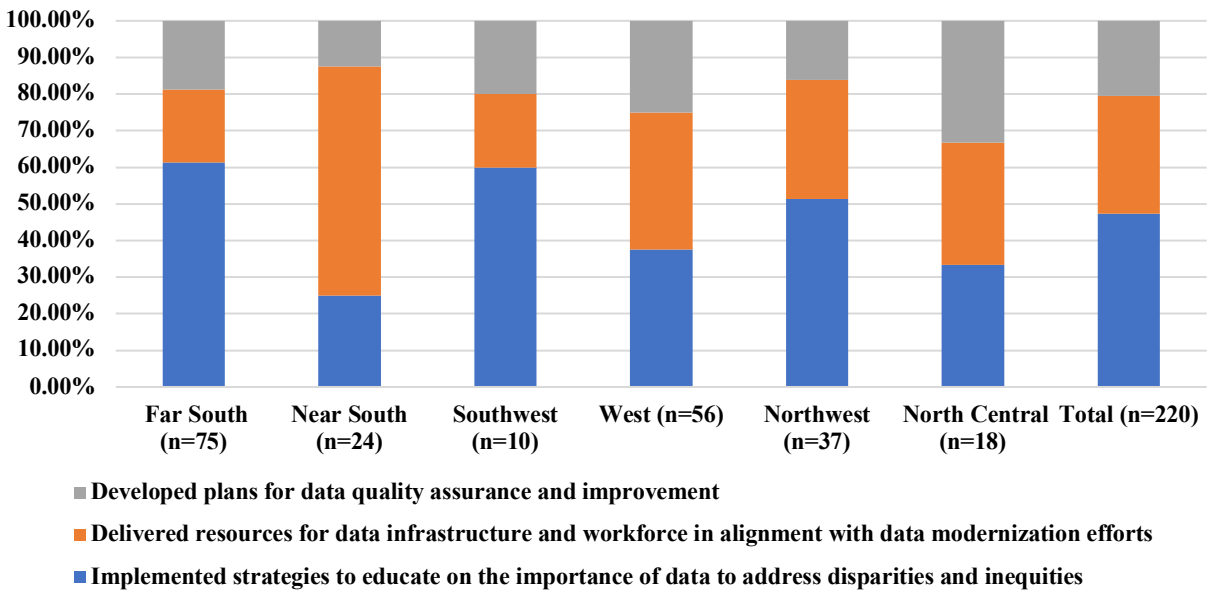
Footnote:

- This table only includes 73 organizations. The CAs missing organizations include: Hyde Park, Kenwood, O'Hare, and Norwood CAs.
- The following categories were collapsed with these original sub-categories:
 - Health-based = Community/health, community-based/youth/mental health, health service provider, mental/behavioral health
 - Social services provider/SDoH-focused = Workforce development, community-based/housing
 - Other community-based/community development organization = NGO; community development; community-based and civic; community-based; community/prevention/youth-based

Appendix Figure A.8
Distribution of Activities, Strategy 2.1 Data Collection, By Region



Appendix Figure A.9
Distribution of Activities, Strategy 2.2 Data Quality, by Region



Appendix Figure A.10
Distribution of Activities, Strategy 3.1 Infrastructure, by Region

